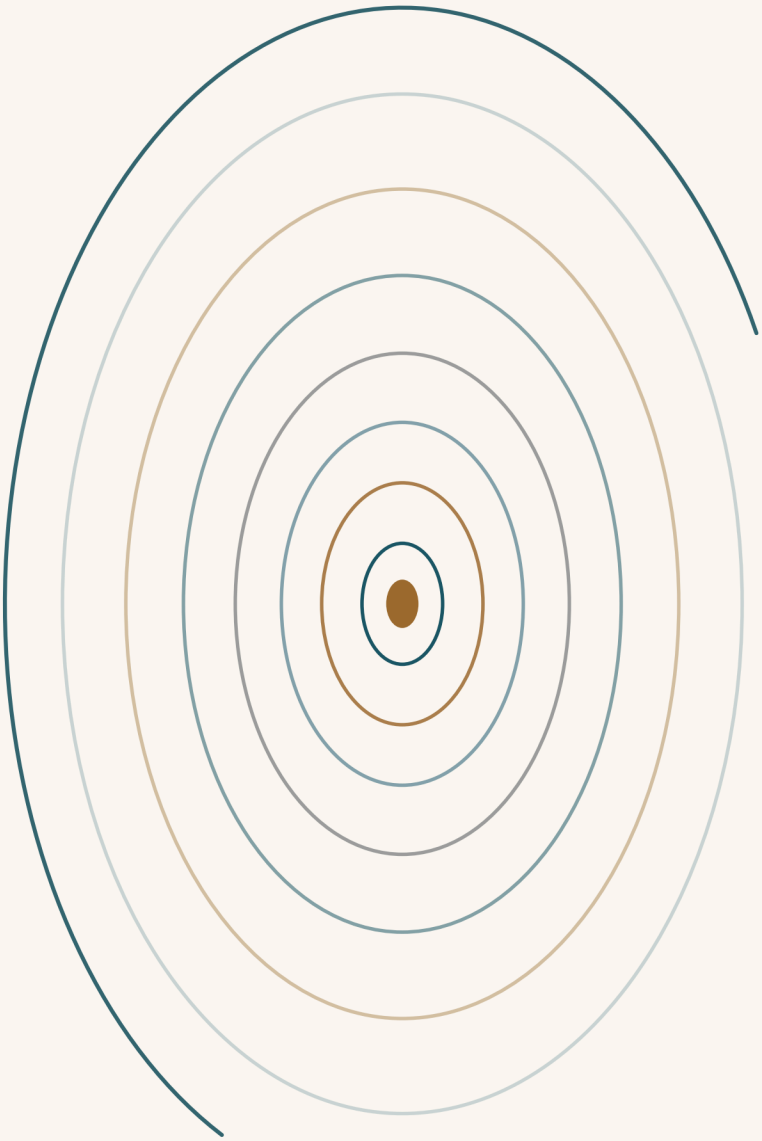
CAC MASTERS SERIES · VOLUME 2

CAC. 701

CHILD DEVELOPMENT THEORIES

MASTER IN CHILDREN AND ADOLESCENT COUNSELING



TRIUNE BIBLICAL UNIVERSITY GLOBAL EXTENSION USA

CHILD DEVELOPMENT THEORIES

CAC. 701

Child Development Theories

CAC. 701 · First Semester · 1.6 units · A complete lecture course for a sixteen-week semester

TRIUNE BIBLICAL UNIVERSITY GLOBAL EXTENSION USA

First Edition · 2026

Student Handbook of Comprehensive Lectures
Master in Children and Adolescent Counseling

First Edition, 2026

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DEDICATION

To the children and young people who sit in the room and are not yet sure whether they can be helped —

and to the counsellors who learn to be worth trusting.

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About This Book

This volume is the complete lecture course for **CAC. 701 Child Development Theories**, a First Semester core course of the Master in Children and Adolescent Counseling, carrying 1.6 units. It is written to be taught from and studied from without any other material in the room.

What the course is for

Every other course in the programme will teach you something to do with a child or an adolescent. This one teaches you why doing it might work, and how you would know whether it did. A counselling theory is not a belief system and not a loyalty; it is a set of answers to four hard questions — what a child is, what maintains the difficulty, by what mechanism change occurs, and what the counsellor therefore does. Answered well, those questions make practice deliberate, teachable and defensible to the parent, the school, the court or the licensing board that asks what you did and why. Answered badly or not at all, practice becomes a series of intuitive moves that cannot be evaluated and cannot be revised when they fail.

The course moves through the four great lineages of counselling theory — psychodynamic and attachment, behavioural and cognitive, humanistic and existential, systemic and family — reading each against the same four questions, and ends where practice actually lives: in the integration of competing accounts into a formulation for one particular child, held to the standard of the available evidence.

How the semester is organised

Sixteen weeks. Weeks 1 to 14 teach. Week 15 integrates: a comparative practicum in which the whole course is applied to a single extended case. Week 16 consolidates for examination. Each week is one lecture unit of about four notional hours — roughly ninety minutes of scheduled teaching, ninety minutes of seminar or practical work, and the remainder your own reading and writing. Each week has the same shape, and the shape is deliberate: outcomes you can be examined on;

the place of the week in the argument; four key concepts each tied to something a counsellor does in a room; three lectures built from propositions rather than narrative; at least one diagram placed where the argument needs one; worked examples; a practical with a debrief; seminar questions; the assessed task and its criteria; the reading, with the core text marked; and one sentence of synthesis.

A note on the child

The material is addressed throughout to work with children and adolescents, and that is not a decoration on an adult curriculum. A child cannot free-associate, may lack the vocabulary for a cognitive dispute, does not choose to attend, and is embedded in a family and a school that are themselves part of the problem and part of the solution. Theories built for adults fail in particular, predictable ways in a room with a nine-year-old. One purpose of this course is to teach you to see the failure coming.

Standards observed in this volume

British spelling in the book's own prose; American spelling preserved in quoted titles, journal names and direct quotations. Citations in APA 7th edition; only works actually cited are listed; no quotation, statistic or page number has been invented. All case material is fictitious and labelled as such; photographs generated for teaching are captioned as such; standardised instruments are described and attributed but never reproduced. Diagrams are drawn for this edition. The file is a tagged PDF conforming to PDF/UA-1. The governing specification is *HANDBOOK-STANDARD.md*, held with the series.

How to Use This Book

Read the outcomes first and last. Each week opens with what you will be able to do on completing it, written as capabilities that can be observed and examined. Read them before the week and again after it. If you cannot do what they say, you have not finished the week, however much of it you have read.

Work the examples as you meet them. Worked examples are set apart and are not optional reading. Cover the solution, attempt it, then compare. The distance between your attempt and the printed one is precisely the material the examination tests.

Do the practicals. Every week carries a practical — a timed exercise, a formulation task, a role-play, an observation or an analysis of supplied material — with a debrief. Most of them require another person or a recording of yourself and a clock. The practical is the part of the week most often skipped and the part that most changes practice. A theory you have only read is a theory you cannot use.

Keep the comparison grid. Figure 1.3 in week 1 is a blank grid: five questions down the side, four approaches across the top. Complete one column each time the course meets an approach, in your own words, and by week 15 reproduce the whole grid from memory and defend every cell. The grid is the course. Everything else is commentary on it.

Argue with the lectures. Each lecture ends in a clinical implication and a seminar question. The seminar questions are not rhetorical; write answers to them. Where a lecture states a contested position, the contest is named, and your task is to hold a position and say what would change it.

For the tutor. Each week is one four-hour block: ninety minutes of teaching, ninety minutes of seminar or practical work, the remainder preparation and follow-up. The three lectures are self-contained and may be reordered where a cohort needs the case before the theory. The assessed task printed in each week is the task set, and the criteria printed with it are the criteria used. Substitute local material in the vignette and the practical where a cohort's placements differ, provided the substitute is fictitious and labelled.

For the supervisor. The formulation exercises are yours. The question that converts knowledge into judgement is always the same: *which of the approaches in this week would formulate this case differently, and what would that formulation lead you to do on Tuesday?* A trainee who can state a mechanism of change and then say what would falsify it is ready for supervised caseload. A trainee who can only describe a theory is not.

Conventions

Convention	Meaning
Week *n*	the *n*th lecture week of the sixteen-week course
Lecture I, II, III	the three teaching sequences within a week
Worked example	a fully solved illustration; attempt it before reading the solution
Practical	a timed exercise with a debrief; requires a partner, a recording or supplied material
Hypothetical / fictitious composite	case material invented for teaching; no real client is described
Illustrative image (AI-generated)	a photograph created for this edition; depicts no identifiable person
Jurisdiction-dependent	a legal or reporting duty that must be checked against local law

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The Semester at a Glance

Sixteen weeks, one lecture unit each. Weeks 1–14 teach the course; week 15 integrates it on a single extended case; week 16 prepares the examination. The right-hand column names the assessed work of each week; the criteria for each are printed inside the week.

Week	Title	Principal lineage or task	Assessed
1	Mapping Development	the four questions; methods; ethics; the child in context	Reading response, 600 words
2	Foundations Before Birth	genes, teratogens, epigenetics, early adversity	Teratogen case analysis
3	The Constructing Child	Piaget: sensorimotor and preoperational thought	Conservation demonstration log
4	The Social Mind	Vygotsky: ZPD, scaffolding, private speech	Scaffolding exercise
5	Processing and Executive Function	attention, working memory, theory of mind	False-belief critique, 800 words
6	Temperament, Attachment, Regulation	goodness of fit; emotion regulation	Goodness-of-fit analysis
7	Language and Communication	milestones, bilingualism, red flags	Language sample analysis
8	Self, Identity, Morality	Erikson, Kohlberg, Gilligan, conscience	Moral reasoning interview
9	Peers, Play and Gender	Parten, friendship, gender theories	Play observation record
10	Family, Culture, Adversity, Resilience	parenting styles, ACEs, ordinary magic	Adversity and resilience map

Week	Title	Principal lineage or task	Assessed
11	Atypical Development I	neurodevelopmental presentations, developmentally read	Developmental timeline case
12	Atypical Development II	developmental psychopathology: equifinality, transaction	Equifinality essay, 1,200 words
13	Integration	the developing child in the counselling room	Formulation portfolio
14	Observation, Screening, Measurement	tools, cultural fairness, outcome measurement	Screening critique
15	The Practicum: Three Ages	case conference across toddler, school-age, adolescent	Practicum presentation
16	Revision and Examination	consolidation, past questions, examination technique	Final examination

Notional hours. 1.6 units ≈ 24 notional learning hours across the sixteen weeks, of which roughly one third is scheduled teaching and two thirds your own preparation, observation and writing. Weeks vary: the practicum week is heavier in supervised work, the revision week lighter in teaching.

Reading load. Each week marks one core text among its six readings. Read the core text before the week's teaching and the remainder across the week. The core texts of the course are Berk's *Child Development*, Santrock's *Child Development*, and Shonkoff and Phillips's *From Neurons to Neighborhoods*.

Child Development Theories

SEMESTER	UNITS	CATEGORY
First	1.6	Core

Every counsellor of children carries a theory of development, whether or not it has been examined. It is in the sentence "he should have grown out of it by now", in the worry "she is not like the others", in the quiet judgement on a mother. This course puts those theories on the table, names them, tests them against evidence, and hands you back a set of instruments for watching a child develop — and for noticing, early and accurately, when development has left its ordinary course.

Child development is not one theory but a conversation among several: the maturational and the learning accounts; Piaget's constructing child and Vygotsky's social mind; the information-processing account with its bottlenecks; the attachment and temperament account with its transactions; the linguistic, the moral, the ecological; and, over all of them, the principles of developmental psychopathology that tell us how ordinary variation shades into clinical concern and back again. The course moves through these in sixteen weeks, and it never lets you forget the fourth question of week 1: what does this mean for the child in front of you on Thursday?

Two commitments govern the volume. The first is to evidence and to its limits: where a finding comes from a narrow sample, you will be told; where a classic claim has been revised, you will meet the revision; where the honest answer is "we do not know", the book will say so rather than decorate. The second is to context: the children of this programme grow up in compounds and classrooms, markets and churches, in several languages and in economies that tax their parents' patience. A development science that cannot speak to that world is not the science this course teaches.

A fictitious family — Ife, three years and eight months, and her brother Kunle, seven — accompanies the course from week 1 to week 15, so that each theory meets the same children and you can watch the theories disagree about them.

1 LECTURE UNIT 1 Teaching week 1 4 notional hours

Mapping Development: Questions, Methods, and the Child in Context

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **State the four questions** of developmental science — what develops, by what processes, with what universality and variation, and to what purpose for the child in front of you — and place any theory on that map.
2. **Compare** longitudinal, cross-sectional and sequential designs, naming what each can and cannot establish about change.
3. **Choose** a method of studying a child — observation, informant report, self-report, task — appropriate to the child's age and the question asked, stating its bias.
4. **Apply** the ecological model to one child, locating factors at micro, meso, exo and macro levels and in time.
5. **Conduct** research and observation with children ethically: consent and assent, power, and the return of findings to families.

Where this week sits

Volume 1 asked what a theory is for. This volume asks what develops, and how we could possibly know. Child development theory is not a museum of famous names; it is a set of answers to four questions, each answer carrying a method and a method carrying a bias. Every week that follows is one lineage's answer; this week builds the frame you will hang them on, and introduces the child who will accompany us all semester.

Key concepts

THE FOUR QUESTIONS

What develops (description and sequence)? By what processes (mechanism: maturation, learning, transaction)? With what universality and variation (which patterns hold across peoples, and which are local)? To what purpose (what does this tell the counsellor in the room)? In the room: the grid on which every theory in this volume is filed.

DEVELOPMENT AS TRANSACTION

The modern consensus: development is neither the unfolding of a programme nor the writing on a blank page, but a continuous transaction between a child who

acts and a context that responds, each changing the other (Sameroff's transactional model, developed in week 12). In the room: the reason "what is wrong with this child?" is always the wrong first question.

SENSITIVE PERIOD
A window in which an experience organises a system far more powerfully than it later can — language phonology, vision, attachment. Not a door that locks, but a gradient of cost. In the room: the difference between urgency and panic when a child presents late.

ECOLOGICAL MODEL
Bronfenbrenner's nested contexts: microsystem (family, class, peers), mesosystem (the links between them), exosystem (settings the child never enters but which reach the child: a parent's workplace, a school board's decision), macrosystem (culture, law, economy, belief), and chronosystem (time). In the room: the checklist that stops a formulation being a portrait of a child with the world airbrushed out.

Lecture I — The four questions and the child who carries them

Proposition 1. Description precedes explanation, and description is harder than it looks. Before any theory, someone had to watch: to notice that most children walk between nine and eighteen months, that a three-year-old explains rain as the sky crying, that a seven-year-old can hold a rule in mind and a four-year-old mostly cannot. Norms are descriptions with distributions, not deadlines; the counsellor who treats a norm as a deadline pathologises the tails of every curve.

Proposition 2. Mechanism is where theories actually differ. Maturation accounts (development as ripening), learning accounts (development as accumulated experience), and transactional accounts (development as a child–context loop) make different predictions about what an intervention can change and how fast. When two therapists argue about a child, they are usually arguing about mechanism while believing they are arguing about the child.

Proposition 3. Universality is a finding, not an assumption. Some sequences hold wherever anyone has looked — sitting before walking, babbling before words, desire-psychology before belief-psychology. Others, including several celebrated "stages", shift with schooling, language and the practices of a culture. The discipline of this volume: for every pattern, ask where it was found, in whom, and whether it has been found elsewhere.

Proposition 4. Purpose governs method. We study development in order to help particular children, and every method has a price: the laboratory gains precision and loses the child's Tuesday; the home observation gains the Tuesday

and loses control; the parent report gains history and loses the present; the child's own report gains the inside and loses the outside. Method choice is therefore an ethical act, not a technical one.

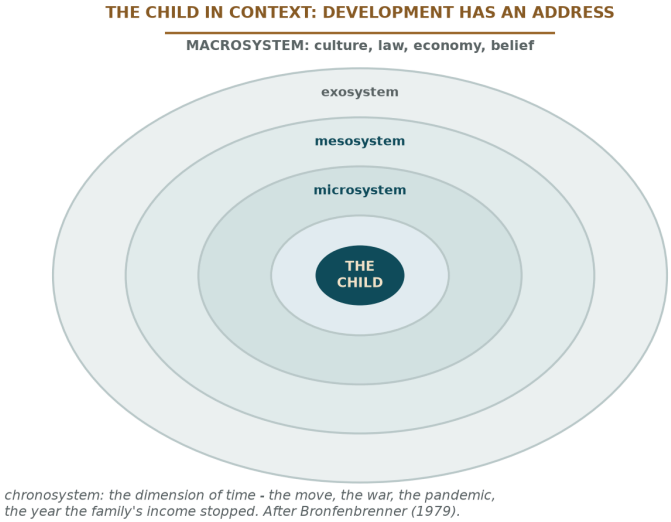


Figure 1.1 Bronfenbrenner's nested ecological systems: microsystem, mesosystem, exosystem and macrosystem around the child, with the chronosystem as the dimension of time. After Bronfenbrenner (1979). *Source:* prepared for this edition.

> **PRACTICE NOTE**
> **Clinical implication.** Begin every case with five minutes of ecology before five minutes of child: who is in the microsystem, which links are working and which are broken, which exosystem decision landed on this family this month, which macrosystem belief is naming this child's difficulty, and what changed in time. The child's symptoms sit inside that map, never outside it.

The running case of this volume

Ife and Kunle — a fictitious composite. Ife is three years and eight months old. She speaks in three- and four-word sentences, insists on the same cup, hides behind her mother's wrapper when strangers greet her, and at nursery settles only after twenty minutes of watching from the doorway. Her brother Kunle is seven: quiet in class, quick at figures, called "proud" by his teacher because he does not ask for help. Their mother trades in the market six days a week; their father drives a commercial bus on night routes; their grandmother lives in the same compound and holds the household together. No real family is described here; every detail is invented for teaching, and the details will accumulate week by week as each theory meets them.

Lecture II — Methods: how we could possibly know

Longitudinal designs follow the same children over time and can describe change within a child — the only design that can. Their prices are time, cost, attrition (the families who leave are not a random sample), and practice effects. **Cross-sectional** designs compare ages at one moment: cheap and fast, but they compare cohorts, not development; a difference between today's eight-year-olds and today's twelve-year-olds may be development or may be four years of history, schooling reform, or a pandemic. **Sequential** designs combine both and can separate age, cohort and time — the design behind the best modern claims, and the reason to read a "age effect" carefully.

Observation, naturalistic or structured, is the royal road of early development: the Strange Situation (week 6) is a structured observation, and your own practicum records will be naturalistic ones. Observation's bias is the observer: we see what our coding scheme permits, and children behave differently when watched — least of all when the watcher is a stranger with a clipboard.

Informant reports (parents, teachers) buy history and context and sell the present: parents are excellent historians of their child and poor calibrated instruments, and teacher reports are framed by class size and by what a teacher has been taught to worry about. **Child self-report** becomes possible in degrees: a four-year-old can report now, an eight-year-old can report yesterday, an adolescent can report a pattern — and each degree has a method that respects it.

Tasks and experiments isolate mechanism: the false-belief task (week 5), the conservation task (week 3), the scaffolded teaching session (week 4). Their bias is the strange adult and the strange room, which depress exactly the competences the task measures.

> **SAFEGUARDING AND DUTY TO REPORT**
> Research and observation with children carry obligations heavier than with adults: consent from the guardian **and** assent from the child in words the child owns; the child's right to stop, honoured the moment it is signalled, not the moment it is spoken; findings returned to the family in usable form, because a family that lent you their child is owed more than a publication; and special care where the observer is also the counsellor, because a child cannot decline the person who holds their secrets.

Lecture III — Norms, ranges and the ethics of the curve

Proposition 1. A norm is a description with a distribution, and the tails are children. Every milestone curve is built from children who walked at nine and children who walked at eighteen; the curve does not prefer either, and a practice that treats the median as a deadline converts ordinary variation into a queue of worried parents and unnecessary referrals.

Proposition 2. Norms carry their sample, and most samples are local. Height, language and even motor milestones shift with nutrition, opportunity and the practices of a culture; a chart built elsewhere describes strangers until it is validated here. The honest use of an imported norm names its population every time it is quoted, which is week 14's discipline arriving early.

Proposition 3. The range is the clinical object, not the point. What the counsellor needs is not "the age" but the width: the interval within which development is ordinary, the flags outside which enquiry begins, and the direction and slope of this child's own course through it. A single measurement locates a child on a curve; two measurements, spaced, locate the child on a course — and courses, not points, are what developmental science and developmental counselling both trade in.

> **PRACTICE NOTE**
> **Clinical implication.** When a parent quotes a milestone app, give them the range, the sample and the slope in three sentences, and write the child's own two measurements beside it. Anxiety falls where the distribution becomes visible.

Worked example 1.1 — Choosing a design, paying its price

Task. A college asks: "does attending our crèche from age two make children's language faster at four?" Propose a design, name what it can establish, and name the confound that will be blamed for whatever you find.

Solution. A cross-sectional comparison of crèche attendees and non-attendees at four confounds attendance with everything that predicts attendance: mothers who enrol a two-year-old differ in education, income, language exposure and anxiety from mothers who do not, and those differences, not the crèche, will explain the gap in either direction. A longitudinal study from age two, measuring language at two before attendance diverges, and again at three and four, establishes sequence and controls the starting point, but attrition will remove precisely the most chaotic households, inflating every estimate. The strongest feasible design is sequential: two intake cohorts followed with the same measures, which separates cohort from age. The confound that will be blamed regardless: family language environment, which is why any honest report measures conversational turns at home (week 7) and states its limits in the first paragraph, not the last.

> **SKILLS LABORATORY**
> **Practical 1 — two ways of seeing.** In pairs at the college observation nursery (or from a supplied recording): ten minutes of **narrative record** of one child — everything, in order, without interpretation — followed by ten minutes of **checklist record** of the same child using a supplied twelve-item list. Compare: what the narra-

tive caught that the checklist could not; what the checklist caught that the narrative buried; and which record you would defend in a case conference. Write 300 words on the difference and keep both records: they return in week 14.

ASSESSMENT

Assessed task. A reading response of 600 words: choose one claim in this week's core text about what is universal in early development, state where and in whom it was established, and argue whether it should travel to the children you will counsel.

Criterion	Weight
The claim stated precisely, with its evidence base	30%
Universality versus variation argued, not asserted	40%
Consequence for practice stated	30%

Formative. Submit your two observation records with the 300-word comparison.

UNIT SYNTHESIS

Synthesis. Developmental science answers four questions — what develops, by what process, with what universality, to what purpose — using methods whose biases must be named before their findings are believed; and it locates every child in nested contexts and in time, because development has an address.

READING (APA 7TH)

- Berk, L. E. (2013). *Child development* (9th ed.). Pearson. — **core text**; the methods chapter.
- Black, M. M., Walker, S. P., Fernald, L. C. H., Andersen, C. T., DiGirolamo, A. M., Lu, C., McCoy, D., Fink, G., Shawar, Y. R., Shiffman, J., Devercelli, A. E., Trudel, M. T., Wodon, Q., & Grantham-McGregor, S. (2017). Early childhood development coming of age: Science through the life course. *The Lancet, 389*(10064), 77–90.
- Bronfenbrenner, U. (1979). *The ecology of human development: Experiments by nature and design*. Harvard University Press.
- Santrock, J. W. (2019). *Child development* (15th ed.). McGraw-Hill Education. — **core text**; read its life-span perspective chapter against Berk's.
- Shonkoff, J. P., & Phillips, D. A. (Eds.). (2000). *From neurons to neighborhoods: The science of early childhood development*. National Academy Press.
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Seminar questions

1. A norm is a description with a distribution. What harm follows when it is taught as a deadline, and to whom?
2. Which of this week's methods would you trust least for a child like Ife, and why?
3. Name one exosystem decision that reached your own childhood before you could name it. What did it change?
4. Why is assent from a child ethically distinct from consent by a guardian? Give the moment in a session where the distinction bites.
5. If a pattern has only ever been found in schooled, urban, Western children, what is its status? A hypothesis, a fact, or a local custom?

Next week

Week 2 goes before birth: genes, teratogens and the windows of sensitivity, epigenetics, and the early adversity that sets a trajectory — with the counselling ethics of never blaming the mother.

2 LECTURE UNIT 2 Teaching week 2 4 notional hours

Foundations Before Birth: Genes, Teratogens and Early Adversity

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Outline** the germinal, embryonic and fetal periods and the principle that teratogenic risk is a function of timing, dose, genetic susceptibility and combination.
2. **Explain** probabilistic epigenesis: how experience regulates gene expression and why neither "in the genes" nor "in the upbringing" is a complete sentence.
3. **Describe** the developmental consequences of undernutrition, malaria in pregnancy, prematurity and early adversity, and the counsel each implies.
4. **Compute** corrected age for a child born preterm and use it to set fair expectations to at least two years.
5. **Counsel** without blame: hold a teratogen or adversity history with a mother without converting it into her guilt.

Where this week sits

Development begins before anyone meets the child. This week is the counsellor's version of embryology: not a memorised timetable but a set of principles about timing and susceptibility, an honest account of what genes do and do not do,

and the public-health realities — undernutrition, malaria, prematurity, adversity — that shape the children this programme will serve. It ends where counselling always begins: with a parent who is afraid it was something they did.

Key concepts

TERATOGEN
An agent that disrupts development before birth. Its effect depends on **timing** (the sensitivity window of the organ then forming), **dose**, the **genotype** of mother and fetus, and **combination** with other agents. In the room: the reason "was it the tablet I took?" rarely has a one-word answer, and the reason the answer must still be sought.

PROBABILISTIC EPIGENESIS
Gottlieb's account: genes are one participant in a developmental system, expressed or not expressed depending on internal and external experience, which they in turn shape. Development is probabilistic, not preformed. In the room: the sentence that ends both genetic fatalism and parental omnipotence.

EXPERIENCE-EXPECTANT AND EXPERIENCE-DEPENDENT
Two kinds of learning the brain is built for: circuits that **expect** species-typical input in a window (patterned light for vision, language sound for phonology, responsive care for stress regulation) and circuits that record **whatever** a particular child meets (this language, this compound, this grandmother's songs). In the room: the distinction between what every child needs and what makes this child theirs.

CORRECTED AGE
For a child born preterm, age counted from the due date rather than the birth date; the fair yardstick for developmental expectations roughly to two years, and beyond that for the most preterm. In the room: the arithmetic that keeps a twenty-month-old ex-premie out of a disability register.

Lecture I — Three periods and the logic of the window

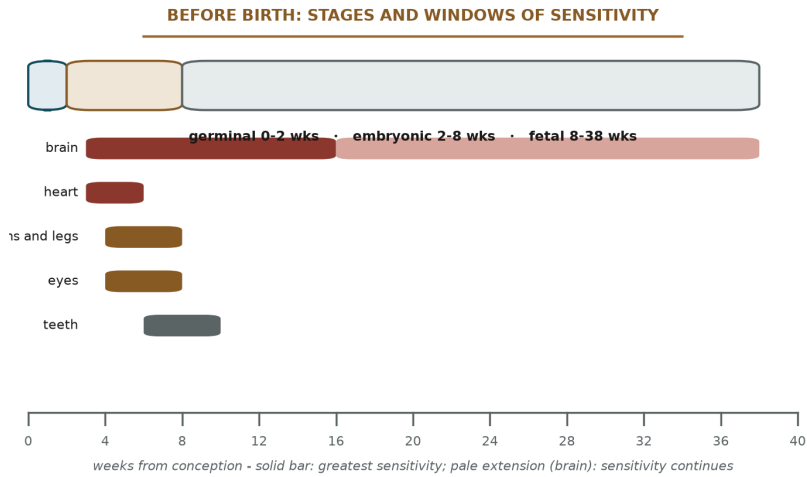


Figure 2.1 Prenatal periods and the sensitivity windows of five organ systems across weeks from conception; the brain's window opens early and stays open, which is why adversity and toxins reach it longest. *Source:* prepared for this edition.

Proposition 1. Timing is the first principle. The embryonic weeks (roughly two to eight from conception) are when organs lay themselves down and when teratogens do their greatest structural damage; the fetal period is when systems grow and refine, and when agents act on function and growth rather than structure. Figure 2.1 shows the windows: the heart's is brief and early, the brain's opens early and never fully closes — which is why the brain is the organ most reachable by later adversity, and the reason week 10's ACEs literature exists.

Proposition 2. Dose, genotype and combination modify everything. Alcohol has no known safe threshold in pregnancy and produces a spectrum — growth, face, and central nervous system, in combinations and degrees — rather than an all-or-none syndrome; valproate in pregnancy raises the risk of neural-tube defects and later neurodevelopmental difficulty; malaria in pregnancy and maternal undernutrition act through the placenta on growth and on the brain's fuel. And the same exposure in two pregnancies yields two outcomes, because susceptibility is partly genetic: the honest clinical sentence is "this raises risk; it does not write the sentence".

Proposition 3. Genes are participants, not authors. Probabilistic epigenesis replaces the blueprint metaphor: gene expression is regulated by experience — by care, by nutrition, by stress hormones — from the womb outward. The practical consequence is symmetry: a genetic vulnerability can be softened by environment (week 6's goodness of fit), and an environmental risk can be buffered

by relationship (week 10's resilience literature). Neither half of nature–nurture is a complete sentence.

> **PRACTICE NOTE**
> **Clinical implication.** Take a prenatal history on every child, once, calmly, as part of the ordinary map: birth weight and weeks, complications, maternal health and medication, alcohol and other substances, and the household's circumstances in that year. Record it as history, not verdict; and record what is unknown as unknown.

Lecture II — Adversity before and around birth, in our context

Undernutrition and stunting. Maternal undernutrition, anaemia and adolescent pregnancy constrain fetal growth; after birth, stunting in the first two years travels with delayed cognitive and motor development at group level, and the mechanism is transactional: a child with less energy explores less, is responded to less, and receives less of the experience-dependent input the brain expects. The counsel is not despair but sequence: nutrition and responsive care together, because either alone underdelivers (the Lancet early-childhood series).

Malaria, infection and inflammation. Malaria in pregnancy contributes to low birth weight; severe early infection and its inflammation reach the developing brain. Public-health measures — nets, intermittent preventive treatment in pregnancy, prompt care — are developmental interventions, and the counsellor who knows this advocates differently.

Prematurity. The preterm infant's brain completes its most vulnerable growth outside the womb, in a world of light, sound and handling it did not expect. Outcomes range from entirely typical to significant disability, and the predictors are weeks at birth, complications, and — again — the environment after discharge. Corrected age is the fair yardstick: a child born at thirty-two weeks is, at four months of age, developmentally expected to behave like a two-month-old, and the expectation should be corrected roughly to two years, longer for the most preterm.

Early adversity and care. Institutional care without a stable responsive adult alters stress regulation, attachment and cognitive trajectories, and the earlier the stable care arrives the better the recovery — the strongest natural experiment in the whole of developmental science, and the argument behind every foster and adoption standard. Family poverty acts not mainly through want of things but through want of dependable, unhurried, responsive care: stress taxes the caregiver, and the caregiver is the child's environment.

> **SAFEGUARDING AND DUTY TO REPORT**
> A prenatal history includes matters that are, in some places, crimes or sins: alcohol use, undisclosed medication, an unwanted pregnancy. The counsellor's frame is protective, not prosecutorial: information given in a session is used to help the child and to support the mother, and where a current pregnancy is at risk the duty is to the unborn child's care, discharged through support and referral, never through blame. Blame is not only unkind; it is ineffective — it is the single most reliable way to ensure the next mother tells you nothing.

Lecture III — Counselling uncertainty: probability, blame and the useful sentence

Proposition 1. Risk is a population statement wearing a personal question. "Does this exposure mean my child will…?" asks for a certainty that probabilistic epigenesis cannot supply; the honest answer converts the personal question into its population form — what the evidence says about groups like this pregnancy — and then returns to the individual with the only sentence that helps: what we watch, what we do, and by when.

Proposition 2. Absolute and relative risk are different medicines. "Doubles the risk" of a very small base is still small; "no safe threshold" of a common exposure is a public-health sentence, not a verdict on one mother. The counsellor who cannot tell the two apart will either frighten or falsely reassure, and both errors are breaches of the same duty.

Proposition 3. "We cannot know" is a complete clinical sentence. Where attribution is impossible, saying so plainly — and refusing to dress ignorance as probability — protects the relationship and the record; the vacuum left by a refused false certainty is filled by the workable plan, which is where the family's energy belongs anyway.

> **SAFEGUARDING AND DUTY TO REPORT**
> Uncertainty counselling is safeguarding: a parent who receives a false certainty of blame may hide the next pregnancy, the next medication, the next truth. The non-blaming sentence is not kindness alone; it is the mechanism by which the next disclosure arrives in time to matter.

Worked example 2.1 — The question every counsellor dreads

Task. A mother of a four-year-old with language and attention difficulties asks, crying: "Was it the malaria tablets in my first pregnancy? It was my fault." Respond in the register this volume requires: accurate about mechanism, honest about uncertainty, and protective of the relationship.

Solution. Three moves, in order. First, the relationship: "You have carried that question for four years. Thank you for trusting me with it." Second, the mecha-

nism, honestly: intermittent preventive treatment in pregnancy is given **because** malaria itself damages the pregnancy; the medicines used in national programmes are chosen for safety in pregnancy, and no one can attribute an individual child's difficulties to a tablet taken years ago — the honest answer to "was it the tablets?" is "we cannot know, and the evidence does not point that way". Third, the turn to the usable: what is known is this child, now — her language sample (week 7), her attention in play (week 5), the interactions that will move her from here (weeks 4 and 6). The sentence that closes: "Whatever began it, what helps it now is in this room, and most of it is you." Blame is refused not by reassurance but by redirection to mechanism and to work.

Worked example 2.2 — Corrected age, worked

Task. Tobi was born at thirty-two weeks' gestation and is now twenty months old by the calendar. His nursery compares him with the twenty-month group and proposes a "developmental delay" note. Compute and counsel.

Solution. Thirty-two weeks is eight weeks early; corrected age is twenty minus eight, or twelve months. Against twelve-month expectations — first words emerging, pulling to stand, joint attention well established — Tobi's reported skills (walking with a held hand, four words, pointing) are within the ordinary range of his corrected age, and the "delay" is an arithmetic artefact. The counsel to the nursery: record corrected age beside chronological age on every review to at least two years; compare him with twelve-month expectations; and watch, because ex-preterm children remain a group worth monitoring for attention, learning and growth — monitoring, not labelling. The counsel to the parents: the number that matters this year is not the calendar but the due date he never got.

SKILLS LABORATORY

Practical 2 — the non-blaming history. In threes: mother, counsellor, observer. The counsellor takes a full prenatal and birth history from a "mother" who carries one hidden fear (supplied on a card: alcohol, a tablet, a fall, a wished-for termination). Requirements: ask every necessary question including the feared one; use the three moves of worked example 2.1 when the fear surfaces; end with the turn to the usable. The observer records one sentence: the moment blame was refused or the moment it was accepted. Rotate twice.

ASSESSMENT

Assessed task. A teratogen and adversity case analysis of 900 words: for a supplied case (preterm birth, maternal anaemia, an unverified exposure fear), state the timing logic, the modifiable factors, the corrected-age computation, and the counselling sequence with the caregiver.

Criterion	Weight
Timing, dose, genotype and combination correctly applied	25%
Corrected age computed and used for expectations	20%
Adversity framed transactionally, not fatally	25%
Counselling sequence non-blaming and specific	30%

Formative. Write the one sentence you would say to a mother who believes her child's difficulty is her punishment.

UNIT SYNTHESIS

Synthesis. Before birth, timing governs risk and experience governs expression: teratogens act in windows, genes act as participants in a probabilistic system, and adversity acts mostly through the caregiver — so the counsellor takes a calm prenatal history, corrects the age of the preterm, and refuses blame while turning to what can now be done.

READING (APA 7TH)

- Berk, L. E. (2013). *Child development* (9th ed.). Pearson. — **core text**; the prenatal chapter.
- Gottlieb, G. (2007). Probabilistic epigenesis. *Developmental Science, 10*(1), 1–11.
- Grantham-McGregor, S., Cheung, Y. B., Cueto, S., Glewwe, P., Richter, L., & Strupp, B. (2007). Developmental potential in the first 5 years for children in developing countries. *The Lancet, 369*(9557), 60–70.
- Rutter, M. (2006). *Genes and behavior: Nature–nurture interplay explained*. Blackwell.
- Shonkoff, J. P., & Phillips, D. A. (Eds.). (2000). *From neurons to neighborhoods: The science of early childhood development*. National Academy Press. — **core text**; the prenatal and early adversity chapters.
- Walker, S. P., Wachs, T. D., Gardner, J. M., Lozoff, B., Wasserman, G. A., Pollitt, E., Carter, J. A., & International Child Development Steering Group. (2007). Child development: Risk factors for adverse outcomes in developing countries. *The Lancet, 369*(9556), 145–157.

Seminar questions

1. Why does the brain's sensitivity window make early adversity more consequential than adversity at nine — and what does "more consequential" not mean?
2. Corrected age protects a child from a label. What protects the same child from the label's opposite, the missed referral?

3. "Genes load the gun, environment pulls the trigger" is popular and wrong. Where exactly does the metaphor fail?
4. What in your own culture names a child's difficulty as the mother's sin, and how will you counsel inside that belief without endorsing it?
5. If probabilistic epigenesis is true, what does a prognosis actually claim?

Next week

Week 3 meets the most famous answer to "what develops": Piaget's constructing child, from the baby who finds the hidden rattle to the seven-year-old who finally sees that the water is the same.

3 LECTURE UNIT 3 Teaching week 3 4 notional hours

The Constructing Child: Piaget from Rattle to Reversibility

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Explain** Piaget's mechanism — schema, assimilation, accommodation, equilibration — and why it makes the child an active constructor rather than a recipient.
2. **Describe** the sensorimotor and preoperational achievements and limits: object permanence and the A-not-B error, symbolic and pretend play, egocentrism, centration, animism, conservation failure.
3. **Administer and interpret** two conservation tasks and one class-inclusion task, recording the child's reasoning rather than only the verdict.
4. **State** the modern critiques — competence versus performance, horizontal décalage, cultural and schooling effects, Siegler's overlapping waves — and what survives them.
5. **Use** the stage account in counselling: matching explanation, task and play to the child's current construction of the world.

Where this week sits

Piaget changed the question from "how much does the child know?" to "how does the child's knowing work?" His stages are now taught with their corrections attached, but the corrections only sharpen his central claim: children do not think less than adults, they think **differently**, coherently, by rules of their own. For the counsellor this is liberation: the child who says the sky is crying is not wrong, they are operating a system — and a counselling method that ignores the system will talk past the child all session.

Key concepts

EQUILIBRATION
Piaget's engine: the child assimilates experience into existing schemas until the mismatch forces accommodation, and the cycle of the two — equilibration — is development itself. In the room: why a child's "wrong" answer is data about their current system, and why contradiction, gently held, is the teaching moment.

OBJECT PERMANENCE
The discovery that things exist unseen, built across the sensorimotor years and visible in its failures: the A-not-B error, where the infant who found the toy at A reaches to A again after seeing it hidden at B — knowing and acting not yet reconciled. In the room: the infant separation game: out of sight is not yet fully out of existence, and by twelve months it very much is.

CENTRATION AND CONSERVATION
The preoperational child centres on one salient dimension — the height of the glass — and therefore judges the poured water to be more; conservation is the discovery that quantity survives transformation, and it arrives dimension by dimension, not all at once. In the room: the reason a five-year-old's grief at the "smaller" cake is logic, not manipulation.

HORIZONTAL DECALAGE
The unevenness within a stage: a child reasons at one level with familiar material and a level below with strange material, conserving number before volume and volume before weight. In the room: the reason stage labels should be used as adjectives, never as nouns, about a child.

Lecture I — The staircase and what each step can hold

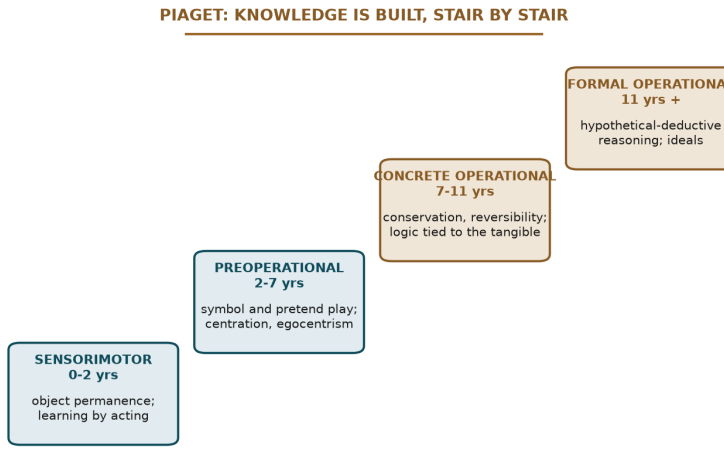


Figure 3.1 Piaget's four stages as a staircase with ages and signature achievements: object permanence, symbol and pretend play, conservation and reversibility, hypothetical-deductive reasoning; with the caveat of horizontal décalage. *Source:* prepared for this edition.

Proposition 1. The sensorimotor child knows by acting. From reflex to intention, from circular reactions to the first mental representation: the infant who pulls a cloth to reach a rattle upon it has invented means–ends; the infant who searches for the hidden rattle has invented existence. Pretend play appears at the top of this stair — the child who feeds a doll is operating a symbol, and symbol is the currency of every later therapy hour.

Proposition 2. The preoperational child represents but centres. Language explodes, play becomes sociodramatic, and the world is organised by perception and by the child's own position: egocentrism is not selfishness but the genuine difficulty of computing another viewpoint (the three-mountains task), and centration makes quantity hostage to appearance. Animism — the sun follows me because it wants to — is the same system applied to causality. These are coherent rules, and a counsellor who knows them hears a four-year-old's guilt after a parent's death ("I was angry at her") as developmental logic requiring careful release, not as pathology.

Proposition 3. Concrete operations are reversible operations. Conservation, class inclusion, seriation: the child can now run a transformation backwards in mind and hold two dimensions at once — but tied to the tangible and the present. Formal operations add the hypothetical: the adolescent who reasons about justice, about the future, about who they might be — and who can therefore be

counselled with hypothesis-testing (volume 1, week 6) in a way the eight-year-old cannot.

> **PRACTICE NOTE**
> **Clinical implication.** Match the method to the stair: with the preoperational child, play, draw and enact rather than talk about; externalise in objects (volume 1, week 11) because objects are their grammar; with the concrete-operational child, use tangible scales, cards and charts; reserve pure verbal insight work for formal operations, and even then check the stair under stress, because anxiety drops every child a step.

Lecture II — What the critiques changed, and what survived

Competence versus performance. Simplified tasks show abilities Piaget dated later: infants violate-of-expectation paradigms look like object knowledge in the first year; three-year-olds pass false-belief tasks in some cultures and versions years before the classic schedule (week 5). Piaget's stages are therefore best read as the ages at which competences become **reliable in standard tasks**, not the ages at which they appear.

Horizontal décalage and overlapping waves. Conservation arrives dimension by dimension; and Siegler showed that at any age a child holds several strategies at once, choosing among them by cost and success — development as changing mixtures, not replacing stages. The staircase survives as a map of dominant strategies, not as a lift with locked floors.

Culture and schooling. Conservation and class-inclusion performance shifts with schooling and with the practices of a community; some cultures' children pass certain tasks earlier and others later, and formal-operational reasoning is exercised where the culture exercises it. Piaget universalised the schoolchild; the correction is week 1′s fourth question, applied.

What survived. Constructivism; the active child; the sequence of sensorimotor achievements; the qualitative difference of preoperational thought; the clinical interview as a method — Piaget's greatest gift to counselling, the habit of asking "how did you know?" and listening to the reasoning rather than scoring the answer.

> **SAFEGUARDING AND DUTY TO REPORT**
> Stage knowledge cuts both ways in safeguarding: a child's concrete-operational logic means a nine-year-old may genuinely not generalise a safety rule to a new setting ("stranger" meant the man in the car, not the neighbour), so safety teaching must be rehearsed across settings rather than stated once; and a child's preoperational egocentrism means they may believe their anger caused a death or a divorce, so every bereavement and separation conversation with a child under seven includes the explicit, repeated release of that belief.

Lecture III — The clinical interview as method: listening for the child's rule

Proposition 1. Piaget's greatest gift to counselling is not the stages but the interview. The habit of asking "how did you know?" and treating the answer as data about a system rather than as right or wrong is directly transferable: the child who says "the sky is crying because it is sad" is stating a causal rule, and the rule is the door into their world.

Proposition 2. The counter-suggestion tests the system, not the obedience. Offering a gentle opposite ("but my friend says the tall glass has more") distinguishes a constructed judgement from a social guess; the child who holds their rule against a polite contradiction owns it, and the child who abandons it at the first breath of disagreement was answering the adult, not the question.

Proposition 3. Emotional statements carry rules too. "Nobody likes me" at six is often a centred judgement on one salient event, not a self-concept; the interview habit — "what made you say nobody? tell me the one who does" — locates the rule and its exception without arguing, and the exception is where the work begins.

> **PRACTICE NOTE**
> **Clinical implication.** In every session with a child under nine, ask one "how did you know?" question and record the answer verbatim. Ten such answers are a portrait of a mind that no rating scale can draw.

Worked example 3.1 — Conservation with Ife, recorded properly

Task. Ife (3 years 8 months) is given two identical beakers of equal juice and agrees they are "the same". One is poured into a tall thin glass. Record and interpret.

Solution. Ife points to the tall glass: "that one plenty." Asked how she knows: "look! e long." The record must keep her words and her reason — centration on height, justification by perception — and must also record the counter-suggestion test: when the juice is poured back and she again says "same", the interpretation is stable preoperational centration, not confusion or compliance. Two cautions for the record: first, the question "which has more?" can be heard by a young child as "which did I make you change?", so a third-party puppet who "thinks they're the same" is introduced and Ife's disagreement with the puppet noted — the difference between a logical judgement and a social guess. Second, the verdict is not "Ife fails conservation"; it is "Ife, at 3:8, centres on height with familiar material, justifies by perception, and is stable across transformations" — a description that will be comparable with the same tasks at five and at seven.

Worked example 3.2 — When the stage is the intervention

Task. Kunle (7) will not begin written work he might not finish perfectly; he tears the page. His teacher calls it pride. Design the counsellor's response using this week's theory.

Solution. Kunle is at the concrete-operational threshold: he can reverse operations in mind — which is exactly the trap, because he can now imagine the finished page and compare it with the present one, and the mismatch is intolerable. The intervention is staged, not verbal: (i) externalise the standard into an object — a "first-draft box" into which imperfect pages go by rule, making imperfection a location rather than a verdict; (ii) use reversibility as the medicine — show the torn page restored by tape and the page rewritten, and have Kunle order photographs of a page from bad to good to see that states are transformations, not identities; (iii) give the teacher the décalage sentence: Kunle's reasoning about numbers runs a step ahead of his reasoning about his own work, so praise his number logic openly and his drafts by rule, never by adjective. "Pride" dissolves into a developmental task: the discovery that a page, like water, remains a page through transformation.

SKILLS LABORATORY

Practical 3 — the clinical interview, Piaget's method. In pairs with a supplied child (or recording): administer number conservation, liquid conservation and a class-inclusion question ("In this box there are eight roses and two daisies; are there more roses or more flowers?"). Record verbatim the child's justifications. Then write 250 words: the child's dominant strategy, the evidence of décalage if any, and one sentence of what you would teach next and why. The mark is for the recording of reasoning, not the verdicts.

ASSESSMENT

Assessed task. A conservation demonstration log: your two recorded tasks with justifications, plus 500 words placing the child on the staircase with evidence, naming one critique that limits your inference and how you honoured it.

Criterion	Weight
Tasks administered cleanly, third-party control used	25%
Child's reasoning recorded verbatim and interpreted as strategy	35%
Stage placement argued with décalage in view	20%
Critique named and its limit honoured	20%

Formative. One paragraph: the preoperational sentence a child once said to you (or in this week's recording) that is logic, not error — and the rule it obeys.

UNIT SYNTHESIS

Synthesis. Piaget's child constructs the world by acting on it, through schemas that assimilate until they must accommodate; the staircase from rattle to reversibility remains the best map of dominant strategies, read with décalage, culture and overlapping waves in view — and its clinical gift is the interview that listens for the child's reasoning, because counselling that ignores the child's system talks past the child.

READING (APA 7TH)

- Berk, L. E. (2013). *Child development* (9th ed.). Pearson. — **core text**; the cognitive-development chapters.
- Piaget, J. (1952). *The origins of intelligence in children*. International Universities Press.
- Piaget, J., & Inhelder, B. (1969). *The psychology of the child*. Basic Books.
- Santrock, J. W. (2019). *Child development* (15th ed.). McGraw-Hill Education.
- Siegler, R. S. (1996). *Emerging minds: The process of change in children's development*. Oxford University Press.

Seminar questions

1. A child says the poured glass has more, then pours it back and says "same again". Is this inconsistency? What would consistency even mean in an overlapping-waves account?
2. Which of Piaget's claims would you defend most strongly today, and which would you retire? Give evidence for each choice.
3. Why does egocentrism make bereavement work with the under-sevens a matter of explicit sentence-making?
4. If formal operations are exercised only where a culture exercises them, what should a secondary curriculum owe every student?
5. Play is Piaget's symbol made visible. What does that make the playroom, epistemologically?

Next week

Week 4 crosses to the other great answer: Vygotsky's social mind, the zone of proximal development, and scaffolding — the theory that makes the teacher, the parent and the counsellor part of the child's cognition rather than spectators of it.

4 LECTURE UNIT 4 Teaching week 4 4 notional hours

The Social Mind: Vygotsky, the Zone and the Scaffold

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Contrast** Vygotsky's social origin of mind with Piaget's individual construction, stating what each explains that the other cannot.
2. **Define** the zone of proximal development and distinguish dynamic from static assessment, stating what each predicts.
3. **Demonstrate** scaffolding with its six moves — recruiting, reducing degrees of freedom, direction maintenance, marking critical features, frustration control, demonstration — and its withdrawal.
4. **Interpret** private speech developmentally, as a tool rather than a fault, and counsel a teacher or parent who hears it as one.
5. **Apply** the zone to emotional and social learning: the scaffolded regulation of a child who cannot yet regulate alone.

Where this week sits

Piaget watched the child alone with the world; Vygotsky watched the child with someone. The difference is not academic: it decides whether the counsellor sees a score or a trajectory, a deficit or an untaught zone. This week installs the most practically powerful idea in the volume — that what a child can do with help today, they can do alone tomorrow, and that help has a grammar — and carries it from homework to heartbreak.

Key concepts

ZONE OF PROXIMAL DEVELOPMENT

The distance between what the child does independently and what they do with a capable partner; the region where teaching lands. In the room: the reason a static test measures yesterday and a scaffolded session measures tomorrow — and the region every intervention should be aimed at.

SCAFFOLDING

The contingent support a partner provides inside the zone — recruiting interest, simplifying, holding the goal, marking what matters, controlling frustration, demonstrating — handed over step by step and withdrawn step by step (Wood, Bruner & Ross). In the room: the grammar of help; support that never fades is not scaffolding, it is substitution.

PRIVATE SPEECH
The child's speech to self: for Piaget, egocentric residue to be outgrown; for Vygotsky, thought in the making — social speech turned inward, audible at first, whispered later, silent finally. In the room: the child muttering through a hard task is not disturbing the class; they are regulating it.

DYNAMIC ASSESSMENT
Test, teach, retest: the measure of the zone rather than of the stock. Its score is the amount and kind of help that moved the child, and it predicts learning better than a static score in several populations, including children whose language or culture the static test does not speak. In the room: the antidote to the label that a single morning's testing can otherwise write on a child.

Lecture I — Mind begins between people

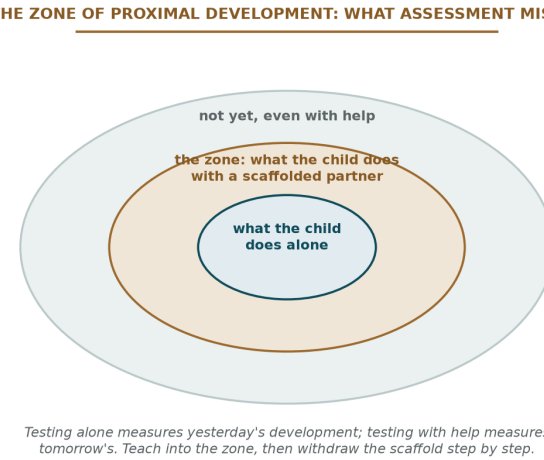


Figure 4.1 The zone of proximal development as three regions: what the child does alone, what the child does with a scaffolded partner, and what is not yet reachable even with help; with the counsel to test with help as well as alone. *Source:* prepared for this edition.

Proposition 1. Every function appears twice. First between people, on the social plane; then within the child, on the psychological plane. Attention is first jointly held ("look!"), memory is first stored in a parent's narrative ("remember when..."), regulation is first co-regulation (week 6). The child's inner tools are internalised conversations — which is why the quality of the conversations a child is given is the quality of the mind they build.

Proposition 2. Tools shape thought. Language is the master tool, but cultures hand children others: counting systems, scripts, calendars, proverbs, prayer.

A child's reasoning carries the marks of its tools; the counsellor who forgets this mistakes a tool difference for a capacity difference, which is week 1's universality question with consequences.

Proposition 3. The zone is where development happens, and it is invisible to static testing. A test administered alone scores the achieved; two children with the same achieved score can have entirely different zones, and it is the zone that predicts what the next term of teaching — or therapy — will produce. Dynamic assessment makes the zone visible by measuring the help: how much prompting, of what kind, before the child crosses.

Proposition 4. Scaffolding has a grammar and a morality. The grammar is contingency: help is given in inverse proportion to competence, marked at the critical features, and faded as competence arrives; the six moves of Wood and colleagues are its verbs. The morality is this: scaffolding aims at its own withdrawal. Help that persists after competence arrives is not kindness; it is the quiet theft of the child's next step.

> **PRACTICE NOTE**
> **Clinical implication.** In every session ask the zone question twice: what did this child do today only with me, and what did I do for them today that they could have done with less? The first answer is next week's target; the second is this week's confession.

Lecture II — Private speech, and the teacher who mishears it

Private speech peaks audibly between roughly three and seven, rises with task difficulty, and falls to whisper and then silence as it becomes inner speech. It predicts better task performance, not worse; children with attentional difficulties often use **more** of it, later, because their regulation work is harder. The teacher who hears muttering as disobedience is hearing a tool as a fault.

The counsel to the teacher has three sentences: what the muttering is (the child thinking aloud, exactly as the teacher once did); what it will do (fade by itself as the skill internalises, typically across the primary years); and what to do instead of stopping it (watch whether it rises when the task hardens — that is the child working, and the moment to offer a scaffold rather than a reprimand). For the child with a language difference, private speech may arrive in the home language first: a further reason never to correct it mid-task.

> **SAFEGUARDING AND DUTY TO REPORT**
> Dynamic assessment carries a safeguarding use: where a child's static scores are low and the culture or language of the test is not the child's, a dynamic session can be the difference between a misdiagnosis of intellectual disability and the discovery of

an untaught zone. Before any label of global delay is accepted, ask what the child did with help that they could not do alone; record both numbers; and insist that the report carry both.

Lecture III — Culture, tools and the classroom: what the scaffold is made of

Proposition 1. Scaffolds are built from cultural materials. The tools a community hands its children — proverbs, stories, songs, counting systems, prayer, the market's arithmetic — are the substance of contingent help; a scaffold built only from the school's materials reaches only the children the school already holds. The counsellor who scaffolds with the family's proverbs is not decorating the method but using its strongest local timber.

Proposition 2. Guided participation is the ordinary classroom's Vygotsky. Rogoff's apprenticeship in thinking describes how children learn inside shared activity with gradual transfer of responsibility — the market stall, the kitchen, the farm, the choir — and much of what a child can do was learned there rather than at a desk. Assessment that sees only the desk under-measures the child.

Proposition 3. The classroom is a zone, and it can be designed. Seating that pairs the assisted with the assistable, tasks whose roles rotate, thinking aloud modelled by the teacher, and private speech tolerated rather than policed: each turns the room into a scaffold without adding a single adult. The counsellor's consultation to a school is most powerful when it designs these ordinary arrangements.

PRACTICE NOTE

Clinical implication. Ask every family one tools question: what does this child already do well with someone, at home, in the market, at church? The answer is the zone's address and the scaffold's material, in the family's own possession.

Worked example 4.1 — Scaffolding Kunle's homework, move by move

Task. Kunle (7) must write three sentences about his weekend. He stalls at the blank page, jaw set. Script the scaffold with the six moves, then script its fading next week.

Solution. *Recruiting:* "Tell me the best thing first — the football or the rain?" (interest before task). *Reducing degrees of freedom:* the page is folded into three boxes; one sentence per box; "we only need the football one now." *Direction maintenance:* after the first sentence, "two boxes left; the rain one is next" — the goal held visibly. *Marking critical features:* "your sentence has who and what; listen: 'Kunle played football.' Who? What? Yours needs the same two." *Frustration*

control: "the first sentence is always the worst; everybody's is." *Demonstration:* the counsellor writes one sentence about their own weekend aloud, thinking visibly, including one crossed-out word. Kunle writes two sentences with the fold and one without. *Fading, next week:* the fold becomes a line, the line becomes a page; the who–what prompt becomes a raised eyebrow; the demonstration becomes "show me how you'd start". By week four the scaffold is a question — "what's the best thing?" — and then it is nothing, which was always the plan. The record notes the zone: three sentences with structural support, one alone; next week's target is two alone.

Worked example 4.2 — The zone of feeling

Task. Ife (3:8) melts at nursery drop-off. Using this week's theory, design the regulation scaffold and name what would make it substitution instead.

Solution. Regulation at three is co-regulation (week 6): the zone runs from "dysregulated alone" to "settled with a familiar adult who lends a slow voice and a predictable sequence". The scaffold: a goodbye ritual of exactly three steps (wrapper hug, window wave, cup on shelf), rehearsed at home in play first (the social plane before the psychological); the key worker receives her with the ritual's words, not new ones; the settling time is recorded daily — twenty minutes, then twelve, then six — because the fading is the measure. The withdrawal: the ritual shortens by one step when settling holds under five minutes; then the key worker's presence recedes to the doorway; then the wave is from the gate. Substitution would be: the key worker holding Ife all morning, every day, indefinitely — settling without transfer, comfort without competence, the zone never crossed. The difference between scaffolding and substitution is the record of fading: if the steps are not shrinking month by month, the adult is not a scaffold, they are a crutch the service has grown fond of.

> **SKILLS LABORATORY**
>
> **Practical 4 — dynamic versus static.** In pairs with a supplied child or recording: administer a static task (e.g., a four-piece pattern completion) and score it; then teach the same task's principle for five minutes using the six scaffold moves and re-administer; record the static score, the amount and kind of help required, and the post-teach score. Write 200 words: which number predicts next month's learning, and what the help profile says that neither score says. Debrief in plenary: whose children are most mis-served by static-only testing in our setting?

ASSESSMENT

Assessed task. A scaffolding exercise: a recorded ten-minute scaffolded teaching session with a child on a task of your choice, annotated move by move, with a written fading plan for the following three sessions (500 words).

Criterion	Weight
All six moves present and contingent (help inversely matched to competence)	35%
Zone identified: achieved level, assisted level, next target	25%
Fading planned concretely across three sessions	25%
Private speech respected or appropriately harnessed	15%

Formative. Write the three-sentence counsel to a teacher who calls a muttering child disobedient.

UNIT SYNTHESIS

Synthesis. Vygotsky relocates mind into relationship: functions appear first between people, tools shape thought, the zone of proximal development is where teaching lands, scaffolding is the grammar of help with withdrawal as its purpose, and private speech is the sound of a child building regulation — so assessment must measure the help as well as the stock, and every intervention aims at the day it is no longer needed.

READING (APA 7TH)

- Berk, L. E. (2013). *Child development* (9th ed.). Pearson. — **core text**; the Vygotsky chapter, read against its Piaget chapter.
- Rogoff, B. (1990). *Apprenticeship in thinking: Cognitive development in social context*. Oxford University Press.
- Santrock, J. W. (2019). *Child development* (15th ed.). McGraw-Hill Education.
- Vygotsky, L. S. (1978). *Mind in society: The development of higher psychological processes*. Harvard University Press.
- Wood, D., Bruner, J. S., & Ross, G. (1976). The role of tutoring in problem solving. *Journal of Child Psychology and Psychiatry, 17*(2), 89–100.

Seminar questions

1. Two children score identically on a static test; their zones differ by half a year. What does the school owe each, and how would it know?
2. Where is the line between scaffolding and doing the task for the child? Give an observable test, not a feeling.

3. Private speech in the home language: what does a school lose by banning it, in Vygotskian terms?
4. Is co-regulation scaffolding? Use the six moves to argue yes or no.
5. Vygotsky makes the counsellor part of the child's cognition. What does that do to the idea of a "neutral" therapist?

Next week

Week 5 opens the mind as a working system: attention, working memory and the discovery that other people have minds of their own — executive function and theory of mind, with the false-belief task administered and argued over.

5 LECTURE UNIT 5 Teaching week 5 4 notional hours

Processing and Executive Function: The Working Mind and Other Minds

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Outline** the information-processing account: attention, working memory, processing speed and strategy development across childhood.
2. **Describe** the three components of executive function and their developmental trajectory, and state why EF is a better school-readiness predictor than many expect.
3. **Administer and score** a standard false-belief task, and interpret a pass and a fail without over- or under-reading either.
4. **Explain** theory of mind's consequences for counselling: perspective-taking, understanding of lie and secret, and the credibility of a child's testimony.
5. **Recognise** EF demands in ordinary classroom failure, and design one accommodation that lowers demand without lowering expectation.

Where this week sits

Two revolutions meet in this week. The information-processing tradition treats the mind as a working system with bottlenecks — and much of what adults call disobedience is a bottleneck. The theory-of-mind tradition discovers that at around four to five the child grasps that minds **represent** rather than copy the world — the single cognitive event beneath every later empathy, every lie detected, and every therapeutic conversation about what someone else was thinking. Both are measurable, both vary with language and context, and both are routinely mistaken for character.

Key concepts

WORKING MEMORY
Holding and using information in mind over seconds; capacity grows steadily through childhood and is severely limited at any age — which is why a three-step instruction loses its third step somewhere in a six-year-old's hallway. In the room: the cheapest, most missed accommodation in education and in therapy.

EXECUTIVE FUNCTION
The coordinating system: inhibitory control (stopping the prepotent response), working memory, and cognitive flexibility (shifting set). Develops steeply across the preschool and primary years, vulnerable to stress, sleep loss and adversity — and trainable in the ordinary sense that use and scaffolding train it. In the room: the dimension on which "could if he tried" is usually false.

FALSE-BELIEF TASK
The test of representational mind: the child who understands that Sally will look where she **believes** the marble is, not where it is, has grasped that minds can be wrong — the foundation of deception detection, of empathy as cognitive act, and of understanding that the counsellor has a mind too. In the room: the four-to-five watershed, shifted months by language, conversation and siblings.

METACOGNITION
Thinking about one's own thinking: knowing what one knows, monitoring comprehension, choosing a strategy. Emerges visibly in the primary years and is the engine of independent learning — and of the reflective work therapy later asks for. In the room: the reason "think about why you did that" is an impossible instruction at five and a fair one at eleven.

Lecture I — The working system and its bottlenecks

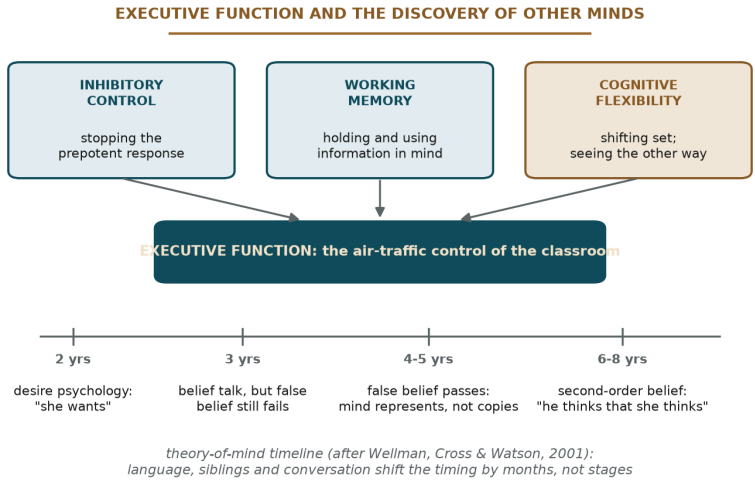


Figure 5.1 The three components of executive function converging on the coordinating system, above a theory-of-mind timeline from desire psychology at two to second-order belief in the primary years; language and conversation shift the timing. After Diamond (2013) and Wellman, Cross and Watson (2001). *Source:* prepared for this edition.

Proposition 1. Much of childhood "misbehaviour" is capacity, not choice. The child who answers before the question ends (inhibition), who loses the third step (working memory), who cannot switch from play to pack-up (shifting) is displaying the ordinary limits of a developing system. The diagnostic habit this week installs: before any moral reading of a failure, ask which component the task demanded and whether the child's system, today, under this stress, has it.

Proposition 2. EF is developmentally honest and socially unequal. EF develops steeply between three and seven and into adolescence; it is depressed by chronic stress, poor sleep and adversity — the mechanisms behind week 10's adversity findings — and it predicts early literacy and numeracy in several cohorts beyond general ability (Diamond, 2013). A classroom that knows this teaches EF in the grain of the day: routines that externalise memory (lists, pictures, songs), games that train stopping and shifting, and transitions announced before they happen.

Proposition 3. Strategy development is the visible face of processing change. Young children do not spontaneously rehearse, organise or elaborate; they must be taught the strategy and then taught **when** to use it — utilisation deficits are the rule. Metacognition follows: the eight-year-old who knows they did not understand the paragraph is operating a monitor the five-year-old lacks.

PRACTICE NOTE

Clinical implication. Audit your own instructions this week: count the steps in each thing you ask a child to do. Then halve them, or write them down, or sing them. The accommodation costs nothing and rescues more children than any referral.

Lecture II — The discovery of other minds

Before three, children operate a desire psychology: people act to get what they want. Belief enters slowly; at three, children grasp that minds can be about things, but a mind that is **wrong** still defeats them — in the standard false-belief task they send Sally to the true location. Between four and five, across cultures and tasks, the majority cross: minds represent, representations can be false, and action follows the representation (Wellman, Cross & Watson, 2001). Language, conversations about minds, and siblings shift the timing by months; deaf children of hearing parents, deprived of that conversation, cross late — the cleanest evidence that the discovery is made in dialogue.

Consequences for the counselling room, in four. *Perspective-taking:* before the watershed, "how do you think she felt?" asks a question the child's system cannot yet compute; use enactment and objects instead. *Lie and secret:* a child who understands false belief understands that a speaker can plant a false mind — the cognitive basis of lying, of being lied to, and of keeping a secret someone planted; safeguarding interviews must therefore date themselves against this competence. *Testimony:* a pre-representational child's account is not less honest but differently suggestible — they answer from the world, not from a monitored representation of it, so leading questions rewrite rather than retrieve. *Therapy itself:* every interpretation you offer is a claim about the child's mind held in your mind; the child who cannot yet hold minds-in-minds experiences interpretation as fact or as nonsense, never as hypothesis — one more reason volume 1′s work with the young is play and enactment, not insight.

SAFEGUARDING AND DUTY TO REPORT

Where a child's disclosure is evidence, this week's science is law-adjacent: a child below the representational watershed is highly answerable to leading questions, not because they lie but because their memory is reconstructive and social. Practitioners do not interview such children investigatively; they record what the child says spontaneously, in the child's words, and refer to trained forensic interviewers. The counsellor's room stays the child's room.

Lecture III — Stress, sleep and the working mind: executive function as state as well as trait

Proposition 1. EF is a state before it is a trait. The same child inhibits, holds and shifts markedly worse when sleep-deprived, hungry, frightened or freshly stressed; chronic adversity taxes the coordinating system day after day, which is one route by which week 10′s adversities become week 5′s "misbehaviour". The clinical rule: never assess the stock on a bad morning, and never moralise a Monday.

Proposition 2. Stress reaches EF through physiology and through attention. Threat physiology prioritises vigilance over working memory — adaptive on the savannah, costly in a classroom — and a child scanning for danger has no spare capacity for the third step of the instruction. This is why trauma-exposed children present as inattentive long before they present as distressed, and why the differential of week 11 must always include "what is this child watching for?"

Proposition 3. Use builds the system, and play is the gym. Games with stopping and shifting rules, pretence with role rules, songs with sequence, and the ordinary scaffolds of routine (lists, pictures, songs for transitions) exercise the components inside relationships rather than drills. The evidence for transfer from computerised training is modest and contested; the evidence for routine, sleep, exercise and stress reduction is boring and strong. Counsel accordingly: protect the night, feed the morning, shorten the instruction, and let the games do the training.

> **PRACTICE NOTE**
> **Clinical implication.** Before any EF conclusion, ask three state questions: how did this child sleep, eat and feel safe today? A formulation that omits the state is assessing the weather and calling it climate.

Worked example 5.1 — False belief with Ife: protocol, score, caution

Task. Administer a Sally–Anne style task to Ife (3:8) and write the record.

Solution. Protocol: enact with two dolls and a visible transfer; control questions first (where is the marble really? where was it at first?) — a child who fails a control question has failed comprehension, not theory of mind, and the trial is void. Test question: where will Sally look? Ife answers the true location (the basket Anne moved it to). Record: fails false belief with controls passed; desire vocabulary present ("Sally want her ball"); belief talk absent. Interpretation, carefully: this is the expected profile at 3:8 — not a deficit, not a prediction of empathy failure, and not stable: re-test at four-and-a-half with conversation about minds in between, because the watershed is near and language-rich interaction moves it. Caution written into the record: the task was administered in Ife's stronger

language; a child tested in a weaker language fails tasks their mind has already passed, which is an assessment error with a child's name on it.

Worked example 5.2 — The classroom failure that is a bottleneck

Task. Kunle (7) "refuses" group work: he dominates or withdraws, and his written instructions lose their endings. Analyse by component and design one accommodation per failure.

Solution. Group work demands simultaneous inhibition (wait your turn), working memory (hold the group's plan while holding your part), and shifting (move between speaker roles) — the full EF triad under social load, which for a seven-year-old with a quiet, slow-to-shift profile is the hardest room in the school. Dominance is inhibition failure under load; withdrawal is the child's own sensible rationing. Accommodations that lower demand without lowering expectation: roles made external and visible (a card that says RECORDER, turned over when the role turns), the group plan written in three boxes rather than held in air, and a two-minute warning before role change so shifting is prepared rather than sprung. For the written instructions: numbered steps on the board remain all lesson — the accommodation is environmental memory, and the expectation (follow all steps) is untouched. Review in four weeks: if dominance and withdrawal persist **with** demand lowered, the question changes from capacity to anxiety or to the ADHD dimension of week 11 — and the record of what was tried is what makes that referral credible.

SKILLS LABORATORY

Practical 5 — the EF audit. Observe (live or recorded) one classroom or nursery session for twenty minutes with a three-column sheet: demand on inhibition / on working memory / on shifting. Record each demand event and each child failure that matches it. Write 250 words: the single demand that produced most failures, and one environmental accommodation for it. Bring the sheet to week 14, where it becomes part of your measurement portfolio.

ASSESSMENT

Assessed task. A false-belief critique of 800 words: administer or observe the task, then critique one popular over-reading (e.g. "fails false belief, therefore cannot empathise" or "passes, therefore can be interviewed investigatively"), using this week's propositions and one empirical caution (language, controls, or timing).

Criterion	Weight
Task protocol correct, including control questions	30%
Over-reading identified and dismantled with mechanism	40%

Criterion	Weight
Empirical caution applied concretely	30%

Formative. One paragraph: an instruction you gave a child this month that their working memory could not hold. Rewrite it.

UNIT SYNTHESIS

Synthesis. The mind works with bottlenecks and with a coordinating system that develops under stress and scaffolding alike; and around four to five the child discovers that minds represent and can be wrong — the watershed beneath empathy, deception, testimony and therapy itself — so the practitioner audits demands before morals, dates every interview against the watershed, and tests in the child's language or not at all.

READING (APA 7TH)

- Berk, L. E. (2013). *Child development* (9th ed.). Pearson. — **core text**; information processing and theory of mind.
- Diamond, A. (2013). Executive functions. *Annual Review of Psychology, 64*, 135–168.
- Flavell, J. H. (1979). Metacognition and cognitive monitoring: A new area of cognitive–developmental inquiry. *American Psychologist, 34*(10), 906–911.
- Gathercole, S. E., & Alloway, T. P. (2008). *Working memory and learning: A practical guide for teachers*. SAGE.
- Wellman, H. M., Cross, D., & Watson, J. (2001). Meta-analysis of theory-of-mind development: The truth about false belief. *Child Development, 72*(3), 655–684.

Seminar questions

1. "He could do it if he tried." Under an EF account, what is this sentence usually hiding, and what would the honest version say?
2. Why does a child who fails false belief answer leading questions differently — and what follows for a safeguarding conversation?
3. A child tested in their weaker language fails. Whose error is recorded on the form, and how should the form say so?
4. Is metacognition trainable, or does it arrive? Use the utilisation-deficit evidence to argue.
5. Which of your own professional routines is an external working-memory device you have stopped noticing?

Next week

Week 6 turns from the cold mind to the warm one: temperament, attachment and the development of emotion regulation — and goodness of fit, the idea that rescues "difficult" children from their diagnoses.

6 LECTURE UNIT 6 Teaching week 6 4 notional hours

Temperament, Attachment and the Regulation of Emotion

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Describe** temperament and its classic dimensions and styles, and state what temperament is not (not destiny, not diagnosis, not character).
2. **Apply** goodness of fit to one case: locating the mismatch between a child's temperament and the demands of their context, and redesigning the context.
3. **Trace** the development of emotion regulation from co-regulation to self-regulation, naming the caregiver behaviours that carry it.
4. **Summarise** attachment's normative course — signals, secure base, internal working models — as developmental science rather than as verdict on mothers.
5. **Counsel** a parent of a "difficult" child: reframe, re-sequence, and protect the relationship that is the child's actual intervention.

Where this week sits

Volume 1 used attachment as clinical theory; this week uses it as development. The question changes from "what category is this dyad?" to "how does a human being acquire the regulation of their own emotion, and what does the environment owe that process?" Between temperament and attachment sits the most humane idea in the volume: goodness of fit — the discovery that a child's difficulty is often a mismatch, and that mismatches are redesignable.

Key concepts

TEMPERAMENT

Constitutionally based individual differences in reactivity and self-regulation, observable early and relatively stable: activity level, rhythmicity, approach–withdrawal, adaptability, threshold, intensity, mood, distractibility, persistence. Classically grouped as easy, difficult and slow-to-warm — descriptions of style, not worth. In the room: the first draft of the person, written in ink that environment can edit but not erase.

GOODNESS OF FIT
Thomas and Chess's principle: outcome depends on the match between the child's temperament and the demands and expectations of their context. The same reactivity is "difficult" in a hurried home and "spirited" in a patient one. In the room: the intervention target that is neither the child nor the parent but the fit between them.

CO-REGULATION
The borrowed regulation of early life: the caregiver's voice, pace, hold and face doing for the child's arousal what the child's own system cannot yet do, until repeated thousands of times it becomes the child's inner function. In the room: why "calm down" is not a strategy at three, and why the adult's own regulation is the child's therapy.

INTERNAL WORKING MODEL
The accumulated expectation of what happens when I signal: built from thousands of episodes of care, carried into new relationships, and revisable — slowly — by later ones. In the room: the mechanism by which early care reaches the counselling hour, and the reason the counselling hour can, in turn, reach forward.

Lecture I — Temperament: the first draft

Proposition 1. Temperament is real, early and editable. Reactivity and regulation differ from the first weeks, are partly heritable, and predict some later outcomes at group level — the inhibited infant is somewhat more likely to be the cautious seven-year-old. But prediction is probabilistic and the editor is environment: reactivity becomes vigour, leadership or volatility depending on what meets it.

Proposition 2. The styles are descriptions of fit-risk, not of children. The "difficult" cluster — intense, withdrawing, irregular — is difficult **to a particular kind of context**: one that values quick settling, regular schedules and low volume. In another context the same child is the one who notices everything and cannot be rushed into danger. The classic New York Longitudinal Study's own conclusion was not a typology of children but a theory of match.

Proposition 3. Goodness of fit makes the context a treatment variable. Figure 6.1 holds the same child in two environments and shows two children. The counsellor's work is therefore often architectural: change the tempo, the threshold, the warning time, the volume of the room, the number of transitions — and watch the "symptoms" that were mismatches dissolve while the ones that were dispositions remain, manageable, in view.

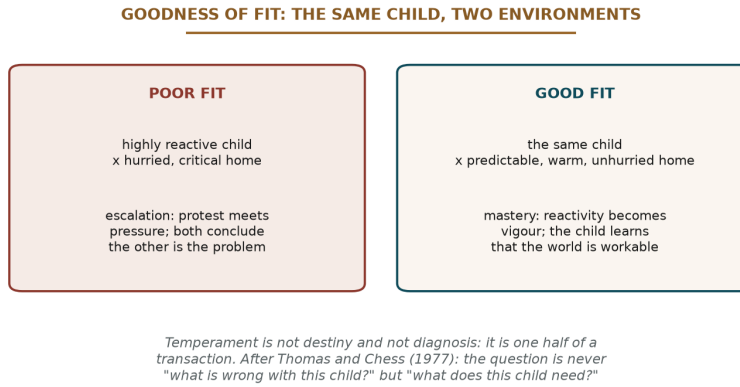


Figure 6.1 Goodness of fit: the same highly reactive child in a hurried, critical environment (escalation) and in a predictable, warm, unhurried one (mastery); temperament is one half of a transaction. After Thomas and Chess (1977). *Source:* prepared for this edition.

PRACTICE NOTE

Clinical implication. Write temperament into every formulation as its own line — reactivity, regulation, approach — and write the context's demands beside it. Where the two rows disagree, you have found free treatment: fit-work costs nothing but attention.

Lecture II — From co-regulation to self-regulation, via attachment

Regulation is built outside-in. The newborn's arousal is managed by the caregiver's body and voice; by six months, face and voice still carry most of it, and the still-face demonstration shows the infant working — turning away, self-comforting, collapsing — when the regulating face goes dead. Across the first year, responsive care converts signals into expectations: the child who is reliably met signals openly; the child whose bids are rejected or unanswered organises around not bidding (volume 1, week 3); the child whose care is unpredictable escalates, because amplification has been the only reliable strategy.

These accumulated expectations are the internal working models — not diagnoses but habits of hope, carried into the nursery, the classroom and eventually the counselling room, where they will be tested against you within three sessions whether you notice or not. Two cautions belong here. First, working models are **revisable**: later responsive relationships edit them, which is the scientific warrant for every late intervention. Second, attachment categories describe **relationships**

and strategies, never the worth of a mother; the science was built to describe care, and its misuse as blame is a corruption of it.

Conscience and self-control grow on the same stock: Kochanska's mutually responsive orientation — the warm, reciprocal partnership of early childhood — predicts conscience internalisation better than discipline style alone. The child cooperates with the regulation of someone they are in tune with; the child obeys the controller only while controlled.

> **SAFEGUARDING AND DUTY TO REPORT**
> Temperament language can launder blame: "difficult child" becomes the parent's alibi or the parent's indictment. The safeguard is the fit-frame in writing: records state the child's style, the context's demands, and the redesign attempted — never a child's character. And where a parent describes a baby as "bad" or "punishing me", hear it as a cry for co-regulation support and for the parent's own regulation: the referral that prevents harm is often to the parent's care, not the child's.

Lecture III — The caregiver's regulation: the hidden half of co-regulation

Proposition 1. Co-regulation is a two-body system. The child borrows the caregiver's pace, pitch, hold and face; therefore the caregiver's own regulation is the child's thermostat. A caregiver in chronic stress cannot lend what they do not have, and the infant's escalating bids then tax the caregiver further — a transaction in which neither party is the villain and both are the treatment target.

Proposition 2. Supporting the caregiver is a child intervention. Sleep, respite, a listening hour, treatment of the caregiver's depression, a cash transfer that removes one emergency: each has been shown, in various samples, to reach the child through the caregiving channel. The referral that sends only the child to therapy while the regulator drowns has treated the symptom and abandoned the mechanism.

Proposition 3. The counsellor is a regulator too. Your pace and pitch in the room are co-regulation with the child and, visibly, a demonstration to the parent. Supervision exists partly because the practitioner's own regulation is equipment that wears: the counsellor who arrives dysregulated becomes the child's second bad morning.

> **SAFEGUARDING AND DUTY TO REPORT**
> Where a caregiver's dysregulation becomes frightening to the child, the safeguarding frame overrides the supportive one: support and protection are sequenced, not chosen between, and the child's safety is the first sentence in the record.

Worked example 6.1 — Ife at the doorway: a fit redesign

Task. Ife (3:8), slow-to-warm, intense, irregular in settling, spends twenty minutes at the nursery doorway each morning; staff report her as "anxious and manipulative". Redesign the fit.

Solution. First delete the adjectives: slow-to-warm is her style; twenty minutes is her regulation time; "manipulative" is a moral reading of a temperament–context mismatch. The context's demands, audited: arrival into a loud free-play room at peak time; goodbye required within one minute of entry because staff are busy; no warning sequence; a different greeter on most days. Four redesigns, in order of cost: (i) a **predictable greeter** — the same key worker receives her every day, because novelty taxes the withdrawing child most; (ii) a **threshold ritual** — goodbye at the gate, not the doorway, with the three-step sequence of week 4, so the transition is one, not three; (iii) **watching time honoured** — a seat at the edge from which Ife may watch as long as she needs, with the rule that watching is participation, announced to staff so no one pulls her in; (iv) **arrival before the crowd** — ten minutes early, when the room is quiet and the greeter is free. Expected course: twenty minutes to eight within three weeks, recorded daily. If settling does not shorten, the hypothesis changes — to separation anxiety of clinical intensity (volume 1, week 5) or to something in the room we have not seen — and the record of the redesign is what makes that second look possible.

Worked example 6.2 — The parent who says "she is punishing me"

Task. Ife's mother, exhausted, says: "She does it to punish me for trading." Compose the counsel.

Solution. Three sentences, in order. Join: "You have been carrying twenty minutes of her crying and a full market day, every day, for months — of course you are tired enough to hear it as punishment." Reframe with mechanism: "She is not punishing you; she is a child whose system needs twenty minutes and one familiar face to change worlds, and right now the world changes too fast and with too many faces. It is her temperament meeting her nursery's timetable, not her heart meeting you." Turn to work: "We are going to change four things at the nursery and one at home — the goodbye ritual is yours, and it will be the thing she carries in her body when you are at the market. In three weeks we will count the minutes together." The reframe matters clinically as well as kindly: a parent who believes the child punishes is a parent whose co-regulation has collapsed into conflict, and the child's regulation cannot recover inside that belief.

SKILLS LABORATORY

Practical 6 — the fit map. Take a referred child you know (anonymised) or a supplied case. Draw two columns: the child's temperament in five lines (reactivity,

regulation, approach, rhythm, intensity); the context's demands in five lines (tempo, noise, transitions, novelty, adult availability). Circle every mismatch. Choose the two cheapest redesigns and write them as instructions a teacher could follow on Monday. 400 words; the mark is for redesigns that cost nothing but attention.

ASSESSMENT

Assessed task. A goodness-of-fit analysis of 900 words on a supplied case: temperament profile, context audit, mismatch list, two redesigns with expected course and review date, and one paragraph on how you would write this in a record without a single adjective about character.

Criterion	Weight
Temperament described in dimensions, not adjectives	25%
Context audited with equal seriousness	25%
Redesigns concrete, cheap, reviewable	30%
Record language safeguarded against character-blame	20%

Formative. Record, verbatim, one co-regulation sentence you actually use with a distressed child, and analyse what it does to arousal (pace, pitch, predictability).

UNIT SYNTHESIS

Synthesis. Temperament is the first draft of the person; attachment and co-regulation are the process by which regulation is borrowed until it is owned; and goodness of fit makes the context a treatment variable — so the practitioner writes style and demands side by side, redesigns the mismatches, protects the revisable working model, and refuses, in writing and in speech, to call a mismatch a character.

READING (APA 7TH)

- Ainsworth, M. D. S., Blehar, M. C., Waters, E., & Wall, S. N. (1978). *Patterns of attachment: A psychological study of the strange situation*. Lawrence Erlbaum.
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- Kochanska, G. (2002). Mutually responsive orientation between mothers and their young children: A context for the development of conscience. *Developmental Psychology, 38*(2), 183–195.
- Shonkoff, J. P., & Phillips, D. A. (Eds.). (2000). *From neurons to neighborhoods: The science of early childhood development*. National Academy Press.
- Thomas, A., & Chess, S. (1977). *Temperament and development*. Brunner/Mazel.

Seminar questions

1. "Difficult" is a relational adjective wearing a personal one. Give three contexts in which the same child is easy, and say what each context does differently.
2. Co-regulation makes the adult's nervous system part of the child's. What does that require of services that send dysregulated adults to dysregulated children?
3. Working models are revisable. What, realistically, revises them — and what is the counsellor's share of that work?
4. Where does goodness of fit stop being an explanation and start being an excuse for leaving an unjust context unchanged?
5. A parent says "she is punishing me". What is the developmental truth inside the sentence, and what is the pain inside it?

Next week

Week 7 follows the child's voice: how language is built from cooing to narrative, what bilingualism does and does not do, and the red flags that must never be waited out.

7 LECTURE UNIT 7 Teaching week 7 4 notional hours

Language and Communication: From Cooing to Narrative

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Sequence** the milestones of prelinguistic and linguistic communication and state the red-flag ages at which absence mandates hearing test and referral.
2. **Analyse** a short language sample: vocabulary breadth, word combinations, and the errors that are normal at a given age.
3. **Explain** what bilingual development is and is not: simultaneous and sequential acquisition, the silent period, and the myths that harm bilingual children.
4. **Describe** the input that builds language — responsive, conversational, book-sharing — and counsel a family on it without class contempt.
5. **Use** language development in counselling: matching the child's grammar, and reading a language history as part of every formulation.

Where this week sits

Language is the child's first technology and the counsellor's main medium; both facts are easy to forget. This week gives the developmental backbone of that medium — the milestone sequence with its points of no waiting, the social engine

of input, and bilingualism described properly, because in the communities this programme serves, a child's strongest language will often not be the language of their school, their tests, or their therapist.

Key concepts

MILESTONE SEQUENCE
Cooing, babbling, first words around the first birthday, a fifty-word vocabulary with two-word combinations by roughly two years, telegraphic speech, the grammar explosion of the third year, and the narrative of the preschool years: a sequence robust across cultures, with wide normal variation around it. In the room: the map against which "late talker" becomes either a watchful wait or a referral.

RED FLAG
An absence at an age where presence is near-universal: no babbling or gesture by twelve months, no single word by eighteen, no two-word combinations by twenty-four. Not a diagnosis: a **referral point** — hearing first, always — because waiting is the only irreversible mistake in this field.

CONVERSATIONAL TURNS
The active ingredient of language input: contingent, back-and-forth exchange — the served and returned volley — which predicts language growth more strongly than the number of words a child overhears. In the room: the counsel that costs nothing: talk **with**, not at; follow the child's look; answer the point before the word.

SEQUENTIAL BILINGUALISM
Acquiring a second language after the first is established: with a normal **silent period** of comprehension before production, mixing and interference that are systematic rather than confused, and a temporary gap in the second language that is a sequence, not a disorder. In the room: the difference between a child learning a language and a child referred for not having one.

Lecture I — The sequence and its points of no waiting

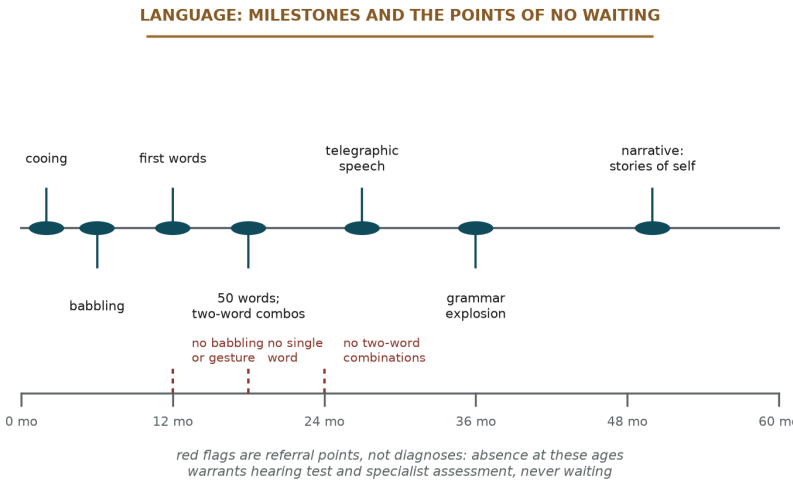


Figure 7.1 The language milestone timeline from cooing to narrative across the first sixty months, with the three red-flag ages at which absence mandates hearing assessment and referral rather than waiting. *Source:* prepared for this edition.

Proposition 1. Communication precedes language by months. Joint attention, gaze following, pointing — the declarative point that says "look with me" — are the foundation on which words are laid; a twelve-month-old who does not point or show is a child to assess even if words later come, because the social engine matters as much as the vocabulary it drives.

Proposition 2. The first lexicon is built by fast mapping and errors that prove the system. Over-extension ("dog" for every animal) and under-extension are hypothesis-testing; two-word combinations obey a grammar of their own ("mummy sock", "more juice") that no one taught; the third year's morpheme errors ("goed", "foots") are the sound of rule discovery, and correcting them into silence teaches the child that speech is dangerous.

Proposition 3. Red flags are about absence, and absence has a first cause to exclude: hearing. Every late language presentation gets a hearing assessment before any other explanation is entertained — including the child who "hears the biscuit tin", because selective hearing of desired sounds coexists with high-frequency loss. The flags of Figure 7.1 are not norms to enforce but ages at which the cost of waiting exceeds the cost of assessing.

> **PRACTICE NOTE**
> **Clinical implication.** Take a language history on every child: first words, combinations, the language of the home and of the church and of the street, who talks with

the child and when, and any period of silence and when it lifted. Four minutes that repeatedly dissolve false diagnoses and occasionally catch true ones.

Lecture II — Bilingualism, described without myths

Myth one: two languages confuse a child. They do not. Bilingual children reach the milestones of each language on the ordinary schedule **in that language**, mix languages within a sentence by rule rather than by confusion, and code-switch as competence, choosing the language of the listener when they can. **Myth two: a bilingual child's smaller vocabulary in each language is a delay.** Total conceptual vocabulary across two languages is ordinary; single-language counts understate a bilingual child, and every single-language test administered to them is a test of their exposure, not their capacity. **Myth three: the home language should be dropped for the school language to thrive.** The opposite: a strong first language is the scaffold the second is built on, and the family that abandons its language loses the conversations in which the child's mind grows — and loses the language of comfort, which is the language therapy will one day need.

The **silent period** of the sequential bilingual — weeks or months of comprehension before production — is normative and protective: the child is building the system before risking the performance. The counsel to a school that calls it mutism: date it, support it with comprehension input and gesture, and review at three months; selective mutism (week 12) speaks in some contexts and not others, whereas the silent period speaks at home fluently — the discrimination is one question to a parent.

SAFEGUARDING AND DUTY TO REPORT

Language is a safeguarding instrument: a child's disclosure arrives in their strongest language, and an interpreter who is a family member is not an interpreter — least of all where the family member is implicated. Where a child's language is not the service's, the record must state which language the child was assessed in and by whom; an assessment in a weak language is not evidence about the child.

Lecture III — Language in the counselling room: matching, waiting, and the five seconds

Proposition 1. Match the child's grammar, then stretch it by one. The counsellor's sentences should sit one clause above the child's own combinations — the linguistic zone of proximal development — because language far above the child's is noise, and language below it is condescension the child hears exactly.

Proposition 2. The five-second wait is an intervention. Questions answered after a five-second pause yield longer, truer utterances than questions

filled by the adult's own guess; the adult who completes the child's sentence has taught the child that listening is optional and that their words were predictable.

Proposition 3. Recast, do not correct. Repeating the child's error back in correct form ("me want red cup" — "you want the red cup; here it is") supplies the model without the verdict, protecting the willingness to speak that every later disclosure depends on. Correction in the room buys grammar and sells trust; the trade is almost never worth making.

> **PRACTICE NOTE**
> **Clinical implication.** Record one session and count: your words per the child's, your questions per the child's utterances, and your average wait after a question. The three numbers are a portrait of the room, and the third is the one that changes first.

Worked example 7.1 — A language sample, analysed kindly

Task. Ten minutes of play with Ife (3:8) yields: "mummy go market", "me want red cup", "doggy runned fast", "why the sky crying?", "me no like that man him shout". Analyse.

Solution. Combinations of three to four words with subject–verb–object order: within expectation at 3:8. "runned": over-regularisation of the past morpheme — evidence of rule acquisition, to be celebrated in the record, not corrected in the room. "me want": pronoun case error, ordinary at this age and gone by four to five in most children. "why the sky crying": a causal question with animist framing (week 3) — the cognitive system working exactly on schedule. The last utterance is not grammar data at all: it is content — a named man who shouts — and the sample's most important line, to be followed in safeguarding terms rather than linguistic ones. The record therefore carries two conclusions: language development within the expected range for age in her home language, with the note that any school-language assessment will understate her; and a disclosure to be pursued this week. Analysis that hears only the morphemes has missed the child.

Worked example 7.2 — The red flag and the waiting parent

Task. Kunle's classmate (24 months) has no two-word combinations, single words around eight, no pointing at twelve by mother's recall. The mother says the neighbour's child spoke at three and "boys are late". Counsel and act.

Solution. Act first, kindly: the twenty-four-month flag is met; the pathway is hearing assessment now, in parallel with a developmental language assessment — not after a waiting period, because the waiting period is the harm. Counsel the mother with the neighbour's child honoured and the population stated: some children do speak late and thrive, and no one can tell at twenty-four months which late talkers those are — which is exactly why the assessment is free of cost to her

and the waiting is not free to him. Give her the mechanism sentence: language is built in turns of conversation, and the assessment will tell us whether his ears, his processing or his opportunities are the bottleneck, because each has a different remedy. Give her work for the fortnight that is also treatment: narrate the market, count the peppers, wait five seconds after every question, and let him be wrong aloud without correction. Review date written on the card: hearing result and sample in two weeks, no exceptions.

SKILLS LABORATORY

Practical 7 — the ten-minute sample. Record ten minutes of child-led play with a child whose first language you share (or use a supplied recording). Transcribe twenty utterances verbatim. Code: word count per utterance, combinations present, one morpheme error, one evidence of rule use, one communicative function beyond requesting (comment, question, protest). Write 200 words: where this child sits against the sequence, and the one sentence of counsel to the caregiver. Bring the transcript to week 14's portfolio.

ASSESSMENT

Assessed task. A language sample analysis: your coded transcript plus 600 words placing the child on the milestone sequence, naming one red flag you checked for and how, and stating the child's languages and which one you assessed.

Criterion	Weight
Transcript verbatim, coding accurate	30%
Placement on sequence with age-appropriate errors read as competence	30%
Red-flag check and hearing logic stated	20%
Languages of the child named and respected	20%

Formative. Write the three-sentence counsel to a parent advised by family to drop the home language.

UNIT SYNTHESIS

Synthesis. Language develops by a robust sequence on a social engine: communication before words, rules discovered through honourable errors, red flags that mandate hearing assessment rather than waiting, and bilingualism that is competence rather than confusion — so the practitioner samples before they judge, assesses in the child's language, and protects the home tongue as the child's instrument of thought and of comfort.

READING (APA 7TH)

- Berk, L. E. (2013). *Child development* (9th ed.). Pearson. — **core text**; the language chapter.
- Hoff, E. (2006). How social contexts support and guide language development. *Developmental Review, 26*(1), 55–88.
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Seminar questions

1. Why is overhearing not enough? What does the conversational turn supply that the radio does not?
2. "Boys are late." What is true in the sentence, what is dangerous in it, and how do you counsel inside it?
3. A bilingual child is tested in the school language and referred. Trace the error and the record that would have prevented it.
4. Over-regularisation is intelligence audible. Which other "errors" in childhood are the same?
5. If the home language is the language of comfort, what does that imply for a therapy conducted in a school language?

Next week

Week 8 asks who the child takes themselves to be: self-concept, Erikson's crises, and the development of moral reasoning — including the critique that changed how moral development is taught.

8 LECTURE UNIT 8 Teaching week 8 4 notional hours

Self, Identity and Moral Development

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Trace** the development of self-concept from physical and possessory description to psychological and comparative self, and state what self-esteem does across childhood.
2. **Apply** Erikson's childhood crises — trust, autonomy, initiative, industry, identity — as a formulation instrument, naming what each unresolved balance looks like in the room.
3. **Sequence** moral reasoning from Piaget's realism through Kohlberg's levels, and **state** Gilligan's and social-domain critiques precisely.

4. **Describe** conscience and prosocial development as relationship-built rather than rule-taught.
5. **Interview** a child about a moral dilemma and code the reasoning, not the verdict.

Where this week sits

A child enters counselling with a working theory of who they are and what they owe — built from being met or not met, trusted or not trusted, able or not able. This week is that theory's development: the self that describes itself, the crises that organise it, and the moral reasoning that justifies it. It closes the volume's first movement; from week 9 the child meets other people.

Key concepts

SELF-CONCEPT
The child's working description of themselves: physical and possessory at three ("I have a red cup"), behavioural and comparative in the primary years ("I can run faster than Tobi"), psychological and trait-based from late childhood, and in adolescence an integrated identity — with the school-age dip in which the child discovers that others rank them. In the room: the register in which the child can hear you; praise pitched above the child's self-concept is not heard.

ERIKSONIAN CRISIS
Each childhood period poses a social crisis whose balance is carried forward: trust v. mistrust (hope), autonomy v. shame (will), initiative v. guilt (purpose), industry v. inferiority (competence), identity v. role confusion (fidelity). Not stages that pass but balances that persist as dispositions. In the room: the shortest useful history — which balance did this child mostly meet?

MORAL REASONING
Piaget's move from realism (rules as external, outcome-based judgement) to relativism (intent, agreement); Kohlberg's levels — preconventional (consequences and exchange), conventional (relationships and law), postconventional (principle) — as reasoning about justice, scored from justification, never from choice. In the room: the seven-year-old who judges by damage is not immoral; they are seven.

CONSCIENCE
Internalised self-regulation with an emotional core: guilt that repairs rather than shame that hides, built in mutually responsive relationships and through induction ("look at her face — she is crying because…") rather than through power assertion.

> In the room: the difference between a child who stops because someone might see and a child who stops because someone else hurts.

Lecture I — The self, built in mirrors that are people

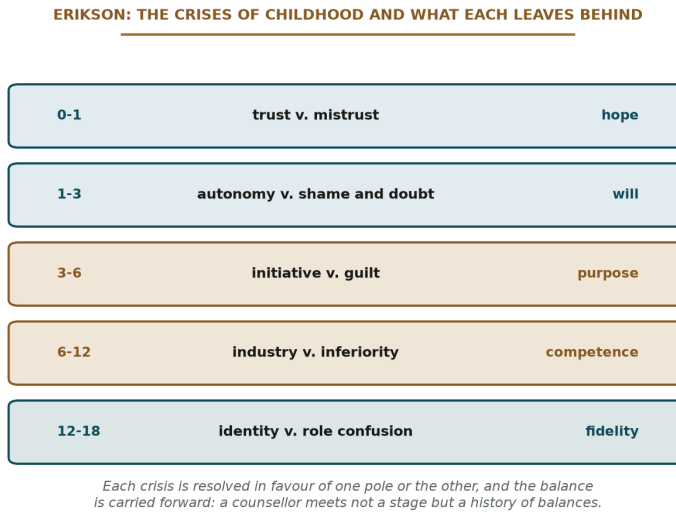


Figure 8.1 Erikson's childhood crises with their ages and the strength each leaves behind: hope, will, purpose, competence, fidelity; each balance is carried forward into the next crisis and into the counselling room. *Source:* prepared for this edition.

Proposition 1. The self is social before it is personal. The infant's sense of agency (I make the rattle move), the mirror and rouge recognitions of the second year, the possession-language of the third ("mine!") that adults misread as selfishness when it is selfhood: each is built in interaction, and each can be read back from it. The school-age child's self-concept is comparative because school is comparative — and the dip in self-esteem around eight to twelve is the child discovering the ranking, not a disorder of confidence.

Proposition 2. Erikson's balances are the counsellor's shortest anamnesis. The child who cannot trust the room (mistrust carried), who cannot choose without shame (autonomy defeated), who proposes nothing (initiative punished), who expects to be bad at everything before trying (industry defeated), who is no one in particular and terrified of it (identity diffuse) — five sentences, each with a history, each with a different therapeutic task. Figure 8.1 is a history-taking form wearing a diagram's clothes.

Proposition 3. Adolescent identity work begins earlier than adolescence. The primary-years industry crisis is identity's foundation: competence

experienced is the material identity is later built from; the child whose industry was defeated arrives at fifteen without raw material, and the identity confusion then looks like laziness to everyone but the developmental reader.

PRACTICE NOTE

Clinical implication. Pitch your language at the child's self-concept: with the under-six, concrete and possessory ("you worked hard on that tower"); with the school-age, behavioural and effort-based, never trait-based ("you kept going" not "you are clever"), because trait praise collapses at the first failure and teaches the child to hide from difficulty.

Lecture II — Moral reasoning and its critiques

Piaget watched children play marbles and judge stories: the young child's morality is realist — rules are external and unchangeable, and the child who breaks twelve cups by accident is naughtier than the child who broke one stealing jam, because damage is the measure. By roughly ten, intent governs and rules become agreements. Kohlberg generalised the sequence into levels and stages using dilemmas (Heinz and the drug above all), scoring the **reason** given, never the choice made.

Gilligan's critique, precisely stated: Kohlberg's instrument prized a justice-and-rights voice and scored a care-and-responsibility voice lower; women and girls in his samples appeared "less developed" by a yardstick built from boys' answers. The correction is not that care replaces justice but that moral maturity has two registers, and an instrument blind to one mis-measures whoever speaks it. **Social-domain theory's critique**, equally important for practice: children do not reason morally about everything at once — they distinguish, from early, the moral (harm, fairness), the conventional (school rules, dress) and the personal (my hair, my friend), and much adult–child conflict is a domain dispute: the adult arguing convention as if it were morality, the child defending the personal as if it were a right.

Conscience and prosocial development grow in relationship, not in lecture: induction — drawing the child's attention to the other's feeling and the repair available — builds guilt-that-repairs; power assertion builds compliance-that-vanishes-when-unwatched; and prosocial behaviour follows empathy's development, from reactive crying in infancy to helping that costs the helper something by the primary years (Eisenberg and colleagues). The child who shares because they are watched is at an earlier station, not a worse soul.

SAFEGUARDING AND DUTY TO REPORT

Moral reasoning knowledge protects children in two directions. First, from adult over-reading: a seven-year-old's outcome-based judgement is not callousness, and a

child's lie to escape punishment is preconventional reasoning doing its job — punish the lie as character and you teach concealment. Second, from adult under-reading: children distinguish moral from conventional early, so a child who says "that rule is stupid but I follow it" and a child who says "hitting is wrong even if teacher doesn't see" are making different claims, and the second is the conscience you are building on. Records of a child's "immorality" are usually records of their age; write the age.

Lecture III — Conscience in practice: induction, repair, and the apology that is not shame

Proposition 1. Induction builds guilt-that-repairs; power assertion builds compliance-that-vanishes. Drawing the child's attention to the other's feeling and to the repair available ("look at her face; what could make it better?") internalises a standard the child owns; punishment externalises the standard into the punisher's presence, so the behaviour returns when the punisher leaves.

Proposition 2. The apology is a repair script, not a shame ritual. "Say sorry" demanded in public teaches performance; the repair sequence — name the harm, ask the harmed what helps, do it — teaches conscience and leaves the child's dignity intact. Where a service insists on the performed apology, the counsellor's work is to convert the ritual into the sequence without open defiance: add the question and the deed to the word.

Proposition 3. Shame and guilt are different destinations. Guilt says "I did a bad thing" and moves toward repair; shame says "I am bad" and moves toward hiding, and hiding is the soil of concealment, of lie upon lie, and of the unreportable disclosure. Every moral conversation with a child is therefore also a decision about which of the two the child will carry home.

> **SAFEGUARDING AND DUTY TO REPORT**
> A child whose file accumulates moral adjectives ("manipulative", "remorseless") is a child whose future hearings are pre-written; the record carries behaviours and contexts, and the conscience work carries the hope.

Worked example 8.1 — The cup story with Kunle, coded

Task. Tell Kunle (7) the standard pair: child A opens the cupboard for a sweet and knocks fifteen cups down; child B takes a sweet they were told not to and knocks one cup down. Who is naughtier, and why? Code.

Solution. Kunle: "A. Him break plenty." Asked whether A meant to: "no, e just open." Asked if that matters: "the cups still break." Code: realism / outcome-based judgement with intent acknowledged but not weighted — the expected profile at seven, sitting at the realism-to-relativism transition. The record's value is the justification, and the follow-up question that locates the zone: "if A was

helping mummy and B was hiding from mummy, which one would you tell mummy about?" — Kunle hesitates, then "B... because e hide." The hesitation is the zone: intention is entering the system under social framing. Next teaching step (week 4's scaffold): stories in which intent and outcome are pitted with the child as judge, then as author. And the Erikson line in the same record: Kunle's industry — "I don't want to write because my writing is bad" — names the balance to protect this term; moral work and industry work will run in the same sessions, because a child who expects to be wrong judges himself by outcomes too.

Worked example 8.2 — The domain dispute in the staffroom

Task. A teacher demands that a nine-year-old apologise for refusing to cut his long hair: "it is school rule and he must respect rules". Advise, using social-domain theory.

Solution. Name the domains aloud: hair is in the personal domain for the child and, in this school's code, in the conventional domain for the institution — and the conflict is real but not moral; no one has been harmed, and demanding an apology recruits the moral register ("respect", "sorry") for a conventional dispute, which teaches the child that all rules are moral and all disobedience is sin, and buys the school a resentment it will pay for at thirteen. Advise the sequence: (i) uphold the institution's right to a dress code transparently, as convention, with its reason stated ("our code says X because Y"); (ii) withdraw the moral demand — no apology required for a domain claim; (iii) give the child a voice within the convention (a hearing, a review date, an exemption route), which is how conventions earn allegiance; (iv) watch the child's own reasoning — if he argues "my body, my choice", he is making a personal-domain claim worth hearing developmentally, not a disciplinary offence. The teacher leaves with a distinction that will save a dozen relationships: morality is about harm and fairness; convention is about how we run this place; and children can tell them apart long before staffrooms can.

SKILLS LABORATORY

Practical 8 — the moral interview. Interview two children of different ages with the cup stories and one personal–conventional probe ("Is it wrong to wear red socks here if the school says white? Is it wrong to hit? Why the difference?"). Code each: realism/relativism or level; domain distinctions present or absent; register (justice, care, or both). Write 300 words comparing the two children's reasoning and naming one teaching step for each. The mark is on the coding of justifications.

ASSESSMENT

Assessed task. A moral reasoning interview record with coding, plus 600 words: the child's Eriksonian balances as you read them from the session and the file, and one intervention pitched at the child's self-concept register.

Criterion	Weight
Justifications recorded verbatim and coded correctly	35%
Domain distinctions analysed	20%
Erikson applied as history, not label	25%
Intervention pitched at the child's self-concept	20%

Formative. One paragraph: which Eriksonian balance was mostly met in your own childhood, and what it costs you to defend it in others.

UNIT SYNTHESIS

Synthesis. The self is built in social mirrors and carries its Eriksonian balances forward; moral reasoning moves from outcome to intent across the primary years in two registers, justice and care, inside a mind that distinguishes moral, conventional and personal from early — so the practitioner codes justifications, pitches praise at the child's register, builds conscience by induction and repair, and refuses to moralise a domain dispute or an age.

READING (APA 7TH)

- Berk, L. E. (2013). *Child development* (9th ed.). Pearson. — **core text**; self, emotional and moral development.
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- Kohlberg, L. (1969). Stage and sequence: The cognitive-developmental approach to socialization. In D. A. Goslin (Ed.), *Handbook of socialization theory and research* (pp. 347–480). Rand McNally.

Seminar questions

1. Trait praise collapses at first failure. What does effort praise do instead, and why does it survive?
2. A child lies to escape punishment. Write the developmental sentence, the moral sentence, and the counsel sentence — and say which order you would speak them in.

3. Gilligan's critique is often summarised as "women are care-oriented". What is wrong with that summary, and what is lost by it?
4. Which domain is a school uniform rule in, and what changes if a school argues it as morality?
5. Which Eriksonian balance do you defend too quickly in the children you meet, and what in your own history is defending it?

Next week

Week 9 sends the child out to meet equals: peer relations, the development of friendship, the stages of play, and the four accounts of gender development argued against each other.

9 LECTURE UNIT 9 Teaching week 9 4 notional hours

Peers, Play and Gender: The Child Among Equals

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Sequence** Parten's play types and use them observationally, recording the share of time rather than the presence of a type.
2. **Describe** the development of friendship understanding and peer status, and identify a neglected or rejected child in a classroom from behaviour rather than from popularity.
3. **Apply** the social information-processing model to one case of peer conflict, locating the step at which the child's processing diverges.
4. **Compare** the four accounts of gender development — biological, social learning, cognitive-developmental and gender-schema — and counsel a family or school from the comparison.
5. **Observe** play for twenty minutes and produce a coded record usable in a case conference.

Where this week sits

Until now the child has met adults: caregivers, teachers, therapists. This week the child meets equals, and the rules change. Peer experience is not a luxury around development; it is a laboratory in which perspective-taking, negotiation, rejection and repair are practised daily, and peer status in the primary years predicts later adjustment beyond what family variables explain. Play is the medium, and gender one of the first organising principles the child imposes on the peer world.

Key concepts

PLAY TYPES
Parten's sequence — unoccupied, solitary, onlooker, parallel, associative, cooperative — describes the social density of play across the early years. Earlier types persist; development is the changing share of time, not the disappearance of a stage. In the room: the observational grammar of the nursery, and the reassurance that a five-year-old playing alone is choosing, not failing.

PEER STATUS
Sociometric positions — popular, rejected, neglected, controversial — measured by nominations rather than by adult impression. Rejected children are actively disliked; neglected children are simply unseen, and the two need opposite interventions. In the room: the difference between a child who needs social skills and a child who needs to be noticed.

SOCIAL INFORMATION PROCESSING
Crick and Dodge's steps: encoding cues, interpreting intent, clarifying a goal, generating responses, deciding, enacting. Hostile-attribution bias at the interpretation step is the engine of much reactive aggression — and a trainable step. In the room: the micro-map behind "he just hits".

GENDER SCHEMA
The cognitive structure that organises the child's world by gender once constancy is achieved: attention, memory and self-evaluation become gender-filtered, so the child seeks the information their schema marks as theirs. In the room: why telling a child "anyone can play" loses to a schema that decides who "anyone" is.

Lecture I — Play and the peer world

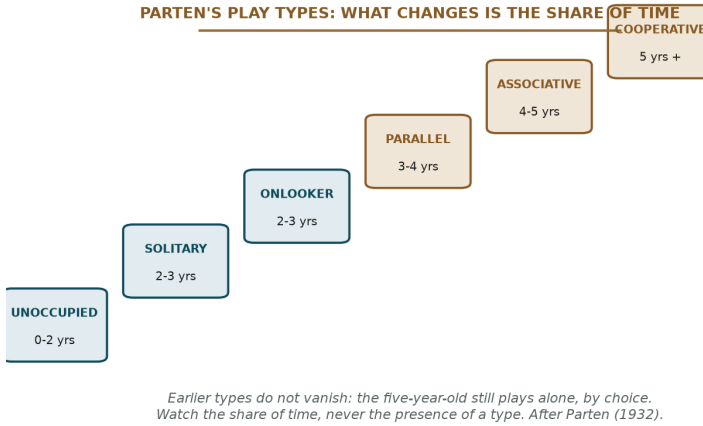


Figure 9.1 Parten's play types with their typical ages: unoccupied, solitary, onlooker, parallel, associative and cooperative; earlier types persist and what develops is the share of the child's time. After Parten (1932). *Source:* prepared for this edition.

Proposition 1. Play is the child's peer curriculum. In parallel play the child learns proximity without conflict; in associative play, exchange of materials; in cooperative play, roles, rules and repair. The nursery that schedules only group activity deprives the child of the solitary and parallel practice that cooperative play is built on; the nursery that never protects solitary time misreads its own best-regulated children.

Proposition 2. Friendship understanding develops in stages. From the momentary playmate of the preschool years ("she has a bike"), through the one-way and two-way fair-weather friendships of the primary years, to the intimate, mutually shared friendship of late childhood and adolescence (Selman). A seven-year-old's "best friend" of four days is not fickleness but stage: friendship as reciprocal service has not yet become friendship as mutual understanding, and counselling that moralises the change mislabels development.

Proposition 3. Peer status is measurable and malleable. Rejection is predicted less by difference than by the interaction of difference with dysregulation: the inattensive child who interrupts, the intense child who escalates. Neglect — the unseen child — is the status that most resembles Kunle at seven: competent, quiet, neither chosen nor refused, and at risk not of conflict but of absence from the peer record altogether. Interventions differ utterly: the rejected child needs regulation and entry skills rehearsed; the neglected child needs visibility engineered by adults who arrange the dyads.

PRACTICE NOTE

Clinical implication. Ask the peer question on every child over four: who chose them yesterday, and for what? A child's peer position is part of the presenting picture, and often the part the child feels most.

Lecture II — When peer conflict becomes a processing map

The child who hits after a bump in the corridor has, in two seconds, encoded the cue (contact), interpreted it (hostile), chosen a goal (retaliation or safety), generated responses (hit, tell, walk), decided and enacted. Crick and Dodge's model locates the divergence: some children under-encode (miss the apology), some over-attribute hostility, some generate only one response, some decide by fear of looking weak. Each step has its own intervention — and "anger management" that addresses none of the steps manages nothing.

The benignant counterpart matters as much: prosocial children process the same bump with a benign attribution because their history supplied benign episodes to encode. Processing is history made procedure.

Lecture III — Gender: four accounts, one child

Biological accounts point to prenatal hormonal organisation and to modest average differences in some domains; they explain little of the variance in any individual child and nothing of the content of gender roles, which varies enormously across cultures and decades. **Social learning** accounts show modelling and differential reinforcement: adults respond to the same behaviour differently by perceived sex, and children imitate same-sex models preferentially. **Cognitive-developmental** accounts (Kohlberg's gender constancy) show the child constructing gender as a category: once the child grasps that gender is constant, they actively seek gender-consistent information. **Gender-schema** theory integrates: the schema, once formed, filters attention, memory and self-worth — the child who refuses the "girls' crayons" is not obeying an adult but protecting a self-category.

The counselling consequence is precise: rules and exhortation address social learning; the schema addresses itself through experience that the schema cannot digest — the boy who discovers he loves the cooking corner does not update by instruction but by an episode the schema cannot file. And where gender itself is the child's question rather than the peer world's rule, the child's own account governs the conversation, and the developmental task is safety and self-authorship, not category enforcement.

SAFEGUARDING AND DUTY TO REPORT

Peer work carries two safeguarding edges. First, bullying is a peer-status phenomenon with a power asymmetry; it is never "conflict" between equals, and mediation between a bully and a target re-injures the target — protect first, then work separately. Second, gender non-conformity attracts peer victimisation; the school's obligation is the child's safety and the peer group's conduct, never the child's conformity, and a counselling referral whose implicit aim is conformity is a harm wearing a helping face.

Worked example 9.1 — Ife at the nursery edge, coded

Task. Twenty minutes of observation: Ife (3:8) spends nine minutes in solitary symbolic play with cups, four minutes watching two children build (onlooker, with one comment unanswered), five minutes beside a child filling buckets — same activity, no exchange (parallel), and two minutes in a chase initiated by the other child (associative, brief). Code and counsel.

Solution. The profile is normative at 3:8: solitary-symbolic and parallel dominant, associative emerging, cooperative absent — exactly the share-pattern of Figure 9.1. The onlooker episode with the unanswered comment is the clinical grain: Ife bid once and did not repeat, consistent with slow-to-warm style (week 6) rather than with peer deficit. Counsel: no intervention on play type; one environmental arrangement — the key worker places Ife beside (not into) a dyad with materials that require sharing one resource (one tap, two buckets), converting parallel into associative without a demand to perform. Re-observe in four weeks and compare shares, not types. The record states the shares in minutes; minutes are the data, adjectives are not.

Worked example 9.2 — Kunle, neglected rather than rejected

Task. Sociometric picture of Kunle's class: few nominations for "like most", almost none for "like least"; teachers describe him as "no trouble, no friends". Design the intervention.

Solution. Kunle is neglected, not rejected: unseen rather than disliked, which is why skills-training would treat a deficit he does not have. The mechanism to change is visibility among equals, engineered by adults: (i) pair tasks with a defined complementary role in which Kunle holds the scarce resource (the ruler, the timer, the answer key) so that contact requires him; (ii) a standing dyad for two weeks — the same partner across tasks, because repeated contact is what converts proximity into acquaintance; (iii) one public competence display in his strength (figures), framed as service ("Kunle will check the totals"), which supplies peers with a reason to approach; (iv) no forced sharing of feelings and no "friendship group", which for a neglected child is exposure without material. Review at six weeks with a second sociometric snapshot: the target is not popularity but a rise in

neutral-to-positive nominations and at least one reciprocal choice. The falsifier: if nominations are unchanged at six weeks, the hypothesis (invisibility, not dislike) is wrong, and the assessment turns to social anxiety and to the child's own wishes — some children prefer few companions, and a counsellor must distinguish the neglected child who suffers from the solitary child who does not.

SKILLS LABORATORY

Practical 9 — the play record. Observe (live or recorded) one child for twenty minutes in free play, sampling every thirty seconds: play type, partner count, bids made, bids answered. Produce the minute-shares table and one paragraph: the child's dominant type against age expectation, one bid sequence transcribed verbatim, and one environmental arrangement you would make. Bring the table to week 14's portfolio.

ASSESSMENT

Assessed task. A play observation record with coded table plus 600 words: peer status hypothesis (rejected / neglected / none) with evidence, one social information-processing step you would target if conflict were present, and the arrangement or intervention that follows.

Criterion	Weight
Sampling and coding accurate, shares in minutes	30%
Status hypothesis distinguished (rejected v. neglected) with evidence	30%
Processing step or arrangement matched to mechanism	25%
Child's own preference considered	15%

Formative. Recall your own peer position at eight. Name it in sociometric terms, and name one adult arrangement that would have changed it.

UNIT SYNTHESIS

Synthesis. Among equals the child practises everything the adult world cannot teach: play types sequence and persist as shares of time, friendship understanding deepens in stages, peer status splits into rejected and neglected with opposite treatments, conflict resolves into processing steps that can each be trained, and gender organises the peer world through schemas that change by indigestible experience rather than by instruction.

READING (APA 7TH)

- Berk, L. E. (2013). *Child development* (9th ed.). Pearson. — **core text**; peer and gender chapters.
- Coie, J. D., Dodge, K. A., & Coppotelli, E. (1982). Dimensions and types of social status in peer groups. *Developmental Psychology, 18*(4), 557–570.
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- Selman, R. L. (1980). *The growth of interpersonal understanding: Developmental and clinical analyses*. Academic Press.

Seminar questions

1. Why is "neglected" not a milder form of "rejected", and what intervention would be actively wrong for each?
2. A child plays alone at six. List the three observations that would distinguish preference, slow-to-warm style and peer difficulty.
3. Hostile-attribution bias is learned from history. What history, and what episode could begin to unlearn it?
4. Where does your school culture reinforce a gender schema you would rather the child did not carry? Name one arrangement that feeds it.
5. Bullying is not conflict. What in the definition makes mediation contraindicated?

Next week

Week 10 widens to the whole context: parenting styles and their cultural limits, adversity and its dose–response, and resilience as ordinary magic — the week that turns a family history into a map of what protects.

10 LECTURE UNIT 10 Teaching week 10 4 notional hours

Family, Culture, Adversity and Resilience

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Describe** the parenting-style dimensions and quadrants, and state the cultural limits of the authoritative ideal with an example from your own setting.

2. **Explain** the family stress model: how economic pressure reaches the child through the caregiver's distress and the interparental relationship.
3. **Summarise** the adverse childhood experiences literature — dose–response, mechanisms, and its three principal critiques — without converting a score into a verdict.
4. **Define** resilience as process rather than trait, and list protective factors at child, family and community level for one case.
5. **Take** an adversity history in a way that protects the teller, and build an adversity-and-resilience map usable in formulation.

Where this week sits

Every child in this volume develops inside a family inside an economy inside a culture. This week is the ecology made clinical: what parenting practices do and do not travel, how poverty and adversity reach the developing child through the people around them, and why some children under the same loads develop well — not because they are harder, but because something protected something. It is the week the counsellor learns to hold a family history without dropping it on the family's head.

Key concepts

PARENTING STYLE
A climate of practice described by warmth/responsiveness and control/demandingness: authoritative, authoritarian, permissive, neglectful. Styles describe practices, not persons, and their outcomes are partly cultural: control that is reasoned and explained travels with warmth in some communities and without it in others, and obedience can be protection where the street is dangerous. In the room: the quadrant is a question about practice, never a verdict on a parent.

FAMILY STRESS MODEL
Economic pressure produces caregiver distress; distress erodes the interparental relationship and the quality of parenting; it is through those proximal processes, not through money itself, that poverty reaches the child's development. In the room: the mechanism that makes support to the caregiver a child intervention.

ACES
Adverse childhood experiences — abuse, neglect, household dysfunction — accumulate and show a dose–response relation with later health and mental-health outcomes at group level. Critiques: the original items were clinical and narrow, culturally situated, and silent on the adversities of poverty, discrimination and com-

munity violence; and a count says nothing about meaning, timing or protection. In the room: a prompt for enquiry, never a prognosis.

RESILIENCE
Good development under significant adversity, produced by ordinary processes — a stable committed adult, self-regulation skills, community routines and beliefs — rather than by rare personal qualities: "ordinary magic" (Masten). In the room: the list of what to protect when everything else is being assessed.

Lecture I — Parenting styles and the question of travel

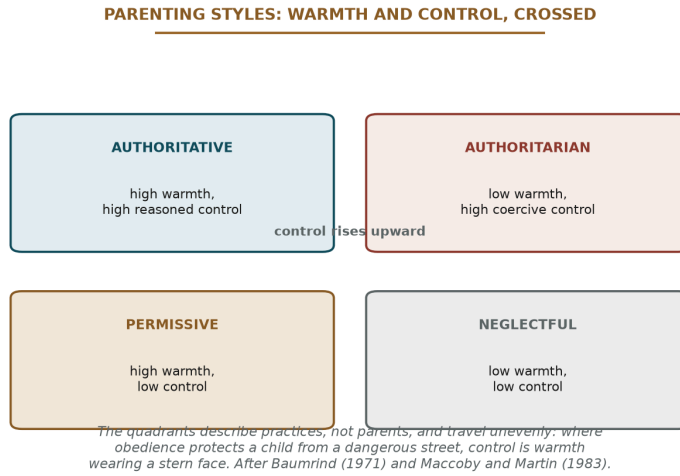


Figure 10.1 The two dimensions of parenting crossed: warmth and control, yielding authoritative, authoritarian, permissive and neglectful quadrants; practices, not parents, and the quadrants travel unevenly across cultures. After Baumrind (1971) and Maccoby and Martin (1983). *Source:* prepared for this edition.

Proposition 1. The dimensions are robust; the ideal is local. Warmth and control organise parenting description across cultures, but the authoritative quadrant was normed on a particular middle-class Western sample, and its advantage is partly the advantage of reasoning-talk that schools reward. In communities where immediate obedience protects a child from traffic, water or violence, high control with high warmth is not authoritarianism missing its warmth; it is warmth doing its job under different risks. The counsellor's error is not using the quadrants but forgetting which quadrant the local danger-reward structure selects for.

Proposition 2. Neglect is the quadrant with the worst outcomes and the quietest presentation. Low warmth and low control — the parent absent in body or attention — travels with the poorest outcomes across studies, and

presents in school not as conflict but as hunger, dirt, lateness and a child who asks nothing. It is also the quadrant where the counsellor's work is least psychological and most practical: presence, meals, routines, and a named adult who notices.

Proposition 3. Style is a climate, and climates change with stress. The same parent is authoritative in a good month and authoritarian in a bad one; the family stress model explains why: economic pressure taxes the caregiver's regulation, and regulation is what warmth and reasoned control run on. Counsel that praises the good-month parent and condemns the bad-month parent has misread a single mechanism as two characters.

Lecture II — Adversity: dose, mechanism, critique

The ACE literature's contribution is cumulative: adversities cluster (where there is one, there are usually others), and the count predicts later mental and physical health at group level in a dose–response gradient. The mechanisms are developmental rather than mystical: chronic stress physiology acting on the developing brain and immune system (week 2′s experience-expectant circuits), disruption of the caregiving relationship that would otherwise buffer stress, and the transactional consequences — a hypervigilant child evokes punishment, a withdrawn child evokes neglect of their needs.

Three critiques belong in every competent use. **Item critique:** the original set omits poverty, racism, community violence, displacement and disability of a family member — the adversities most present in many of the communities this programme serves; a low ACE score can therefore mean a safe childhood or an uncounted one. **Meaning critique:** the same event under a protective adult who explains and stays is a different developmental dose from the same event alone; the count cannot see the buffer. **Verdict critique:** a score used as prognosis removes agency from the family and the future from the child; the dose–response is about groups, and every individual child sits somewhere the group cannot say.

> **SAFEGUARDING AND DUTY TO REPORT**
>
> Asking about adversity is an intervention with a cost: each question reopens something. Three rules govern: ask once, in order, and record once, so no family tells the story five times to five services; ask with the buffer in the same sentence ("who was with you? who found out? who stayed?"), so the telling rehearses protection as well as harm; and never ask a child about a parent's wrongdoing in a room the parent can hear. Where disclosure meets a mandatory-report threshold, the child's safety governs and the family is told what will happen next, in plain words, before anyone else is told.

Lecture III — Resilience: ordinary magic, listed

Masten's finding is deliberately unromantic: resilience under adversity arises from ordinary adaptive systems — at least one committed adult, the child's own regulation and problem-solving skills, school and community routines that continue, and cultural or faith meanings that hold the suffering inside a larger story. None of these is rare; what is rare is their simultaneous absence, and that absence, not the adversity alone, is what predicts poor outcome.

The counsellor's use is architectural. For each child under load, list the protective factors at three levels — child (skills, temperament, meanings), family (the committed adult, the routines, the interparental climate), community (school, faith community, neighbours, cash) — and mark which are present, which are strained and which are absent. The intervention then aims at the strained, not at the absent: reinforcing a grandmother who is still there outperforms conjuring a father who is not. Resilience work is maintenance work on the ordinary, done before the ordinary breaks.

> **PRACTICE NOTE**
> **Clinical implication.** End every adversity history with the buffer question and the magic question: "who stayed?" and "what has held you up since?" The answers are the treatment plan's first materials, and they are the family's own.

Worked example 10.1 — The score that is not a verdict

Task. A school social worker hands you an "ACE score of 4" for Kunle and Ife's household (parental separation pending, father's night work counted as absence by the tool, mother's treated depression, an uncle's imprisonment) and asks whether the children are "high risk". Respond.

Solution. First, audit the items against this family: father's night work is counted by the tool as absence but is, in this family, provision — he sleeps days in the same compound and coaches Kunle's figures on Sundays; the item has mis-scored love as loss. Mother's depression is treated and, in her good months, the household climate is warm; timing and treatment are invisible to the count. The uncle's imprisonment carries stigma in the community that the count also cannot see, and that stigma — not the imprisonment — may be the active adversity for the children at school. Second, refuse the verdict: the score is a prompt, and what it prompts is enquiry, not prognosis; the dose–response is a group statement and these are two individuals. Third, replace the score with the map: adversity items audited and corrected (two confirmed, one mis-scored, one community-stigma item added by the family); buffers listed (grandmother daily, father Sundays, mother's treatment adherent, church youth group for Kunle); strains marked (mother's sleep, the stigma at school, the pending separation's uncertainty for Ife).

The plan addresses the strains: a school conversation about the stigma, a routine card for Ife's mornings during the separation, and protection of the Sunday figures hour as the family's strongest buffer. "High risk" becomes a list of what to protect — which is a plan, and which a score never was.

Worked example 10.2 — Parenting style across the street

Task. A teacher reports that Ife's mother "shouts orders and never explains", and proposes parenting classes. Advise using this week's frame.

Solution. Observe before classifying: in the market, the same mother explains prices, negotiates, and teaches Ife to count change with patience; at the compound gate at dusk, with a cooking fire and three children and a street with traffic, her commands are short and immediate. The difference is not style but risk-structure: explanation is a luxury of safe rooms, and the street is not a safe room. The quadrant description that fits is high control with high warmth under danger — authoritarian by coding, protective by function. Advising "explain more" without changing the risk-structure would cost the mother seconds she does not have and buy nothing for Ife. The counsel that helps: relocate the explanation to the safe room (the ritual and the reasons at bedtime, week 4's sequence), keep the gate commands short, and give the teacher the distinction between practice and climate — and the sentence that ends the referral: parenting classes are for climates; this family has a street.

SKILLS LABORATORY

Practical 10 — the adversity and resilience map. In threes, with a supplied anonymised case or a case you carry (consent and anonymisation checked): audit the adversity items including the locally relevant ones the standard list omits; list buffers at child, family and community level; mark strains; choose two strained buffers to reinforce and say exactly how. 400 words plus the map as a drawn page. The mark is for the buffer audit, not the adversity count.

ASSESSMENT

Assessed task. An adversity and resilience analysis of 1,000 words on a supplied case: audited adversity list with mis-scored and omitted items named, buffer map at three levels, two reinforcement plans, and one paragraph stating what the ACE literature does and does not license you to say about this child's future.

Criterion	Weight
Adversity audited, critiques applied (items, meaning, verdict)	30%
Buffer map at three levels, strains marked	30%
Reinforcement plans concrete and aimed at the strained	25%

Criterion	Weight
Prognosis discipline: group versus individual stated	15%

Formative. Write the buffer question and the magic question in your own words, in the language you counsel in.

UNIT SYNTHESIS

Synthesis. Context reaches the child through practice and through people: parenting styles describe climates that shift with stress and travel with culture; economic pressure and adversity act via the caregiver and the stress physiology of the developing child; the ACE count is a prompt with three critiques attached; and resilience is ordinary magic — the committed adult, the routine, the meaning — so the practitioner audits adversity without verdict, lists the buffers, and reinforces the strained ordinary before it breaks.

READING (APA 7TH)

- Baumrind, D. (1971). Current patterns of parental authority. *Developmental Psychology, 4*(1, Pt. 2), 1–103.
- Berk, L. E. (2013). *Child development* (9th ed.). Pearson. — **core text**; family and socialisation chapters.
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- Masten, A. S. (2001). Ordinary magic: Resilience processes in development. *American Psychologist, 56*(3), 227–238.

Seminar questions

1. Name one parenting practice in your community that the quadrants mis-score, and say what the mis-scoring costs a family.
2. Why does the family stress model make support to a caregiver a child intervention, and what follows for service design?

3. An ACE score of 4 is handed to you as "high risk". Which sentence of yours refuses the verdict without refusing the enquiry?
4. Resilience as ordinary magic: which ordinary thing in your own childhood was the magic, and who maintained it?
5. Where is the line between auditing a family's adversity and auditing a family's worth?

Next week

Week 11 turns to atypical development: the neurodevelopmental presentations read as differences in developmental course — attention, autism, learning — with the biases that decide who gets seen and who gets missed.

11 LECTURE UNIT 11 Teaching week 11 4 notional hours

Atypical Development I: The Neurodevelopmental Presentations

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **State** the neurodevelopmental group of DSM-5-TR and ICD-11 and the developmental logic that binds it: onsets in the developmental period, deficits in personal, social, school or occupational functioning, and a course rather than an episode.
2. **Describe** ADHD dimensionally — inattention, hyperactivity, impulsivity — with its developmental course, its executive-function substrate, and the referral biases by gender and context.
3. **Describe** autism spectrum presentation across ages, the early red flags, and the double empathy problem as a critique of one-sided social deficit accounts.
4. **Describe** specific learning disorders and their relation to language development, and explain why response to teaching is part of the definition.
5. **Build** a developmental timeline for one referred child, separating onset, course, pervasiveness and impairment from the presentation of the moment.

Where this week sits

Weeks 1 to 10 described the ordinary course and its variation. This week asks what it means when a child's development is not within the range of variation but on a different course — and insists that the question be developmental before it is diagnostic. A diagnosis names a pattern; a counsellor needs the pattern's history, its pervasiveness across rooms, its cost to this child's days, and the child's own account of their difference. The categories are useful; the categories are not the child.

Key concepts

NEURODEVELOPMENTAL GROUP
Intellectual developmental disorders, communication disorders, autism spectrum disorder, ADHD, specific learning disorder, motor disorders: grouped because they begin in the developmental period, affect functioning across contexts, and run as courses rather than episodes. In the room: the reason a neurodevelopmental question is always a history question.

DIMENSION AND CUT-POINT
Attention, activity, social communication and reading skill are dimensions on which all children vary; a disorder is a cut-point on a dimension, placed where impairment begins. Cut-points are decisions, and decisions carry the biases of whoever decides. In the room: why "how much inattention?" precedes "is it ADHD?", and why the same child crosses the line in one classroom and not another.

PERVASIVENESS AND IMPAIRMENT
Two gates between difference and disorder: the pattern appears across contexts and informants (pervasiveness), and it costs the child something they want — friendship, learning, ease (impairment). In the room: the two questions that most referrals have not answered.

DOUBLE EMPATHY PROBLEM
Milton's critique: the social difficulty in autism is not a one-sided deficit but a mutual mismatch of communicative styles between autistic and non-autistic people; autistic people communicate readily with one another. In the room: the reorientation from "teaching the child to pass" to building a two-way bridge, and the humility it requires of the non-autistic practitioner.

Lecture I — ADHD as a difference in course

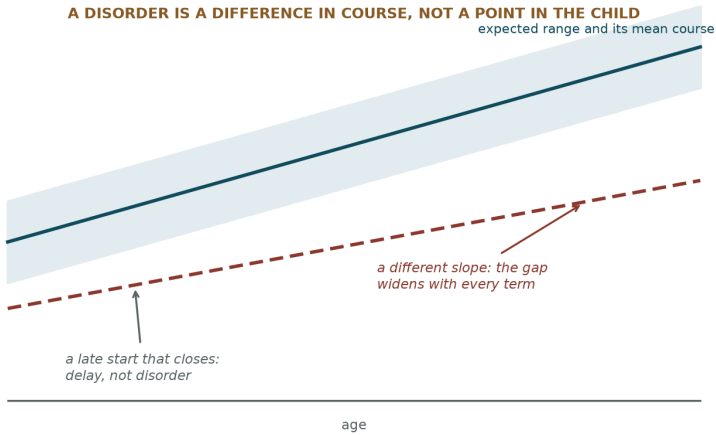


Figure 11.1 Two developmental courses against the expected range: a late start that closes (delay) and a different slope whose gap widens with every term (disorder); the distinction is in the course, never in a single point. *Source:* prepared for this edition.

Proposition 1. Attention is a dimension; the disorder is the upper tail with impairment. Inattention, hyperactivity and impulsivity vary continuously across children; the diagnostic cut-point sits where the pattern is pervasive, persistent across six months or more, present in two or more settings, and costs the child function. The executive-function account (week 5) explains much of the texture: inhibition, working memory and shifting are the systems whose immaturity produces the lost instructions, the blurted answers, the unfinished page.

Proposition 2. The presentation travels with context and with sex. The same child is more impaired in a forty-pupil classroom with three transitions an hour than in a quiet room with one adult; and girls, whose presentation skews to inattention without disruption, are under-referred for years while boys with the same scores are over-referred relative to impairment. A referral rate that tracks disruptiveness rather than impairment is measuring the classroom's tolerance, not the child's difficulty.

Proposition 3. Course matters more than state. Onset in the developmental period, persistence across years, and a gap that widens as demands rise (Figure 11.1) distinguish the neurodevelopmental course from a state — an anxious child who cannot concentrate since the separation, a hungry child who cannot sit, a child taught in a language they do not yet hold. Each of those is treatable at cause, and each is mislabelled daily.

PRACTICE NOTE

Clinical implication. Before any ADHD conversation, collect three settings' evidence (home, school, and your room or a community setting) and one week of demands: the pattern's pervasiveness is the first gate, and it is closed more often by absence of evidence than by absence of disorder.

Lecture II — Autism spectrum: red flags, course, and the two-way bridge

Autism spectrum presentation is defined by persistent differences in social communication and social reciprocity, and by restricted, repetitive patterns of behaviour, interest or activity, with onset in the early developmental period and sensory differences threaded through both. The early red flags belong to every counsellor's literacy: no response to name by twelve months with hearing excluded, no declarative pointing, no shared enjoyment shown by bringing or showing, regression in language or social engagement at any age — the last an urgent referral whatever else is true.

Across ages the presentation changes shape: the preschooler's difference in pretend play and peer interest; the school-child's literalness, justice-sensitivity and special interest that is a resource as often as a restriction; the adolescent's exhaustion from camouflage, which presents as anxiety or refusal long before anyone names its cause. Camouflage, most documented in girls and in verbally fluent children, is the reason a diagnosis can arrive a decade after the suffering began.

The double empathy problem reorients the work: if the difficulty is mutual, the intervention is bilateral — the child's own communicative preferences honoured (directness, processing time, written options, the interest as a bridge), and the environment taught the child's language as the child is taught the environment's. A therapy whose goal is indistinguishability from peers has chosen a value and called it a treatment; a therapy whose goal is the child's own communication, comfort and competence has chosen the child.

SAFEGUARDING AND DUTY TO REPORT

Diagnostic labels carry administrative consequences — support, funding, sometimes exclusion — and the counsellor sits at that hinge. Two duties follow: never let a label travel ahead of its evidence (a school's "he is autistic" is a hypothesis to assess, not a fact to file), and never let the absence of a label withhold support that a child's functioning plainly needs. Support follows function, not paperwork.

Lecture III — Specific learning disorders and the language beneath them

Specific learning disorder names persistent difficulty in acquiring academic skills — reading accuracy and fluency, written expression, mathematics — substantially below expectation for age and schooling, not attributable to intellectual disability, uncorrected sensory problems, or inadequate instruction. The last exclusion is the developmental point: a child who has not been taught to read has not a disorder but a debt, and response to systematic teaching is therefore part of the assessment, not an afterthought.

The reading difficulty sits on the language continuum of week 7: phonological processing — hearing and manipulating the sounds within words — is the substrate, and a child whose language history is thin (late talker, unaddressed hearing loss, or a school language that is not the home language without support) arrives at reading with one hand tied. Assessment must therefore name the child's languages and the language of instruction before any score is interpreted, and the bilingual child's reading difficulty must be assessed in both languages or not claimed at all.

Comorbidity is the rule rather than the exception: reading difficulty with attention difficulty, with language disorder, with the anxiety that years of public failure produce. The developmental cascade runs both ways — the child who cannot read stops reading, and stops reading into a widening gap (Figure 11.1′s slope) — which is why the counsellor's first move for a learning difficulty is often protective: rebuild the child's relationship with being taught before the gap becomes identity.

Worked example 11.1 — The timeline that refuses the label

Task. Chidi (6 y 4 m) is referred by his school as "hyperactive and unteachable": leaves his seat, calls out, one unfinished task in the record. Build the developmental timeline before any category.

Solution. *Onset:* mother dates the high activity and low settling to infancy; nursery records at three describe "always moving, joyful"; no onset event. *Course:* persistence across four years and three settings (home, nursery, school) — pervasiveness gate passed. *Impairment:* Chidi's own words — "teacher no like me; I no get star" — and the record: no finished task, two friendships strained; impairment gate passed, and the impairment includes the school's response. *Competing states excluded:* hearing and vision screened (clear); sleep history (adequate); language: home language differs from school language, and Chidi's school-language comprehension is thin — a contributor that must be measured, not assumed away; adversity: none reported, buffers strong. *Demands audit:* forty-one pupils, floor seating, three transitions before break, instructions given once, orally, in the school language. The timeline yields a formulation before a diagnosis: a pervasive,

impairing, early-onset activity and attention pattern consistent with the ADHD dimension, sitting on top of a school-language comprehension gap and inside a classroom whose demands maximise his difficulty — three targets, one of which (the language gap) is treatable to cause and one of which (the demands) is free. The referral question "is it ADHD?" becomes "which of these three courses are we changing this term, and in what order?" — and the label, when a specialist confirms it, arrives as a description of the first course rather than as an explanation of the child.

Worked example 11.2 — The girl nobody referred

Task. Amaka (9) is described as "quiet, dreamy, helpful"; her books are empty in three subjects; her mother says she "lives in her head". Teachers see no problem. Assess the referral gap.

Solution. Inattention without disruption is the classic under-referral: no classroom cost to the teacher, so no referral, while the child's cost accumulates in empty books and in the private conclusion that she is slow. The assessment sequence: three settings' evidence (home: long solitary play, forgets chores, loses materials; your room: warm, talkative once settled, working memory span thin for age on games; school: the empty books) — pervasiveness present; impairment present but internalised (shame rather than disruption); language and hearing checked; teaching adequacy checked (she has been taught; the gap widens despite teaching, Figure 11.1's slope). The formulation names the inattentive presentation of the attention dimension with impairment, plus the secondary self-concept wound of week 8 ("I am slow") that now protects her from trying. The intervention holds both courses: the attention course with the specialist and the school's accommodations (written as well as oral instructions, task chunks, seat placement), and the industry course with evidence-based praise at effort and finished-draft rituals. The safeguard note for the file: the referral gap was systemic, not accidental — record what would have caught her at six, so the next quiet girl is caught.

SKILLS LABORATORY

Practical 11 — the developmental timeline. For a supplied referred case, draw the timeline page: conception to now, marking onset claims with their informant, setting changes, demands changes, assessments and their languages, and every competing state excluded with its evidence. Beneath it, three lines: pervasiveness verdict, impairment verdict (in the child's words if available), and the competing courses still open. One page, drawn, brought to the tutorial.

ASSESSMENT

Assessed task. A developmental timeline case analysis of 1,000 words: timeline page plus written analysis separating onset, course, pervasiveness and impairment; competing states excluded with evidence; the child's languages and the language of instruction named; and a three-course formulation with the term's order of work.

Criterion	Weight
Timeline complete, informants and dates marked	25%
Pervasiveness and impairment gates argued	25%
Competing states excluded with evidence, languages named	25%
Formulation as courses with an ordered plan	25%

Formative. One paragraph: a child you know who was mislabelled or missed by a referral system; which gate failed, and whose tolerance the system was measuring.

UNIT SYNTHESIS

Synthesis. Neurodevelopmental presentations are differences in developmental course, read through gates of pervasiveness and impairment on dimensions that all children vary along; attention, autism and learning difficulties each carry referral biases — by sex, by disruptiveness, by language — and each carries a critique that reorients practice, from the double empathy problem to the teaching-debt exclusion; so the practitioner builds the timeline first, excludes the states, names the languages, and lets the label arrive last, as a description of one course among the ones being changed.

READING (APA 7TH)

- American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders* (5th ed., text rev.). — the neurodevelopmental chapter, read with its caveats.
- Berk, L. E. (2013). *Child development* (9th ed.). Pearson. — **core text**; atypical development chapters.
- Cicchetti, D., & Cohen, D. J. (Eds.). (2006). *Developmental psychopathology* (2nd ed.). Wiley. — the neurodevelopmental volumes' introductory chapters.
- Milton, D. E. M. (2012). On the ontological status of autism: The "double empathy problem". *Disability & Society, 27*(6), 883–887.
- Santrock, J. W. (2019). *Child development* (15th ed.). McGraw-Hill Education.
- World Health Organization. (2019). *International classification of diseases* (11th ed.).

Seminar questions

1. Why is "response to teaching" part of the definition of a learning disorder, and what does that make a school that has not taught?
2. A girl with the same attention scores as a referred boy is never referred. Whose tolerance is the system measuring, and what would catch her at six?
3. The double empathy problem: what in your own communicative style would an autistic child reasonably find difficult, and what will you change first?
4. Camouflage costs. What does a decade of passing buy a child, and what does it cost them?
5. Support follows function, not paperwork. Give one case where waiting for the label withheld the help.

Next week

Week 12 installs the principles that organise everything atypical and typical alike: developmental psychopathology — equifinality, multifinality, heterotypic continuity and the transactional cascade — with the caution that categories are hypotheses about children, not children.

12 LECTURE UNIT 12 Teaching week 12 4 notional hours

Atypical Development II: The Principles of Developmental Psychopathology

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Define** developmental psychopathology and state why its unit of analysis is the developing course rather than the present symptom.
2. **Apply** equifinality and multifinality to cases, and explain why each defeats a one-cause account of a presentation.
3. **Explain** heterotypic continuity — anxiety becoming depression, difficult temperament becoming conduct difficulty — and use it to read a family history forward.
4. **Use** the concepts of developmental cascade and transaction to show how a difficulty recruits its own maintenance across systems.
5. **Judge** whether a presentation is deviant for age and context, using age-appropriateness as the first criterion of abnormality in childhood.

Where this week sits

Week 11 read the categories. This week installs the grammar beneath them. Developmental psychopathology's founding insight is that pathology in childhood is not a set of diseases that happen to occur in small people but a set of deviations from, and variations of, the developing course — so every clinical question is a developmental question: deviant from what, for this age, in this context, on this trajectory? The principles of this week are the ones you will use in every formulation for the rest of your career, including the typical children's.

Key concepts

EQUIFINALITY
Different early paths to the same presentation: conduct problems at nine arriving from harsh unpredictable care, from an unmet language disorder, or from a neurological vulnerability. In the room: the reason the presentation never explains itself, and the reason two children with identical behaviour need different treatments.

MULTIFINALITY
Different outcomes from the same beginning: early adversity eventuating in recovery, in anxiety at eleven, or in conduct difficulty at nine, depending on what intervened. In the room: the scientific death of prognosis-by-history, and the scientific life of protection as a variable.

HETEROTYPIC CONTINUITY
Continuity of an underlying vulnerability across changing expressions: the inhibited toddler's wariness becoming the seven-year-old's selective silence becoming the thirteen-year-old's social anxiety and low mood; the difficult infant's intensity becoming oppositional conflict under coercive parenting. In the room: the reason a family history of "shyness" belongs in an adolescent depression formulation.

DEVELOPMENTAL CASCADE
The sequence by which a difficulty in one domain recruits others across time: reading failure to avoidance to lost practice to widened gap to self-concept wound to school refusal — each step a new maintaining system, so late intervention must open several doors, not one. In the room: the map of why "just start again" fails at year four of a cascade.

Lecture I — The course is the unit

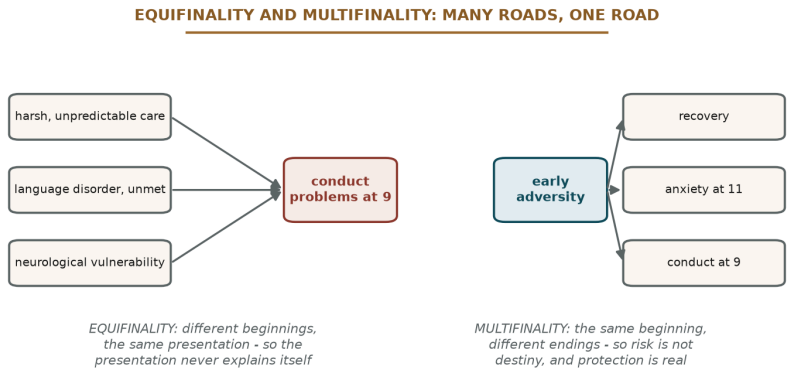


Figure 12.1 Equifinality and multifinality: three different beginnings converging on one presentation, and one beginning diverging into three outcomes; risk is not destiny and the presentation never explains itself. *Source:* prepared for this edition.

Proposition 1. Deviance is judged against the developmental norm, and the norm is contextual. The same behaviour is normative at two and deviant at seven — tantrums, night waking, fear of the dark, parallel play, private speech — so the first clinical question is never "is this bad?" but "is this expected here, at this age, in this context?" A behaviour that is expectable is not a symptom however loud it is; a behaviour that is off-age however quiet it is belongs in the formulation (a nine-year-old's enuresis after a dry year is a sentence, not an accident).

Proposition 2. Equifinality obliges the differential history. Three children present with the same conduct difficulty at nine (Figure 12.1, left). The first needs a caregiving intervention, the second a language and communication intervention with the classroom's demands changed, the third a medical and neuropsychological pathway. A service that treats the presentation delivers two wrong treatments out of three, and will blame the children for the failures of its own uniformity.

Proposition 3. Multifinality obliges the protective audit. The same early adversity diverges (Figure 12.1, right), and the divergence is not random: it tracks the buffers of week 10 — the committed adult, the routine, the meaning, the school that held. Risk assessment that counts adversities without counting protections is half a science and, in practice, half a cruelty.

PRACTICE NOTE

Clinical implication. Write into every formulation the two principle-sentences: "this presentation could arise from at least these courses…" (equifinality) and "given this history, these outcomes remain open…" (multifinality). The sentences keep the formulation humble and the plan revisable.

Lecture II — Continuity that changes its clothes

Heterotypic continuity is the reason developmental histories read like family novels: the vulnerability persists while the symptom changes costume with development. The behaviourally inhibited toddler does not become "the same thing" at thirteen; the wariness that at two was clinging becomes at seven the refusal to read aloud, at thirteen the avoided party and the unposted application, and under adolescent self-scrutiny the low mood that arrives when a temperament meets a mirror. The internalising trajectory classically runs anxiety then depression; the externalising trajectory runs difficult temperament then oppositional conflict then conduct difficulty, with the coercive parent–child loop (week 6′s transaction) as the engine between stages.

Two clinical duties follow. First, **read the history forward**: a presenting depression at fourteen is interrogated for the earlier costumes — inhibition, selective silence, somatic complaints — because the formulation of the course differs from the formulation of the episode, and the prevention of the next costume differs from both. Second, **read the present backward with restraint**: heterotypic continuity is a group pattern, and most inhibited toddlers do not become depressed adolescents; the principle licenses enquiry, not prophecy, and the counsellor who prophesies has converted a probability into a self-fulfilling expectation in a parent's ear.

Lecture III — Cascades and transactions: how difficulties recruit the world

The developmental cascade explains maintenance across systems: the child's reading difficulty produces avoidance; avoidance produces lost practice; lost practice widens the gap relative to peers; the widening gap produces the public failures; the public failures produce the self-concept wound (week 8); the wound produces refusal; refusal produces the school's concern and the parent's pressure; the pressure produces the battles that now maintain the refusal independently of the reading. By year three, treating the reading alone opens one door in a locked house.

The transactional model (week 1) is the cascade's engine at the dyad level: the child's difficulty evokes a response that feeds the difficulty — the intense child evokes coercion, the withdrawn child evokes neglect of their needs, the anxious

child evokes accommodation that starves them of disconfirmation (volume 1, week 5). Transaction is the reason family-involved work is not an add-on to child treatment but often the treatment: the loop is the target.

The categories, finally, in their place. Diagnostic categories are hypotheses with utility: they predict course, recruit evidence, and open services. They are not the child, not the cause and not the plan; and their boundaries in childhood are softer than their numbering suggests, with comorbidity the rule and dimensions beneath every cut-point (week 11). Developmental psychopathology's discipline is to use the category as one sentence inside the formulation — "the pattern meets criteria for X, which predicts Y and responds to Z in these samples" — surrounded by the developmental sentences that make it true or false for this child.

> **SAFEGUARDING AND DUTY TO REPORT**
> The principles carry a safeguarding edge in the opposite direction from over-diagnosis: age-appropriateness cuts both ways, and some presentations are never normative at any age — sexualised behaviour beyond the child's developmental knowledge, cruelty to animals, fire-setting with planning, a disclosed secret with an adult's name attached. "It is probably developmental" is a hypothesis to test, and testing it includes asking the questions whose answers would end it.

Worked example 12.1 — Three nines, one presentation

Task. Three children present with the same conduct difficulty: lying, taking things, Explosions when challenged. Histories: (a) care by rotating relatives after a mother's departure at two, discipline erratic and sometimes frightening; (b) late language, still thin comprehension of the school language, episodes clustered after long verbal instructions; (c) uneventful history, high activity from infancy, incidents impulsive and immediately regretted. Formulate each in two sentences and name the treatment each implies.

Solution. (a) Equifinality via caregiving: conduct difficulty as an adaptation to unpredictable care — trust tested, control taken because control was never given; treatment begins with a stable committed adult and predictable discipline (week 10), not with consequences for lying, which is the child's only reliable instrument. (b) Equifinality via communication: conduct episodes as the behavioural tail of incomprehension — the child who cannot parse the rule breaks it and, challenged, cannot narrate why; treatment is language support and demand redesign (weeks 7 and 11), with the "lying" re-read as confabulation under verbal pressure. (c) A neurodevelopmental course: impulsivity with immediate regret, pervasive and early; treatment is the attention course with specialist input plus external structure (week 11), and the moral reading withdrawn from the file. Same presentation, three treatments, and a service that delivered the same package to all

three would produce three failures with three different true causes — which is the sentence equifinality exists to prevent.

Worked example 12.2 — Reading the history forward at thirteen

Task. Bisi (13) presents with low mood and school refusal. Compose the heterotypic enquiry.

Solution. Four questions to the history, in order. *Infancy and preschool:* was she slow to warm, clinging at drop-off, wary of strangers? (the inhibited costume). *Primary years:* selective silences — refusing to read aloud, mute at assemblies — and somatic complaints on performance days? (the second costume). *Late childhood:* friendships narrowing to one or none, parties declined with reasons that change? (the third). *Now:* the mood's content — "everyone saw me shake" — which locates the depression inside a social-evaluation history rather than beside it. The formulation then reads: an inhibited temperamental line, heterotypically continuous through selective silence and social avoidance, now depressed under adolescent self-scrutiny, maintained by avoidance of every evaluating situation and by a school response that removed demands without removing the fear (accommodation, volume 1 week 5). The plan follows the course, not only the episode: behavioural re-entry graded through non-evaluating situations first, the cognitive work on self-evaluation, and — because multifinality keeps other doors open — the protective audit of week 10, because the one friend who remains is the buffer this plan must not spend.

SKILLS LABORATORY

Practical 12 — the cascade map. Take one case you know or a supplied case with a multi-year history. Draw the cascade as a chain of at least six links across at least three systems (child, family, school, peers), marking at each link the transaction — what the child's difficulty evoked and how the evoked response fed it. Then circle the two links where an intervention this term is cheapest, and say what each would stop. One drawn page plus 200 words.

ASSESSMENT

Assessed task. An equifinality essay of 1,200 words: for one presentation of your choice (conduct difficulty, school refusal, or low mood), construct three plausible developmental courses with their evidence requirements, state the treatment each implies, and argue which single early observation would best discriminate between two of them.

Criterion	Weight
Three courses genuinely distinct, mechanisms named	35%

Criterion	Weight
Evidence requirements specified per course	25%
Treatments matched to mechanisms	20%
Discriminating observation observable and dated	20%

Formative. Write one heterotypic sentence about a case you know: the earlier costume, the present one, and the vulnerability you believe connects them — labelled as hypothesis.

UNIT SYNTHESIS

Synthesis. Developmental psychopathology makes the course the unit: deviance judged against age and context, equifinality obliging the differential history, multifinality obliging the protective audit, heterotypic continuity reading the vulnerability through its changing costumes, cascades and transactions explaining how difficulties recruit their own maintenance — and the diagnostic category restored to its proper size, one predictive sentence inside a developmental formulation.

READING (APA 7TH)

- Cicchetti, D., & Cohen, D. J. (Eds.). (2006). *Developmental psychopathology* (2nd ed.). Wiley. — **core text** for this week; the conceptual chapters.
- Berk, L. E. (2013). *Child development* (9th ed.). Pearson.
- Rutter, M., & Sroufe, L. A. (2000). Developmental psychopathology: Concepts and challenges. *Development and Psychopathology, 12*(3), 265–296.
- Sameroff, A. J. (2009). The transactional model. In A. J. Sameroff (Ed.), *The transactional model of development: How children and contexts shape each other* (pp. 3–21). American Psychological Association.
- Santrock, J. W. (2019). *Child development* (15th ed.). McGraw-Hill Education.

Seminar questions

1. A behaviour is expectable at two and deviant at seven. What is the counsellor's sentence to a parent who believes the reverse in either direction?
2. Equifinality makes uniform treatment unethical in what precise sense? Name the sense.
3. Heterotypic continuity licenses enquiry but not prophecy. Where is the line in a conversation with a worried parent?
4. Draw from memory one cascade you have watched in practice. Which link did the service treat, and which link was cheapest?
5. Some presentations are never normative. What in your training tells you which, and what does it miss?

Next week

Week 13 integrates the volume: the developmental formulation — gates of difference, delay and disorder, the age in every sentence, and the plan with its falsifier — worked in full on Ife and Kunle.

13 LECTURE UNIT 13 Teaching week 13 4 notional hours

Integration: The Developmental Formulation

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Write** a developmental formulation of one page: description, developmental history, the three gates (difference, delay, disorder), mechanism across levels, the age-appropriateness verdict, the zone, the plan and its falsifier.
2. **Run** the three gates on a concern in order — difference, then delay, then disorder — and state what each gate excludes.
3. **Place** the age in every clinical sentence, distinguishing what is normative here from what is not.
4. **Integrate** the volume's strands on one child: cognitive, emotional, linguistic and contextual evidence in a single coherent account.
5. **Present** the formulation to a family and to a panel, including what remains unknown and who else must be told.

Where this week sits

Thirteen weeks of strands become one instrument. The developmental formulation is this volume's counterpart to volume 1's five-P formulation, with one addition that changes everything: the age. A formulation without the age is a formulation of a small adult; the developmental formulation asks, of every sentence, whether it is true of a child of this age in this context — and it carries the semester's running case to its integration.

Key concepts

THE THREE GATES

Difference (context, culture, language: assess in the child's own language and settings first), delay (timing: corrected age, sequence, range, with watchful waiting that has a date), disorder (pervasive across contexts, impairing, off-course in slope rather than in start). Only what passes all three is a disorder, and it remains a formulation, never a label. In the room: the ordered discipline between a concern and a category.

AGE-APPROPRIATENESS VERDICT
For each presenting behaviour, the explicit sentence: expected here, or not, for this age and context — with the norm and its range named. In the room: the sentence that dissolves half of all referrals and sharpens the other half.

DEVELOPMENTAL ZONE
What this child can do next with help, in the domain of the difficulty (week 4): the formulation's target, stated as the assisted level and the next independent step. In the room: the difference between a plan and a wish.

CASCADE LINK
The maintenance chain across systems (week 12), drawn as links with the transactions marked, so the plan can aim at the cheapest openable link rather than at the loudest symptom. In the room: the reason the plan has an order.

Lecture I — The gates, in order

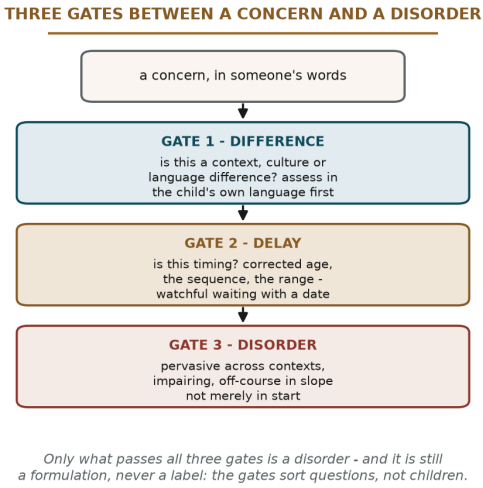


Figure 13.1 The three gates between a concern and a disorder: difference (context, culture, language), delay (timing, corrected age, sequence), disorder (pervasiveness, impairment, slope); the gates sort questions, not children. *Source:* prepared for this edition.

Proposition 1. Order is protection. The gates run in a fixed order because each later gate is more expensive and more stigmatising than the one before: a language or context difference misread as delay wastes months; a delay misread as disorder writes a sentence on a child's file that outlives the delay. Gate 1 asks whether the assessment so far has been conducted in the child's own language and settings; gate

2 asks whether the child is simply earlier on an ordinary sequence (with corrected age where relevant); gate 3 asks the disorder questions of week 11 — pervasiveness, impairment, slope.

Proposition 2. Watchful waiting is an intervention with a date. Gate 2′s outcome is often monitoring, and monitoring without a review date is neglect with paperwork. The dated wait names what will be watched, by whom, with what measure, and what will happen on the date — including referral if nothing has changed, which is the sentence that keeps the waiting honest.

Proposition 3. The gates do not exclude each other's findings. A child may pass gate 1 (assessed correctly in her language), pass gate 2 (no delay on sequence) and still not meet gate 3 — a difference without disorder, needing support rather than treatment; or meet gate 3 in one domain while a delay explains another. The formulation records each gate's verdict separately, because services read the verdicts, not the adjectives.

> **PRACTICE NOTE**
> **Clinical implication.** Write the three gate-verdicts as three labelled lines at the top of every developmental formulation. A reader — a school, a medic, a court, the child at twenty — should see in three lines what was excluded and how.

Lecture II — The one-page instrument

The developmental formulation fits a page because selectivity is its discipline:

1. **Description**, in the family's and child's words first, then the observer's: what brings them, since when, in whose day it bites.
2. **Developmental history**, compressed: prenatal and birth (week 2), milestones with the child's languages (week 7), temperament (week 6), attachments and separations, school and peer course (week 9), adversities audited with buffers (week 10).
3. **Gate verdicts**, three lines.
4. **Age-appropriateness verdicts**, one line per presenting behaviour, with the norm named.
5. **Mechanism across levels**: the child's own systems (attention, language, regulation, cognition), the dyads (the transactions that maintain), the contexts (demands, resources, culture), each with the strand and week it came from.
6. **Cascade links**, drawn, with the cheapest openable link circled.
7. **Zone**: assisted level now, next independent step, in the domain of the difficulty.
8. **Plan in order**, with dates; **falsifier**: the observation, dated, that would show the formulation wrong; **who else must be told**, and what the family has agreed about telling.

9. **What remains unknown**, stated as enquiry rather than as silence.

Nine lines of thinking, one page of writing. The page is complete when a colleague could continue the case from it and a family could correct it.

> **SAFEGUARDING AND DUTY TO REPORT**
> The formulation is a travelling record. Three rules from volume 1 apply with developmental force: write the child as a person developing, not as a list of deficits — the page may be read aloud to the child one day; name uncertainty as uncertainty, especially at the gates; and never let a cultural or familial observation become a character verdict in a document that outlives the case. Where the formulation touches possible harm, the safeguarding sentence overrides every other line and says so.

Lecture III — Ife and Kunle, integrated

The semester's running case closes its developmental movement here. Ife at four-and-a-bit: slow-to-warm temperament in a good-fit redesign (weeks 6 and 9), language within expectation in her home language with the school-language question open (week 7), separation behaviour age-appropriate in duration and falling (weeks 6 and 12), buffers strong and one strain — the pending separation of her parents (week 10). Gate verdicts: difference — school-language contribution assessed in home language first, excluded as sole cause; delay — settling time within range and shortening, no sequence delay; disorder — not met: pervasiveness absent (settles fully at home and with grandmother), impairment falling, slope on-course. Formulation: a temperamental style meeting a normative developmental task (separation-individuation) inside a family transition, maintained briefly by an over-fast goodbye sequence now redesigned; plan: hold the redesign, protect the routine card through the separation, review settling minutes monthly; falsifier: if settling time rises above fifteen minutes for four consecutive weeks after the separation settles, or if regression appears in language or toileting, the formulation returns to gate 2 and the separation work becomes the governing strand.

Kunle at seven-and-a-bit: neglected peer status engineered toward visibility (week 9), industry protected through effort-praise and the figures competence (week 8), inattention questions answered as context-sensitive and sub-threshold with a demands audit attached (week 11), buffers: father's Sunday hour, grandmother, church group (week 10). Gate verdicts: difference — school-language comprehension thin and contributing, support in place; delay — none; disorder — attention dimension elevated in one setting only, impairment mild and external (teacher tolerance), not met this term. Formulation: a quiet, slow-to-shift child whose difficulties are transactions with demanding contexts, with two protective strands to spend carefully and one strain — the same family transition. Plan: the sociometric engineering continues to its six-week review; the demands audit goes to the teacher as two accommodations (written plus oral instructions; a

two-minute warning before transitions); the Sunday hour is protected in writing through the separation; falsifier: teacher and home ratings converging upward on inattention at the six-week review opens gate 3 properly, with three settings' evidence and a specialist conversation.

Two children, one family transition, nine lines each. That is the instrument.

Worked example 13.1 — The gate that was skipped

Task. A six-year-old is referred with "speech delay" on the basis of a school-language sample alone; the family's language is not the school's. The file already carries the word "disorder". Reconstruct the correct sequence and the sentences that repair the record.

Solution. Gate 1 was skipped: no sample exists in the child's home language, so the "delay" is an artefact of sequential bilingualism's ordinary gap (week 7). The repair has three parts. First, the assessment: a home-language sample with a speaker of that language, hearing screen, and a bilingual vocabulary count across both languages (total conceptual vocabulary, not single-language). Second, the record: an addendum, not a deletion — "the earlier 'disorder' entry rested on a single-language sample; gate 1 (difference) was not assessed; it is now assessed with these results; the gate-2 and gate-3 questions reopen on [date] with [measure]." Records are repaired by addition and dating, never by silent erasure, because the earlier entry travelled and may be quoted. Third, the family conversation: what happened, in plain words, without defending the institution at length — "the first test was in a language that is not hers at home; that was our error; here is what we will do instead" — and the child's experience checked, because a six-year-old who was tested and found wanting in a half-known language carries that hour with her. The falsifier for the repaired position: if the home-language sample shows combination and vocabulary below the home-language expectation for age, gate 2 opens properly and the referral stands on evidence rather than on arithmetic.

Worked example 13.2 — The ninety-second developmental version

Task. Reduce Ife's formulation to ninety spoken seconds for her parents.

Solution. "Here is what we see, in plain words. Ife is not anxious in a worrying way; she is a child who needs time and one familiar face to change worlds, and that is her style from birth, not anything anyone did. Her settling at nursery was long because the goodbye was fast and the room was loud, and since we changed those two things the minutes have fallen from twenty to six — that is her development working, not a treatment working. Her speech is right on time in your language; the school language is new to her and we will keep watching it the honest way, in both languages. What we are holding now is her routine through the family change: the same three steps every morning, the same card, the same grandmother at four o'clock. And here is our promise: if the mornings lengthen

again for four weeks running, or anything she had learned goes backwards, we will say our picture was wrong and we will look again, together. Nothing about her is a disorder; a lot about her is development, and development is what we are here to protect." Ninety seconds; the gates in plain words; the falsifier as a promise.

SKILLS LABORATORY

Practical 13 — the formulation panel. In fives on a supplied complex case (full history pack): thirty minutes to write the one-page formulation; five minutes to present gates, mechanism, zone, plan, falsifier; six minutes of panel interrogation using week 15 of volume 1's six question types plus two developmental ones — "what is normal here for this age?" and "which gate's verdict is weakest, and what would strengthen it?" Rotate so every student presents once and panels twice.

ASSESSMENT

Assessed task. The formulation portfolio piece: a one-page developmental formulation of a supplied case with all nine elements, plus the recorded ninety-second family version, plus 300 words naming the weakest gate verdict and the observation that would strengthen it.

Criterion	Weight
Gate verdicts labelled, ordered, evidence-attached	25%
Age-appropriateness verdicts with norms named	20%
Mechanism across levels with strands cited by week	20%
Zone, plan order, dated falsifier	20%
Family version in their words, uncertainty stated	15%

Formative. Write the nine element-headings from memory, then check; the ones you forgot are the ones your cases will miss.

UNIT SYNTHESIS

Synthesis. The developmental formulation is the volume made instrument: gates of difference, delay and disorder run in protective order; the age placed in every sentence; mechanism gathered across the child's systems, dyads and contexts; the cascade drawn to find the cheapest link; the zone named as the target; the plan ordered with a dated falsifier and a list of who must be told — one page, nine elements, complete when a colleague could continue from it and a family could correct it.

READING (APA 7TH)

- Berk, L. E. (2013). *Child development* (9th ed.). Pearson. — **core text**; re-read the integrative final chapter.

- Cicchetti, D., & Cohen, D. J. (Eds.). (2006). *Developmental psychopathology* (2nd ed.). Wiley. — the formulation and assessment chapters.
- Mash, E. J., & Barkley, R. A. (Eds.). (2014). *Comprehensive clinical assessment of children and adolescents* (3rd ed.). Guilford Press.
- Shonkoff, J. P., & Phillips, D. A. (Eds.). (2000). *From neurons to neighborhoods: The science of early childhood development*. National Academy Press.
- Squires, J., & Bricker, D. (2009). *Ages & Stages Questionnaires, Third Edition (ASQ-3) user's guide*. Paul H. Brookes. — skimmed now for week 14's tools.

Seminar questions

1. Why must gate 1 precede gate 2 and gate 2 precede gate 3? Give a harm produced by each possible inversion.
2. Watchful waiting with a date: what exactly does the date commit the service to, and what does it commit the family to?
3. Which of the nine formulation elements do you most often omit under time pressure, and what does its omission cost the child?
4. A formulation may be read aloud by the child at twenty. Which sentence of yours would you most wish to revise, and how would you write it now?
5. Ife's gates all closed. What would have been different in the room — and in the record — if gate 3 had been run first?

Next week

Week 14 supplies the instruments: observation schedules, screening tools and their cultural fairness, multi-informant variance as data, and the assembly of your semester portfolio into a measurement practice you can keep.

14 LECTURE UNIT 14 Teaching week 14 4 notional hours

Observation, Screening and Measurement: The Instruments and Their Honesty

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Select** a developmental screening or rating instrument appropriate to age, language and setting, stating its norming population and its limits.
2. **Run** the screening pathway in order: concern recorded, screen in the child's language plus hearing and vision, focused assessment of the zone, then monitor-with-a-date or refer-with-the-record.

3. **Interpret** multi-informant variance (child, parent, teacher) as data about contexts rather than as error to be averaged away.
4. **Critique** one instrument's cultural fairness: norming sample, item content, translation versus adaptation, and what the critique licenses you to say.
5. **Assemble** a personal measurement portfolio from the semester's records, with a review date for each.

Where this week sits

Every judgement of this volume eventually meets an instrument: a checklist, a schedule, a rating scale, a sample, a screen. Instruments extend the eye and corrupt the judgement in equal measure, and the difference between the two is knowing what the instrument was normed on and what it was never asked about. This week builds the practitioner's measurement practice — and collects the semester's records (weeks 1, 5, 7, 9) into the portfolio you will keep.

Key concepts

SCREENING VERSUS ASSESSMENT

Screening sorts quickly among the many (is this concern worth a closer look?); assessment describes slowly among the few (what is this, for this child?). A screen is never a diagnosis, and a negative screen is never a clean bill — it is a date on which to look again if the concern persists. In the room: the sentence that keeps a screened-negative child on the watch list.

NORMING POPULATION

The children on whom an instrument's cut-points were set. Scores are comparisons with that population; a child outside it is being compared with strangers, and the score's meaning changes accordingly. In the room: the first question about any number: "compared with whom?"

MULTI-INFORMANT VARIANCE

Child, parent and teacher reports of the same child disagree systematically, because children are different people in different rooms; the disagreement locates the difficulty's context (home, school, peer group, or the child's inner world that only self-report reaches). In the room: variance as the finding, not as noise — and the reason averaging informants destroys the information.

ADAPTATION VERSUS TRANSLATION

Translating an instrument's words is not adapting its items: "rides a tricycle" presumes a tricycle; "plays pretend games with dolls" presumes the dolls of the norming

culture. Adaptation tests whether the item measures the same construct here, and re-norms where it can. In the room: the difference between a fair score and a fluent one.

Lecture I — The pathway and its order

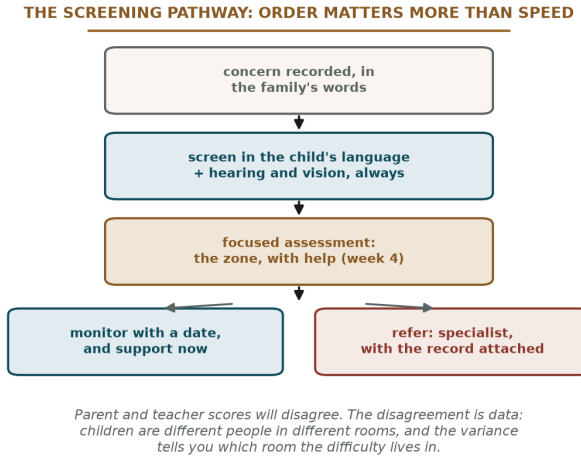


Figure 14.1 The screening pathway: concern recorded in the family's words; screen in the child's language with hearing and vision always; focused assessment of the zone with help; then monitor-with-a-date or refer-with-the-record attached. Parent–teacher disagreement is data about which room the difficulty lives in. *Source:* prepared for this edition.

Proposition 1. Hearing and vision precede everything. A surprising share of "developmental" referrals are auditory or visual, and every language, attention and learning question is unanswerable until the senses are cleared. The screen that skips the audiologist is not a shortcut; it is a gamble with the child's years.

Proposition 2. Screens in the child's language or not at all. Parent-completed developmental screens (the ASQ family among the most used) exist in many translations, but translation is not adaptation and neither is norming; where no adapted norm exists, the screen's output is a structured conversation with the parent — valuable, and labelled as such — rather than a score. The label travels with the number into the file.

Proposition 3. The focused assessment is the zone, not the stock. After a positive screen, the assessment that changes practice is week 4's dynamic one: what the child does alone, what with help, and what the help profile says. Static re-testing after a screen mostly re-measures the screen; the assisted level measures the next term.

PRACTICE NOTE

Clinical implication. Attach the record to every referral: the screen and its language, the hearing and vision results, the assisted-level profile, the settings' evidence, and the family's own words. A referral without its record invites a re-assessment from zero, and the child pays for the repetition.

Lecture II — Multi-informant variance as the finding

Parent, teacher and child scores on the same construct correlate modestly at best, and the pattern of disagreement is informative in four canonical shapes: **home-only elevation** (difficulty in the family climate: look at stress, conflict, parenting fit); **school-only elevation** (difficulty in the classroom's demands or peer ecology: look at class size, language of instruction, peer status); **cross-informant elevation with child silence** (the child's inner experience not reaching self-report: common in young children and in stoic cultures — and a reason to use play and projective-adjacent methods with care); **child elevation alone** (the inner world no observer sees: anxiety, low mood, self-criticism — the shape that most often ends in "nothing wrong" files and is the most dangerous).

The discipline: report the variance, never average it. A file that says "scores were inconsistent" has destroyed the data; a file that says "elevated at school, not at home, with the child reporting X" has located the work. Where informants disagree about severity rather than context, the tie-breaker is function: which room is the child losing something they want in?

Lecture III — Cultural fairness: four questions per instrument

Norming: on whom were the cut-points set, and when? A mid-century Western norm applied to today's children in this region compares them with strangers across both space and time. **Items:** what does each item presume — objects, routines, sleeping arrangements, foods, toys — and which items change meaning here? **Translation or adaptation:** was the version in your hand validated here, or merely rendered? **Consequence:** what decisions ride on the score, and who bears the error in each direction — the false positive's label or the false negative's lost years?

The honest conclusion is not instrument-nihilism: adapted screens and ratings still outperform unstructured impression for catching delay early, and early catching is the whole game. The conclusion is labelled use: the score, its population, its language and its limits in one sentence in the file, every time — so that the next reader, and the child at twenty, can see exactly what the number was worth.

SAFEGUARDING AND DUTY TO REPORT

Instruments in safeguarding contexts carry asymmetric error costs: a screen that under-detects neglect because its items presume middle-class homes (clean clothes, regular meals, toys) can miss the neglect it was built to find, while over-detecting poverty as neglect punishes families for being poor. Where neglect is the question, the instrument is a prompt to look, and looking means the home visit, the growth chart, the school attendance record and the child's own account — never the checklist alone.

Worked example 14.1 — The divergent file, read properly

Task. Kunle at eight: teacher rating elevated on inattention; mother rating within range; Kunle's own one-page self-report: "I am fine; the class is too fast." Interpret and act.

Solution. The shape is school-only elevation with child attribution to context — not the classic home/school split but a child who locates the difficulty outside himself, which is both information and a defence to hear gently. Read the variance as data: the difficulty lives where demands are fast, oral and single-channel (the classroom), and does not live where demands are slow, written or one-to-one (home, church, your room). The interpretation is therefore not "Kunle overstates" or "mother under-reports" but a demand-context interaction consistent with week 11's formulation: sub-threshold attention sensitivity expressed under high oral load. Action: no new label; the two accommodations continue and are now evidence-based by variance; add one written-channel trial for four weeks (instructions on card plus oral) with the teacher rating repeated at the end — the variance itself becomes the outcome measure: if school elevation falls toward home levels, the demands hypothesis is confirmed by its own instrument; if it does not, gate 3 reopens with three settings' evidence. The file sentence: "informant variance interpreted as context-specific demand sensitivity; written-channel trial to [date]; teacher re-rating due [date]; no averaging performed."

Worked example 14.2 — A screen and its sentence

Task. A nursery asks you to "do the ASQ" on a thirty-month-old whose home language is not the nursery's. Compose the practice and the file sentence.

Solution. Practice: administer the adapted home-language version with the parent as respondent, in the parent's language, with you present to observe rather than to score; hearing and vision screened the same week; the tricycle-type items replaced in conversation with local equivalents (what does she ride, climb, carry?) and the substitution noted. Suppose the communication item set flags concern: the focused assessment is then a home-language language sample (week 7) plus a dynamic vocabulary task in both languages (week 4), because the screen's flag may be a true language delay, a bilingual gap, or an item artefact — and only the sample

and the zone can tell. The file sentence, one line: "ASQ-3 administered [date] in [language] adapted version, parent respondent; communication area flagged; norming population [X], not this child's population, score treated as structured parental report; hearing and vision clear [date]; home-language sample and two-language dynamic vocabulary assessment due [date]; review date [date]." Every number in the file now carries its worth in writing.

SKILLS LABORATORY

Practical 14 — the portfolio assembly. Gather your semester records: the two observation records (week 1), the EF demand audit (week 5), the language sample (week 7), the play record (week 9), and any case timelines. For each: one line stating what it measures, what it cannot measure, its review date, and the decision it currently supports. Bind them with a one-page index. This portfolio is the assessed artefact of the week and the seed of your practice's measurement file.

ASSESSMENT

Assessed task. A screening critique of 900 words: choose one instrument you have used or been given; state its norming population, three item presumptions, its translation/adaptation status, and the error costs in each direction for your setting; then write the file sentence you would attach to any score it produces.

Criterion	Weight
Norming population and its distance from the child stated	25%
Item presumptions concrete and local	25%
Adaptation status verified, not assumed	20%
Error costs in both directions argued	15%
File sentence complete (score, population, language, limits, date)	15%

Formative. Submit your portfolio index page.

UNIT SYNTHESIS

Synthesis. Instruments extend the eye when their limits travel with their numbers: hearing and vision first, screens in the child's language and labelled by norming population, focused assessment as the zone rather than the stock, multi-informant variance read as the location of the difficulty and never averaged away, cultural fairness asked as four questions per instrument — and every score in the file carrying one sentence of what it was worth, with a review date attached.

READING (APA 7TH)

- Achenbach, T. M., & Rescorla, L. A. (2000). *Manual for the ASEBA preschool forms & profiles*. University of Vermont, Research Center for Children, Youth, & Families.
- Berk, L. E. (2013). *Child development* (9th ed.). Pearson. — **core text**; the assessment appendix.
- Mash, E. J., & Barkley, R. A. (Eds.). (2014). *Comprehensive clinical assessment of children and adolescents* (3rd ed.). Guilford Press.
- Squires, J., & Bricker, D. (2009). *Ages & Stages Questionnaires, Third Edition (ASQ-3) user's guide*. Paul H. Brookes.
- World Health Organization. (2019). *International classification of diseases* (11th ed.). — for the functional language of impairment.

Seminar questions

1. "Compared with whom?" Name an instrument used in your setting and answer for it honestly.
2. Why is averaging informants worse than reporting their disagreement? Give a case where the average hid the child.
3. A screen flags concern in a language with no adapted norm. What exactly may you write in the file, and what may you not?
4. False positive versus false negative: which error does your service tolerate more easily, and who pays for that tolerance?
5. Your portfolio: which record in it would most change a decision if it were lost, and what does that say about your practice?

Next week

Week 15 is the practicum: three ages — a toddler with a language red flag, a school-age child with attention and learning questions, an early adolescent with mood and peer difficulties — each formulated and defended before the panel.

15 LECTURE UNIT 15 Teaching week 15 4 notional hours

The Practicum: Three Ages, One Discipline

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Formulate** three cases at three ages — toddler, school-age, early adolescent — using the one-page developmental instrument, under panel conditions.
2. **Defend** each formulation's weakest gate verdict and its falsifier under interrogation.

3. **Adapt** method and language to developmental age: play and parent work with the toddler, concrete external tools with the school-age child, collaborative hypothesis-testing with the adolescent.
4. **State**, for each case, who else must be told and what the family has agreed about telling.
5. **Give and receive** panel feedback in the register of the profession: mechanism, evidence, child.

Where this week sits

The volume's examination of practice. Three fictitious composites stand for the three doors your work will actually open: the toddler brought by a parent who has been told to wait; the school-age child brought by a school that has stopped waiting; the adolescent who has not been brought at all. Each is formulated in full, presented in five minutes, and defended for six. The discipline is the semester's: gates in order, the age in every sentence, the plan with its falsifier, and the child's own words somewhere in the room.

Key concepts

AGE-GRADED METHOD

The same instrument, three deliveries: with the toddler, the parent is the method and play is the language; with the school-age child, external tools — cards, charts, boxes, rituals — carry the work the inner speech cannot yet; with the adolescent, the work is collaborative hypothesis-testing between two people, one of whom is not yet sure they are a person. In the room: method follows development, not preference.

THE WAITING PARENT

The toddler case's true client is often the parent who has been told "wait and see" without a date, and whose trust in services is now the scarce resource. In the room: the dated wait (week 13) as the repair of a broken promise.

THE REFERRING SYSTEM

The school-age case arrives inside a system that has already formed a verdict; the formulation must answer the system's question while refusing its adjective. In the room: the panel skill of holding two audiences — the child and the institution — without betraying either.

THE UNBROUGHT ADOLESCENT

The adolescent case tests whether the formulation can hold a client who declines the room: engagement as the first intervention, consent and confidentiality as its

materials, and the gate questions run on sparse data with the sparseness declared. In the room: the honest formulation's line, "what we do not know, and how we will earn it".

Lecture I — The three cases

Case A — Zara, 2 y 6 m (fictitious composite). Brought by her mother after a nursery remark that Zara "has no words like the others". Home language differs from the nursery's; no words in either language beyond four; no pointing at eighteen months by recall; hearing never tested. The mother has been told twice to wait, without dates, and asks, quietly, whether the professionals think she is a bad mother.

Case B — Emeka, 8 y 2 m (fictitious composite). Referred by his school with "attention and reading problems, possible dyslexia, mother uncooperative". Records: oral instructions lost, reading below class expectation, two unfinished assessments; the mother has missed three meetings. Your first phone call learns that the meetings were scheduled at her market hours and that she believes the school wants her son out.

Case C — Titi, 13 y 4 m (fictitious composite). Not referred: self-referred through a school counsellor's open-door hour, then stopped coming. One session attended: spoke of "nothing being worth the stress", of a friendship group that reorganised without her, and of a mother who "would fall apart if she knew anything was wrong". Declines further sessions; agrees to one more if "nothing is written down that my mum will see".

Lecture II — The panel's questions, and the answers that hold

The panel asks six questions per case, and each has a legitimate shape of answer.

What is normal here for this age? — Zara: four words at thirty months is below expectation in any language and is a red-flag territory (week 7), but the absence of pointing at eighteen months is the heavier grain and the hearing test the first door; Titi: friendship reorganisation and mood volatility at thirteen are normative, but "nothing being worth the stress" is not a normative sentence at any age and is screened for risk in the session, not in the file.

Which gate verdict is weakest? — Zara: gate 1 (both languages unsampled) and the missing hearing test make every later verdict provisional; declared, not hidden. Emeka: gate 1 again — reading assessed in the school language only, and the "uncooperative mother" is a gate-1 finding about the system, not about the mother. Titi: gate 3 cannot be run at all on one session; the formulation states that its disorder question is open and names the single session that would advance it.

What maintains this now? — Zara: the waiting without dates (maintenance by system), plus possible hearing; Emeka: oral-only instruction plus a home–

school rupture that removes the one person who could coach reading at night; Titi: avoidance of the reorganised group plus a family rule that the mother must be protected, which makes Titi the family's regulator at thirteen.

What is the zone? — Zara: joint-attention and gesture work with the mother as scaffold, assisted level now; Emeka: reading with written-plus-oral input and a phonological task with help; Titi: one more session, on her terms, as the assisted level of engagement itself.

What is the falsifier, dated? — Zara: home-language sample and hearing result by [date]; if combinations absent in the home language, gate 2 opens with a named referral. Emeka: four-week written-channel trial with teacher re-rating (week 14's variance method). Titi: if she does not return by [date], the counsellor initiates one low-demand contact (a note she collects, not a summons), and risk screening is repeated at that contact.

Who else must be told, and what has the family agreed? — Zara: the nursery, with the mother present when it is said; Emeka: the school's leadership and the mother in one meeting, the adjective withdrawn in the first sentence; Titi: confidentiality and its limits stated in her words at the start of session two, including the one circumstance in which the mother is told the same hour, and Titi's agreement recorded.

SAFEGUARDING AND DUTY TO REPORT

Case C carries the week's sharpest edge: an adolescent's confidentiality is the material of engagement and a right, not a courtesy — and it has one limit the counsellor states before it is needed: risk to life. The sentence is prepared, not improvised: "What you tell me stays with me, except one thing: if I believe your life is in danger, I will tell you first, and we will tell together." A counsellor who cannot say that sentence plainly should not hold an adolescent's secrets.

Lecture III — Age-graded method: one instrument, three deliveries

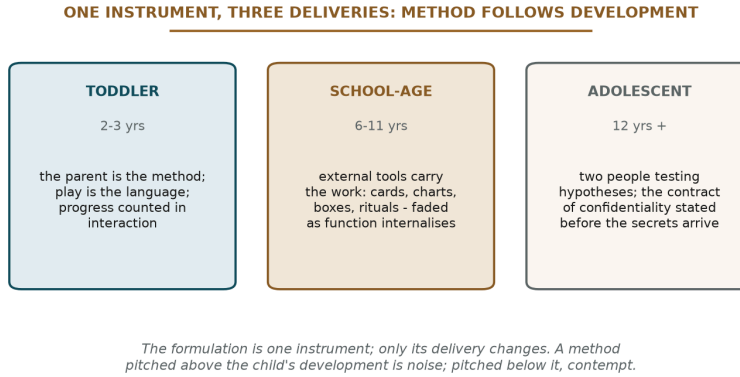


Figure 15.1 Method graded to development across the three ages of the practicum: with the toddler the parent is the method and play the language; with the school-age child external tools carry the work; with the adolescent the work is collaborative hypothesis-testing under an explicit confidentiality contract. *Source:* prepared for this edition.

Proposition 1. With the toddler, the parent is the method. A two-year-old cannot be treated around the caregiver; the intervention is the caregiver's sensitivity, language and routine, coached in play and in the kitchen rather than in the chair. The child's progress is measured in the dyad's interactions, and the falsifier is an interaction count, not a score.

Proposition 2. With the school-age child, external tools carry the work. Cards, charts, boxes, rituals and the fold in the page do the holding the child's inner speech cannot yet do (weeks 4 and 5); the tool is faded as the function internalises, and the fading is the record of success. Concrete operations mean concrete instruments: the eight-year-old's formulation belongs on a card the eight-year-old can hold.

Proposition 3. With the adolescent, the work is two people testing hypotheses. The early adolescent can hold minds-in-minds and hypotheses about themselves (volume 1, weeks 3 and 6); the method is therefore collaborative: the formulation is drafted with them, in their words, with its uncertainty visible, and the confidentiality contract stated before the secrets arrive. Engagement is not the prelude to this work; at thirteen, engagement is the work, and every other intervention waits on it.

PRACTICE NOTE

Clinical implication. Before any session, ask the age question of the method: what can this child hold in mind today, and what must the room hold for them? The answer chooses the materials before the materials choose the session.

Worked example 15.1 — The five-minute presentation, Case B

Task. Present Emeka's formulation to the panel in five minutes, in the order the instrument requires.

Solution. Ninety seconds of description, opening with the mother's sentence ("they want my son out") and the school's ("possible dyslexia, mother uncooperative"), held side by side without adjudication. Thirty seconds of history: birth and milestones unremarkable; language in the home language strong by mother's account and untested by anyone else; reading taught in the school language from year one. Gate verdicts, thirty seconds: gate 1 failed by the system twice — no home-language sample, and the "uncooperative" label is a scheduling artefact now corrected by one phone call; gate 2 open — reading delay may be timing plus teaching-debt in a second language; gate 3 not met on current evidence — pervasiveness untested (home reading unobserved), impairment real but school-located. Mechanism, sixty seconds: oral-only instruction meeting a thin school-language comprehension (week 7), plus a home–school rupture removing the night coach (week 10), plus a class demand structure that maximises his loss (week 11) — three courses, one of which (the rupture) is repairable this month. Zone, thirty seconds: with written-plus-oral input and a phonological game with help, Emeka reads two clauses ahead of his alone level; the next independent step is one paragraph with a card. Plan and falsifier, sixty seconds: repair the alliance first — one meeting at her hours, the adjective withdrawn in the first sentence, the market-day schedule honoured; the written-channel trial four weeks with teacher re-rating; home reading in the home language twice weekly, mother as expert rather than defendant; falsifier dated at the re-rating. Close, thirty seconds, with Emeka's own sentence from the one observation session — "the book is shouting" — and the panel's sixth question answered before it is asked: who must be told is the school leadership and the mother, in one room, and the family has agreed to the telling provided the word "uncooperative" leaves the file by addendum this week.

Worked example 15.2 — The panel's hardest question

Task. The panel asks on Case C: "Your formulation rests on one session. Why should we accept any of it?" Answer in the register of the profession.

Solution. "You shouldn't accept it as a formulation of Titi; you should accept it as a formulation of what one session can hold. What it holds: a thirteen-year-

old whose mood sentence is not normative at any age and was screened for risk in the room, with a negative screen and a safety plan she co-wrote; a peer ecology that reorganised without her in the term's first month; a family rule that makes her her mother's regulator, which is a maintaining factor I can defend from two observations inside the session and from the literature on parentified children; and a confidentiality condition that is the engagement itself. What it does not hold: any gate-3 verdict, any zone beyond the next contact, and any claim about what she is like when no professional is present. The falsifier is the return date; the method for earning the next sentence is one low-demand contact she controls; and the file says all of this in those words. A formulation that declares its own thinness is not weak practice; it is the only honest practice on one session — and the alternative, a confident formulation built from nothing, is what panels should be failing."

SKILLS LABORATORY

Practical 15 — the practicum, run as panels. In fives across the full block: each student presents one of the three cases (allocated so all three are presented across the group) for five minutes, defends for six, and panels twice. The fifth member records, per presentation, which of volume 1's five failure modes appeared (technique salad, lineage loyalty, false neutrality, unanswerable hedging, the child absent) and which of this volume's two developmental failure modes: the ageless formulation (no age-appropriateness verdict anywhere) and the gateless formulation (a category without gates). The cohort's three most common modes go on the board for week 16.

ASSESSMENT

Assessed task. The practicum portfolio: the five-minute recorded presentation of your allocated case, your panel record of two peers' presentations (one page each, in the register of mechanism–evidence–child), and a 300-word reflection naming your own dominant failure mode across the two volumes and its repair.

Criterion	Weight
Instrument complete under time: gates, age verdicts, zone, plan, falsifier	35%
Defence under interrogation: weakest verdict owned, sparseness declared	25%
Age-graded method evident in the plan	20%
Peer records in the professional register	10%
Reflection: failure mode named and re-paired	10%

Formative. Write the confidentiality sentence you will actually use with adolescents, in your counselling language, and rehearse it aloud once.

UNIT SYNTHESIS

Synthesis. The practicum tests the volume's discipline at three ages: method graded to development, gates run in order with their weakest verdict owned, the referring system answered without adopting its adjective, the unbrought adolescent held by confidentiality and dated contact, and every formulation declaring its own thinness where it is thin — because the panel, like the family and like reality, trusts the practitioner who can show the work and name its edges.

READING (APA 7TH)

- Berk, L. E. (2013). *Child development* (9th ed.). Pearson. — **core text**; re-read your weakest strand's chapter before presenting.
- Cicchetti, D., & Cohen, D. J. (Eds.). (2006). *Developmental psychopathology* (2nd ed.). Wiley.
- Geldard, K., Geldard, D., & Foo, R. Y. (2018). *Counselling children: A practical introduction* (5th ed.). SAGE. — the age-graded method chapters.
- Mash, E. J., & Barkley, R. A. (Eds.). (2014). *Comprehensive clinical assessment of children and adolescents* (3rd ed.). Guilford Press.

Seminar questions

1. Which of the three cases would you most dislike to receive on a Friday evening, and what in you makes it so?
2. "Mother uncooperative" is a gate-1 finding about the system. Name three other file adjectives that are really findings about services.
3. An adolescent's confidentiality has one limit. Why must the sentence be prepared before it is needed?
4. The ageless formulation: where in your own writing this semester did an age-appropriateness verdict go missing?
5. What did the panel process teach you about receiving feedback that the receiving of marks never could?

Next week

Week 16 consolidates the volume and prepares the examination: the course map, the retrieval schedule, specimen questions with model answers, and the closing argument of the developmental position.

16 LECTURE UNIT 16 Teaching week 16 4 notional hours

Revision, Integration and Examination

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Reconstruct** the volume from memory as one map: four strands, the shared ground, and the developmental position.
2. **Answer** the four questions of week 1 — what develops, by what process, with what universality, to what purpose — for any case in the volume.
3. **Use** retrieval practice and spaced revision on a four-week schedule you can keep.
4. **Write** examination answers in the three required formats: the comparative essay, the short developmental answer, and the case-based formulation, each to its mark scheme.
5. **State** the ethical obligations this volume attaches to developmental knowledge: the age in every sentence, the gate before the category, and the child's words in the room.

Where this week sits

The last week of CAC. 701 Child Development Theories. Nothing new enters; everything is retrieved, examined and closed. The block runs the consolidation, the examination, and the volume's closing argument — which is a single sentence about what a developmental education does to a counsellor's eye.

Key concepts

THE FOUR QUESTIONS RETURNED

What develops (sequences and their ranges); by what process (maturation, learning, transaction); with what universality and variation (which patterns travel, which were local all along); to what purpose (the child in front of you, this week). In the room: the revision key and the examination's hidden mark scheme.

RETRIEVAL PRACTICE

Attempting recall before re-reading, spaced over days; the fluency of re-reading is the least reliable feeling of knowing. In the room: close the book, draw the map, then open the book and mark in a second colour; the colour of the corrections is the syllabus.

THE DEVELOPMENTAL SENTENCE

The examination's highest-mark form: "for a child of this age in this context, X is expected / not expected, because Y, and the mechanism that would change it is Z, which we would know is working by W." In the room: age, norm, mechanism, observation — four clauses, most of the marks.

WHAT THEORY FORBIDS

Each strand's prohibition: Piaget forbids teaching above the child's construction; Vygotsky forbids help that never fades; the temperament account forbids character adjectives; the ACE literature forbids prognosis; the categories forbid being the child. In the room: the ethical content of a development course.

Lecture I — The map of the volume

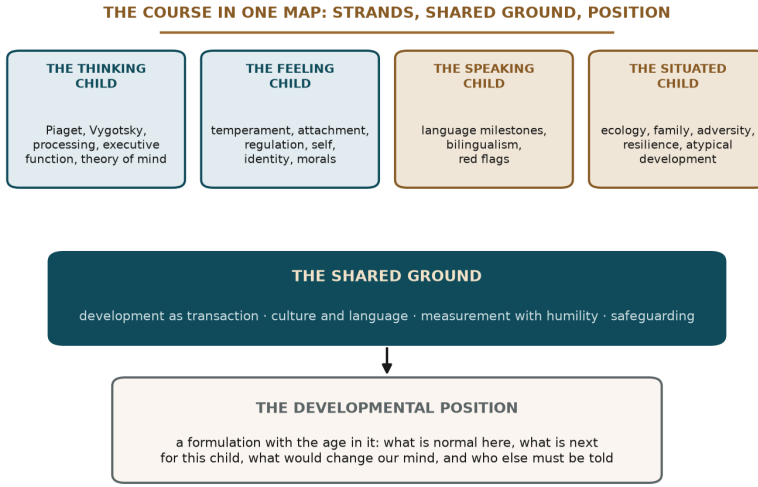


Figure 16.1 The course in one map: the thinking child, the feeling child, the speaking child and the situated child, converging on the shared ground of transaction, culture and language, measurement with humility and safeguarding, issuing in the developmental position — a formulation with the age in it. *Source:* prepared for this edition.

The four strands, one line each. The thinking child: Piaget's constructor, Vygotsky's apprentice, the processing system with its bottlenecks, the executive functions and the discovery of other minds (weeks 3–5). The feeling child: temperament and goodness of fit, attachment and co-regulation, the self and its Eriksonian balances, moral reasoning in two registers (weeks 6, 8). The speaking child: the milestone sequence with its red flags, bilingualism as competence, the input that builds language (week 7). The situated child: ecology, parenting

climates under stress, adversity and ordinary magic, peers and gender, and the atypical courses read developmentally (weeks 1, 9–12).

The shared ground. Development as transaction — the child acting and the context answering, each editing the other; culture and language as conditions of every claim, including this volume's; measurement with humility — the number and its worth in one sentence; safeguarding overruling all of it, including the gates, when harm is in the room.

The developmental position. A formulation with the age in it: what is normal here, what is next for this child, what would change our mind, and who else must be told. Memorise the sentence; it is the answer to the largest question the examination can ask and to most of the ones your career will.

Lecture II — Four weeks of revision that works

Week A: retrieval. Draw Figure 16.1 from memory; then answer the four questions for Ife, for Kunle and for one case of your own, one page each, unseen. Mark in a second colour; re-attempt in three days.

Week B: gates and norms. For each of six presentations (tantrums at two and at seven; no words at eighteen months; inattention at six; low mood at thirteen; parallel play at three; night waking at four), write the age-appropriateness verdict with its norm and range, then the gate you would run first and why. This is the examination's Format 2 in embryo.

Week C: mechanisms and prohibitions. For each strand, the three-sentence mechanism answer and the three-sentence prohibition (week 16's fourth keypoint), plus one sentence on whose sample the strand's norms were built on and what you therefore check.

Week D: timed practice. One Format 1 essay and one Format 3 formulation under time; then the portfolio's index page re-read, because your own records are the best past paper you will ever sit. Sleep before the examination is part of the syllabus; the memory you are examining was consolidated in it.

> **PRACTICE NOTE**
> **Clinical implication.** The same four-week cycle is reflective practice under another name: retrieve what you expected, compare with what happened, correct the map, try again. Revision discipline and clinical supervision are one skill at two ages.

Lecture III — The examination: formats, marks, closing argument

Format 1 — the comparative essay (one from two, 45 minutes, 40%). Typical stem: *"Compare Piaget's and Vygotsky's accounts of why a seven-year-old fails a conservation task, and argue which should govern the first three weeks of teaching for a child who has never been to school."* What earns marks: mechanism-level comparison, the strongest form of the rejected account, a governing choice with

a dated falsifier, and the schooling caveat of week 3 stated as evidence rather than as piety. What loses marks: biography instead of mechanism, and neutrality.

Format 2 — the short developmental answer (six from eight, ten minutes each, 30%). Typical stem: *"State whether night waking at four years is expected, name the norm and its range, and give one mechanism that would maintain it and one observation that would show the mechanism operating."* Four clauses carry the marks: age-verdict, norm, mechanism, observation. Definition without the age-verdict earns the lowest band; the age-verdict without the observation earns the middle; all four earn the top.

Format 3 — the case-based developmental formulation (one compulsory, 45 minutes, 30%). A new case with a history pack and two informants' ratings. Required: the nine elements of week 13, the informant variance read as context, the gates in order with the weakest verdict owned, and the ninety-second family version. The examiner reads the gate verdicts first, because their absence is the fastest indicator of an unformed developmental mind.

ASSESSMENT

Examination. Three hours, closed book: one comparative essay (40%), six short developmental answers (30%), one case-based formulation (30%). Coursework already assessed: week 1 observation records, week 3 conservation log, week 4 scaffolding exercise, week 5 false-belief critique, week 6 goodness-of-fit analysis, week 7 language sample, week 8 moral interview, week 9 play record, week 10 adversity map, week 11 timeline, week 12 equifinality essay, week 13 formulation portfolio, week 14 screening critique and portfolio index, week 15 practicum.

Examiner's note. Candidates are not rewarded for naming theorists or milestones; they are rewarded for placing the age in the sentence, the mechanism under the age, the observation under the mechanism, and the child's own words somewhere on the page. An answer that could have been written without the case supplied has not read the case supplied.

Worked example 16.1 — A specimen Format 2 answer, marked

Task. Stem: *"State whether night waking at four years is expected, name the norm and its range, and give one mechanism that would maintain it and one observation that would show the mechanism operating."* Write the four-clause answer, then mark a weaker version.

Solution. Top-band answer: "Night waking at four is within the expected range when it is occasional and self-settling: brief night wakings occur in a substantial minority of preschool children and are normative; what is not expected is waking that requires parental presence every night and recurs most nights. One maintaining mechanism is negative reinforcement: the child's calling brings the parent, and the parent's arrival removes the child's distress, strengthening

the calling while starving the self-settling that would otherwise consolidate. The mechanism is shown operating if, on nights when the parent delays entry by agreed minutes, the child's self-settling rises across two weeks while total waking falls — and if it does not, the maintenance hypothesis is wrong and the formulation returns to fear content, sleep hygiene or a medical question." Four clauses: age-verdict with range, mechanism as process, observation that confirms, observation that refutes. Middle-band version marked: "Night waking at four is normal because children wake; it is maintained by habit and shown by the child waking." — the age-verdict lacks the range and the qualifier, the mechanism is a label ("habit") not a process, and the observation restates the symptom; two clauses of four.

Worked example 16.2 — The case-based formulation in forty-five minutes

Task. A new case is supplied with a history pack and two informants' ratings (teacher elevated, parent within range; child silent on self-report). Plan the forty-five minutes and give the skeleton.

Solution. Minutes: 5 reading the pack and marking the two ratings' divergence; 5 the description in the family's words from the pack's quotation; 5 the three gate verdicts with the weakest named (gate 1: no home-language evidence in the pack — declared); 8 the age-appropriateness verdicts per presenting behaviour with norms; 7 mechanism across levels, including the variance read as school-located demand sensitivity (week 14); 5 the cascade links with the cheapest circled; 5 zone, plan order, dated falsifier, who must be told; 5 the ninety-second family version drafted aloud; 5 review against the nine elements. Skeleton sentences: "For a child of eight in this context, losing oral instructions is expected under a forty-pupil oral-only regime and not expected as a stable trait, because the home rating and the one-to-one room disagree with the classroom; the mechanism that would change it is a written-plus-oral channel with a two-minute transition warning, which we would know is working by the teacher re-rating falling toward the home level at four weeks; the variance itself is our outcome measure, and no averaging is performed; what we do not know is the home-language reading level, and the sample due [date] will tell us." The examiner's first look is at the gate verdicts; the second is at whether the child's own words appear anywhere — in this skeleton, they do not yet, which is the one mark lost, and the candidate who notices it in review adds the quotation and takes it back.

The closing argument. You began this volume with four questions and a three-year-old at a nursery doorway. You end it with the same four questions and a discipline: to ask what is normal here before asking what is wrong; to run the gates in their protective order; to read the variance between informants as the location

of the difficulty; to count the buffers as seriously as the adversities; to let categories arrive last and leave first; and to keep, in every formulation, the sentence that says what would change your mind and by when. Development is not a timetable and a child is not a milestone chart. A developmental education, properly taken, does one thing to a counsellor's eye: it makes the eye slow enough to see a course instead of a state — and fast enough, when the red flags rise, to move the same day. The next volumes give you the disorders in full, the counselling methods, the ethics and law, and the supervised hours. Take the four questions, and the age, with you into all of them.

SKILLS LABORATORY

Practical 16 — the last seminar. In fours, ninety minutes. Unseen, each writes a Format 2 answer to the same stem; the group marks all four against the four-clause scheme, defends the marks, and agrees which to submit. Second half: each states one thing the volume changed in how they will watch a child, and one thing they still cannot do — and the group names where it is taught, in which volume or which placement. No one leaves without both statements made aloud.

UNIT SYNTHESIS

Synthesis. The volume reduces to one map and four questions: four strands over a shared ground of transaction, culture, humble measurement and safeguarding, issuing in the developmental position — a formulation with the age in it, gates in order, buffers counted, categories last, falsifier dated, and the child's words in the room. Revise by retrieval, examine by the developmental sentence, and practise by the same discipline.

READING (APA 7TH)

- Berk, L. E. (2013). *Child development* (9th ed.). Pearson. — **core text**; re-read the integrative chapter last.
- Dunlosky, J., Rawson, K. A., Marsh, E. J., Nathan, M. J., & Willingham, D. T. (2013). Improving students' learning with effective learning techniques: Promising directions from cognitive and educational psychology. *Psychological Science in the Public Interest, 14*(1), 4–58.
- Roediger, H. L., & Karpicke, J. D. (2006). Test-enhanced learning: Taking memory tests improves long-term retention. *Psychological Science, 17*(3), 249–255.
- Santrock, J. W. (2019). *Child development* (15th ed.). McGraw-Hill Education. — **core text**; the closing chapter against Berk's.
- Shonkoff, J. P., & Phillips, D. A. (Eds.). (2000). *From neurons to neighborhoods: The science of early childhood development*. National Academy Press.

Seminar questions

1. Which strand's norms travel least well to the children you will serve, and what will you check because of it?
2. State from memory one prohibition per strand. Which do you most often break under pressure?
3. Why is the age-verdict the examination's first clause, and the formulation's first gate?
4. What in this volume would you change for the next cohort, and with what evidence?
5. Where do you stand — culturally, professionally, personally — and how did that position shape which findings you found believable?

Next week

The examination, and then CAC. 702 and the volumes that follow. Take the four questions, and the age, with you.

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